Child DEVT AND Psychology

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DIPLOMA IN PRIMARY TEACHER EDUCATION
CHILD GROWTH AND DEVELOPMENT NOTES

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PURPOSE OF THE COURSE

This course will help the learner conceptualize the process of growth and development from conception to early childhood. It will give the learner the required knowledge, skills and attitudes towards expectant mother, infant, toddlers and pre-scholars.

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COURSE DESCRIPTION

This course entails definition of terms related basic concepts in child Development theories of child Development; Piaget theory of cognitive development, Erik Erikson theory of social development, Sigmund theory of psycho sexual development, Kohlberg theory of moral development .Application of theories to early childhood .conception and sex determinants, factors influencing growth and Development labor and the birth process, types of birth and their implication on infant, Birth complications , neo natal Development in various aspects, nurture vs nature controversy .Brain development; parts of the brain, psycho-social stimulation

TEACHING/LEARNING METHODOLOGIES

This course will be offered and/or undertaken through tutorials, group discussions, group and individual assignments, presentations, interactive questions and answers, micro-teaching lessons, peer teaching and e-learning interactive forums. The learner will be required to go through this training module, make notes based on the objectives of the course and attempt the questions given at the end of every lesson. Tutorial classes will be organized by the university where the lecturer will take the student through the course. The purpose of tutorials is to help the learner conceptualize the course. It is, therefore, important to study the module before attending the tutorials. Further reading in this area of curriculum is encouraged. However all information gathered should be within the course description and objectives.

COURSE ASSESSMENT AND EVALUATION

Learning will be assessed through sit in Continuous Assessment Tests (CATs), Take Away Assignments and a Main Examination. The CATs will constitute 30% and the Examination 70%

MODULE ORGANIZATION

This module is organized in form of lessons. In every lesson an introduction is given. The introduction shows what the lesson constitutes. This is followed by the lesson objectives. Then, the learner is taken through the lesson content. The content may be sub-divided into sub-topics depending on the nature of the topic in a lesson. A summary of what has been learnt is given. At the end of the lesson the learner is given activities. These activities are in form of questions and practical. It is good to note that this is not a text book. The content is therefore in form of lecture notes. Further reading from text books is recommended. A list of books to refer to is given at end of the module.

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COURSE CONTENT

- Definition of terms like pre-natal stage, infancy, development and growth.
- Nature versus nature controversy.
- History and significance of child development study.
- Theories of child growth and development.
- Pre –natal development and birthing process.
- Environmental influences on pre-natal development.
- Brain development and its function.
- Perceptual and physical development during infancy.
- Language development in infants.
- Social and emotional development during infancy.
- Moral and spiritual development.
- Cognitive development.

COURSE OBJECTIVES

a) Define terms like pre-natal stage, infancy, development and growth.

b) Discuss the nature versus nature controversy.

c) Explain the history and significance of child development study.

d) Discuss theories of child growth and development.

e) Describe pre –natal development and birthing process.

f) Explain environmental influences on pre-natal development.

g) Discuss brain development and its function.

h) Explain perceptual and physical development during infancy.

i) Describe language development in infants.

j) Discuss social and emotional development during infancy.

k) Explain moral and spiritual development.

l) Describe Cognitive development.

LESSON ONE: INTRODUCTION TO PRENATAL AND INFANCY DEVELOPMENT

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Objectives

By the end of this session, you should be able to:

a) ***Explain terms like prenatal stage, infancy, development and growth.***
b) ***Describe the nature versus nurture controversy.***

Contents of the module

This module is about prenatal and infant development. .

The terms ***growth*** and ***development*** are often used interchangeably. While they remain interrelated, the concept of growth and that of development differ essentially. These two terms represent two concepts that are characteristic of a human being from birth to death. .

Growth refers to the increases in physical size such as changes occurring in height and weight. Between birth and age one, the average baby grows by about 20 centimetres, increasing its overall length by some 40 percent. The infant gains about 7 kilograms, tripling its birth weight while the brain doubles in size. During infancy the facial proportions change from the foetal look of the newborn to the smooth roundedness characteristic of the new infant.

Growth also refers to changes in body proportions. If you have observed children growing you may have realized that different parts of the body grow at different rates, at different times, leading to changes in body proportions-. This is why the body proportion of an adult-is different from that of a newborn. For example the newborns head is relatively larger than the adults with respect to the rest of the body. The infant's legs and arms are also proportionately shorter than the other parts of the body when compared to an adult. Such changes in body proportion are a clear indication of growth.

Development

Development refers to change in the level of functioning. Human beings go through many types of change and some of these changes are developmental. Development can be defined as the gradual accumulation of relatively permanent age-related changes involving intensity and complexity of functioning.

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Development is gradual. It is not something that takes place right before our eyes. Rather it is observed or measured over a period of months or years. For example if you saw a two year old child everyday you are less, likely to notice developmental changes in him or her than a person who sees the child and does not see him or her again until after 3 years. Such a person is likely to comment, For example an infant's creeping movements combine with advances in strength and balance toward the end of the first year to produce walking.

The other characteristic of developmental change is that it is typically irreversible under ordinary circumstances. For example when a child learns to formulate sentences, he/she may not revert to using single word utterances.

Principles of Growth and Development

There are two principles of change: ***Continuity*** of change and ***discontinuity*** of change. Continuity principle emphasizes that development involves gradual, cumulative change from conception to death. A child's ability to walk, though seemingly on abrupt event, is the result of months of growth and practice.

scontinuity of development is the view that development involves distinct stages in the lifespan. In the development of an egg (ovum) into a baby the fertilized egg changes becoming more like a person – this development is continuous. Youand I cannot say at-which point in time the egg changed into a baby. The change is not drastic but gradual. The caterpillar on the other hand changes into a butterfly, a different kind of organism altogether. An example of such a change is the emergence of the ability to think abstractly about the world from not being able to do so. This is a qualitative, discontinuous developmental change.

The module addresses development in prenatal and infancy stages of a child's life. There are two stages of a child's life that are presented in this module.

The Prenatal period

The prenatal period refers to the period of time from conception to birth. It is a time of tremendous growth and development beginning from the moment the sperm cell from the male unites with the egg cell (ovum) from the female to form a single cell which develops in to a complete organism complete with a brain and behavioural capabilities in approximately 9 months (gestation period) at which time the period ends. At this point the organism is born. After birth the organism gets to the next stage, infancy.

Infancy

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Infancy is the developmental period that extends from birth to about 2 years. It is a time of extreme dependence on adults. Many changes take place in the child during this and later stages. These changes occur in different "arrears" or ***domains.***

Domains of change

These are the arrears in which growth and development occur. They include

Cognitive domain which includes thinking, understanding and language.

Physical domain which includes physical features like appearance, size and weight as well as motor abilities

Social emotional domain

Significant changes occur in all these arrears. .

Determinants of Growth and Development

Changes that occur in human beings do not take place in a vacuum. Rather they are the product of certain factors and processes. The question of what brings about these changes has been a subject of much debate. One evidence of this is a debate referred to as the ***nature- nurture controversy.***

Nature-Nurture Controversy

It is one of the oldest controversies in the history of philosophy and human department is the nature - nurture controversy. It is all about whether behaviour is primarily determined by biological or genetic factors within the organism (nature) or by experience in the external world (nurture). This issue can be traced to the arguments of ancient Greek philosophers like Plato and Aristotle. The debate continued on and off throughout the centuries. In the modern research, the nature side was presented by maturation theorists. The nurture side was a advocated by the behavioural theorists. By 1950's, it was realized that neither side could offer a complete explanation of human behaviour. Behaviour was found to be a product of the interaction between maturation and experience. This interaction is not a simple additive process. Hence we cannot easily determine the percentage contribution by each to the overall process of development. This interaction is complex with the contribution of each element continuously influencing and being influenced by the contributions of the other.

For example, many adults have had the experience of trying to calm a crying baby. You find that the more the baby cries, the more tense the person holding him/her becomes and the more tense the adult becomes, the more the baby cries. Can we determine how much of the infant's distress was caused by the adult and how much was caused by the

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baby? Certainly no. We can only conclude that both the adult and the baby continue to influence and to be influenced by each other. Both heredity and environment combine in such an interactive fashion to influence behaviour.

Definition of terms

Continuity of development: The view that development is gradual and cumulative.

Development: Changes in the level of functioning.

Discontinuity of development: the view that development involves distinct stages in the lifespan.

Domain: Arrears in which changes in growth and development Occur **Nature:**

Biological or genetic factors within the organism.

Nurture: Experience in the external world.

Growth: Changes in size and body proportions.

It also dealt with the issue of nature and nurture controversy.

References.

i) Bee, H. (1995) The growing child, Harper Collins Publishers new York.

ii) Black J.K & Puckett, M.B 1992.The young Child; Development from Pre birth through age eight

Activity

With reference to the infancy period of 1-2 years.

1. ***Write down major changes that are likely to occur from birth.***
2. ***What are the implications of these in later life of the child?***
3. ***What impact might it have in a child's education later?***
4. ***State ways in which heredity and environmental factors influences behaviour and development.***

Summary

The session addressed issues of growth and development.

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LESSON TWO: HISTORY AND SIGNIFICANCE OF CHILD DEVELOPMENT STUDY

Objectives

By the end of this session you should be able to:

a) ***Explain the importance of child development study.***
b) ***Describe the significance of child development to parents and teachers.***
c) ***By the end of this session you should be able to:***
d) ***Explain the importance of child development study***
e) ***Describe the significance of child development to parents and teachers***

Important of the study child development

There are many myths about child development for example you have heard parents and others comment on how today's children are growing up. For example today's kids have it all, if you do that, this will happen, a child is retarded because of a curse from God e.t.c.

Sometimes we believe some of these statements. However, research sometimes shows a different picture. It gives us a proven knowledge about children. The study of child development furthers our understanding about the nature of the child. It also helps us to understand the nature of the adult. The experiences of infancy and childhood contribute to adult personality and behaviour. Understanding the child helps us to understand who we are, why we are who we are and how we became that way.

Studying children can also lead to ways of enhancing their development and improving their life. For example, when we study the determinants of development, we are able to know how we can enhance it.

The study of child development can help you in a career e.g. as a teacher, caregiver, pediatrician or a judge who handles children or custody cases. Knowledge about children is a key to the success in any child-related field. Whatever your reason, you will find the study of children intriguing and full of useful information.

History of the study of Child Development

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Scholars and philosophers throughout all ages have suggested ways to rear and educate children. Their views continue to influence child psychology and education. The modern era of research began in the late 1880's. This is when the study of children began to take shape as a scientific endeavour. However, interest in the nature of children has ancient roots. Some of the ancient philosophers whose views have had a profound impact on child development include John Locke, Rousseau and Charles Darwin.

John Locke (1632-1704)

He was an English Philosopher. He saw a child as an incomplete adult; governed by strong urges and desires that need to be controlled. He believed that the environment played a crucial role in the child's growth and development. He advocated constant supervision in order to appropriately mould the child's development. He thus believed that people are capable of influencing the child.

At Locke's time, most scholars held that heredity largely determined the differences between human beings that is what we become is determined by our genetic make up. In contrast, Locke believed that education and experience were fundamental determinants of development. He proposed that at birth, the child's mind was a ***"tabula rasa"***, or blank slate ready to be written by the environment (Krogh, 1994). It is receptive to all kinds of learning hence guidance and education was essential during childhood.

The goal of education according to Locke was to enhance self discipline and self control Locke therefore advised parents and educators to teach self discipline as early as possible so children would learn to deny themselves the satisfaction of their own desires.

Jean - Jacques Rousseau (1912 - 1778)

He was a French philosopher. He wrote almost a century after Locke. According to Rousseau the environment was a critical determinant of development. He argued that children did not need strict supervision. Rousseau saw a child as a "noble being" that possesses intuitive knowledge of right and wrong but encounters pitfalls because of restrictions imposed by the society. Rousseau suggested that children be allowed to explore the world and use the environment to suit their needs and interests. In contrast to Lockes "tabula rasa" notion, Rousseau regarded children as having abilities and interests which change as they grow and as they learn about the world. The role of the adult is therefore to provide children with opportunities to explore and learn.

As we have seen, Rousseau and Locke held different views about the nature of the child and the role of education. These views brought about a fundamental division among researchers referred to as the nature - nurture controversy. That is, the debate about whether human traits are genetically endowed or wholly shaped by experience.

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Conclusion

Locke and Rousseau contributed to our understanding of children and encouraged the study of childhood although their ideas were based on speculation and philosophical beliefs rather than on scientific study.

Charles Darwin

He came up with a scientific approach to the study of children. His works on natural selection and evolution development of human beings sparked interest in the study of human beings. The publication of his "Origin of Species" (1859) is considered the most important force in establishing child study as a scientific endeavour. Darwin brought about the notion of the evolutionary process. He argued that human beings are not set apart from other species. Transmission of traits occurs through heredity. His writings sparked an interest in the study of any possible link between the human being and such species as apes and the biological make up of adults and children Darwin argued that just like the origin of man could be traced from that of apes, the origin of adults could also be traced from his origins, which are in the nature of the child. Scientists began to see children as interesting objects to study. This was the beginning of the study of child development as a scientific endeavour.

Early Observational Studies (Baby Biographies)

Observation was one of the first methods used to study children. Carefully recorded observations were compiled into day-to-day accounts of children's development known as baby biographies. Darwin in particular used this method to study his own son. Darwin son's baby biography was written in 1840-1841 and published in 1877. Several other baby biographies were written, compiled and published. They paved way for later discoveries in child development.

Stanley Hall and Early Experimental Studies

With time an interest in children as objects of scientific study grew. Child development studies became more objective and scientific. With time, researchers studied large groups of children. G. Stanley Hall (1846-1924) was one of the most notable scientists in child development study. He began experimental research on child development. Hall investigated the child's mind in order to understand the mind of the adult. He developed a tool called a "questionnaire" which is still in use today.

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Activity

a. ***Explain the significance of child growth and development***

b. ***State three importance of the study of child growth and development***

Summary

The session covered:

1. The importance of child development study.
2. The history of child development.

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LESSON THREE: THEORIES OF CHILD GROWTH AND DEVELOPMENT

Objectives

By the end of the session you should be able to:

a) ***Explain a number of child development theories.***
b) ***Differentiate the given theories.***
c) ***State the implications of these theories in the learning process.***

Introduction

Theories of learning and research on child development have inspired and helped us to understand better and appreciate infants' and young children's growth. They are based on patterns according to age, and some of their habits are formed at a tender age.

Piaget's Cognitive Theory

According to this theory, two complementary cognitive processes play a major role in promoting change and increasing children's cognitive understanding of their world. Children use their current knowledge of how the world works as a framework for the **assimilation** of new experiences. Assimilation is the attempt to fit new interaction into the existing cognitive structures.

They also modify their existing knowledge base by incorporating new information into its schemas or mental structures. Schemas are organized patterns of behaviour where a child incorporates new experiences from the environment. Through the process of **accommodation**, they modify these frameworks in response to these inputs from their environment. Accommodation is the changing of an existing structure to cope with new information.

Piaget identified the following stages of cognitive development.

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Sensory-motor Stage (birth – 2 years)

This period is characterized by the child's reflex activity when he is involved in systematic natural reflex activities to assist him in learning the environment. The environment is mainly maternal nipples in the first month after birth. Between one and four months, the child is involved in **circular reactions**; a form of self-exploration which is aimed at assisting him adapt to the environment. Circular reactions consist of the child's first stumbling upon some experience as a consequence of some act and secondly, trying to recapture the experience by re-enacting the original movements repeatedly in a kind of rhythmic circle. Circular reactions are good for intellectual development. There are three types of circular reactions; primary, secondary and tertiary.

- **Primary Circular Reactions:** In these reactions, a child performs repetitive acts like suckling thumbs, clapping hands or grabbing a foot. In this stage infants look only at objects that are directly in front of them.

- **Secondary Circular Reactions:** It deals with the exploration of the outside world where the child manipulates toys and objects in the environment. One major observation during the sensorymotor stage is that, the child has not acquired the concept of object permanence. At this stage, infants do not comprehend that objects have existence of their own. When a toy vanishes, they don't look for it. When a young infant does not perceive an object, he behaves as though it does not exist.

  At about four months of age, the infant shows some awareness of the permanence of objects. A child is moiré likely to search visually for an object if its loss is associated with interruption of their own movements than if another person has hidden it. The child will now anticipate the path of a moving object and look at the location where it can be expected to appear. She will search for a partially visible object, but not a covered one, and even if he/she watches as an object is being covered, he/she will not attempt to retrieve it. For example, if an adult hides a desired toy under a blanket, the infant will not search for it, even though he/she saw it being hidden.

  Most people find it difficult to conceive of a world in which objects that are not within their perception cease to exist. Even though we can no longer see, hear or touch an object, we generally assume that it continues to exist, that is, we accept the idea of object permanence. **Object permanence** is the belief that an object exists or is present at the place it was previously seen even if it is invisible to the child. Young children do not have a conception of the permanence of objects. For a very young child, when their mother goes out of the room or when a favourite toy drops over the edge of the bed, it is not only "out of sight" and "out of mind" but

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also "out of existence". Between 18 –24 months, the child has fully developed a full sense of object permanence. This is the result internalizing new experiences in the schema through secondary circular reactions and problem solving.

- **Tertiary Circular Reactions:** This stage ranges from 12 – 24 months. The infant's curiosity begins and novel stimuli are important. The infant experiments with the environment and is also systematic with trial and error. Problem solving begins and the infant begins to develop insight and can think without action. This is achieved by invention of new means through mental combinations.

Pre-operational Stage (2 – 7 years)

At this stage, the child has not developed logical or operational thinking although his language has developed.

Between 2 – 4 years, which is the pre-operational period, the most important characteristic is that the child's thinking is based on animistic thinking where inanimate objects are perceived as being alive and possessing human characteristics. Between 4 – 7 years which is referred to as intuitive stage, 2 characteristics emerge: conservation and reversibility, whose rules the child is unable to know.

In **conservation**; for instance, if the child is given 2 glasses with equal amounts of milk, then the milk from one glass is poured into a plate and he is asked to indicate which one contains more milk; the glass or the plate, the child will respond that the glass has more milk because the glass is taller than the plate. **Reversibility** is another area a child at intuitive level of pre-operational stage has difficulties with reversing actions. If the child is asked to determine whether the milk in the glass will remain the same amount when poured into a plate, he will say no.

Pre-operational children share with adults the ability to represent reality to themselves by means of signs and symbols. A young child's thought process is internal. Pre-operational thought is unsystematic, inconsistent, illogical and incorrect; for example, a child might reason that because a dog has four legs and horses too, then dogs are horses. The conclusion is illogical and incorrect. The pre-operational child exhibits **animism** in thinking. Because the child has life, thoughts and feelings, then the child sees all things as having life.

Pre-operational thought is perceptually bond. This means that children judge the world by the way it looks to them. For example, two halves of a cookie look more than a whole cookie; so the child prefers the halves to the whole.

SIGMUND FREUD'S PSYCHOSEXUAL THEORY

Particular interest was taken in personality development during the first 5 years of a child's life.

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Oral Stage (0 – 2 years)

This is the period young infant's needs are met through the gratification of its oral needs. His mouth is the source of pleasure; food, sensual sucking, gumming and biting. Feeding is their main source of pleasure as it stimulates the mouth, lips and tongue. Infants enjoy nipples, fingers and anything they can put into their mouths. At first, they suck and swallow, but when their teeth begin to erupt, biting and chewing become important. This stage has 2 sub-stages:

- Oral sucking stage (0 – 7 months)
- Oral biting stage (8 – 18 months)

Infants, whose oral needs are not met, may become fixated in the oral stage. Because they have not adequately resolved this stage, they will continue trying to meet their oral needs throughout life. Infants who receive so much gratification at the oral stage that they do not want to abandon this pleasure may also become fixated. They may become compulsive eaters, smokers, nail biters, tongue suckers, thumb suckers, over-dependent, and will chew a lot later in life.

Anal Stage (2 – 4 years)

This is pleasure in developing cleaning habits. The child is beginning to face the pleasures of reality and at the same time, the ego is beginning to differentiate itself from the id. The child's mode of expressing pleasure is experienced in emptying of the bowels which produces a great relief and pleasure. A battle between his will and that of his parents begins to occur. Parents begin to potty train their child. If methods of potty training and attitudes towards it are too strict and threatening, the child may retaliate by holding back faeces or by releasing them at inappropriate times. A fixated child may develop the following habits later: bed-wetting, butt-swinging, excessive cleanliness and orderliness or clumsiness. Sigmund Freud believed that personality consists of 3 stages: the id, the ego and the superego.

The id: This consists of instincts. It is unconscious and it represents the deep and the unacceptable part of personality that has no conscience of reality with the external world. It does not reason at all and so produces awkward behaviours. It is the most primitive behaviour and its aim is immediate gratification of issues or wishes. The id tries to satisfy wishes immediately they are encountered and it cannot tolerate discomforts brought about by tensions in our bodies. Its aim is to make sure that an individual is comfortable at all times. It is self-centred, seeks immediate pleasure, satisfaction and avoids pain.

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The ego: Freud came up with the second stage, ego, when he realized that the id could not explain behaviour satisfactorily. This is the conscious part that deals with the demands of reality. It makes rational decisions and abides by the principles of reality. The ego is more organized with rational system of personality, that is, it thinks. What makes the ego different form the id, is that the ego is reality-oriented whereas the id is pleasure-oriented.

The ego operates within the norms of the society. It controls reasoning, problem solving and decision making. It operates on reality principle where it delays gratification of a person's urge until a suitable method is found. The process of gratification will involve several things:

- *Perception.* This looks at the need in relation to the situation.
- *Learning.* Previous learning experiences are used to find out the realities of achieving objectives.
- *Memory and reality testing of the consequences of a particular urge.* It looks at the possible action and finds out its applicability in a given situation.

The ego operates by way of secondary processes which involve realistic thinking and also planning using higher mental processes. In most cases, the ego will completely satisfy the need. It will always delay gratification until an appropriate object is found. In addition, it will not consider social and moral values; instead, it will go for the object that will satisfy the urge.

Super-ego: This is the moral branch of personality which considers whether something is right or wrong. It is the highest mode of thinking and unlike the ego; it represents the ideals and values of the society. It does not only try to satisfy the need but it also considers ethics. The ideals and values of a society are learnt through socialization. Children will observe their parents and imitate their actions through direct teaching. These ideals are fostered through reinforcement, where desirable behaviour is rewarded and undesirable behaviour punished. As this occurs, the superego will register punished behaviour in a part called conscience and rewarded behaviour will be registered in a part called the ego ideal. As the behaviours are registered in respective parts of the super-ego, the conscience starts serving the purpose of punishing the individual by making him feel guilty or worthless, while the ego ideal serves the role of rewarding the individual by conveying a sense of pride or personal value.

These 3 components of personality i.e. the id, the ego and the super-ego work together and are tied to one another. Every time id tries to seek pleasure, the ego tries to test reality and the super-ego seeks perfection. These 3 structures of personality try to adjust to one another and it is this adjustment that results in human personality. The adjustment will differ from one person to another and that is why we have varying degrees of personality.

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Erik-Erikson's Psychosocial Theory

Erik believed that Freud's theory underestimated the influence of personality. His theory traces personality across a lifetime and according to him the person goes through a number of crises. During the crises, the society and culture have an important part to play. In each stage, there is a conflict, and if it is not resolved, the person struggles with the crises that affect the development of healthy ego, and resolution in each stage lays foundations for resolving crises of later stages. He emphasized that one's personality continues to develop even after childhood and throughout the entire life span.

According to Erikson, personality development is affected by the influences of the society and other events in the environment. He concluded that culture, society and historical events have events which are more important in the development of one's personality than biological development during childhood.
Successful resolutions involve balancing a negative trait with a positive trait in development of a virtue.

The following are some of the stages he identified:

- **Trust versus mistrust (birth to one year):** During this stage, the child is completely dependent on adults for survival. He learns to trust the environment if his bodily needs are met, has comfort, love and does not live in fear or apprehension about the future. However, if his needs are not met, then he learns to mistrust. With time, the mistrust is extended to other people in the environment. The mistrustful infant will also lack confidence in itself as a person worth of attention. In Erikson's theory, a healthy resolution of the trust versus mistrust conflict is represented by an infant learning to trust himself and others in the world. A certain amount of mistrust, however, is seen as healthy since the child has discovered that not everyone or everything responds in a predictable manner. Overall, the child's basic trust encourages continued interaction with people and surroundings.

- **Autonomy versus shame and doubt (1 – 3 years):** Autonomy means that the child is basically able to do something for himself/herself; for example, pulling or pushing objects, walking, dressing and going to the toilet among others. When a child is able to perform such tasks, he feels autonomous. Children are also able to carry out basic communication and to exercise their minds over the environmental issues. When a child is ridiculed or criticized excessively, he loses his confidence and develops shame and doubt.

Activity

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1. ***Explain what the parents and caregivers can do to ensure that infants develop a healthy development of attachment in Erik Erikson's stage of basic trust and Anatomy.***
2. ***Explain how behaviour can be conditioned using classical conditioning by Ivan Pavlov's theory, operant conditioning of B.F Skinner, and social learning theory of Albert Bandura.***
3. ***Describe the four stages of Piaget's stage of cognitive development.***
4. ***What is the implication of these stages to teaching and learning?***

Summary

This session addressed various theories of child development.

These include:

- Sigmund Freud's Psychoanalytic theory.
- Erik Erikson's Psychosocial theory.
- Charles Darwin's maturation theory
- Ivan Pavlov's classical condition

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LESSON FOUR: PRENATAL DEVELOPMENT AND BIRTH.

In this session, the biological beginnings of the human beings from conception to birth, will be examined

Conception

The first step in the development of a human being is that moment of conception when a single sperm cell from the male pierces the wall of the ovum (egg cell) from the female. Ordinarily a woman produces one ovum (egg cell) per month from one of her two ovaries. When a man ejaculates during sexual intercourse he deposits 300,000 - 500,000 sperm cells in the woman's vagina and only 1 will permeates the wall of the ovum. Others die off in the 3-4 day

Objectives.

By the end of the module you should be able to:

a) ***State importance of studying child growth and development.***
b) ***Explain theories of child development.***
c) ***Describe stages of development in prenatal and infancy periods.***
d) ***Explain various factors that influence development during prenatal and infancy stages***

journey through the acid fluid of the vagina and uterus to the fallopian tubes. With the union of sperm and egg cells, a new human being is formed. Assuming that both partners are fertile and that sperms are introduced into the

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woman's body at the optimal point in the woman's monthly cycle, the chances are excellent that the woman will become pregnant.

Prenatal Development

Approximately 266 days, or 38 weeks, elapse from the date of fertilization to the birth of the baby. This period of time is referred to as prenatal stage of development. Although growth and development during this time is highly continuous, scientists have divided prenatal development into three time periods: period of the zygote (or germinal period), period of the embryo, and period of the foetus. An alternative approach divides this time into three successive trimesters or three-month periods.

Zygote Period

The first two weeks following fertilization the period of the zygote. This is also referred to as the germinal period. During the first three days of this period, the zygote (fertilized egg) will remain in the fallopian tube. The original cell divides into two cells after 36 hours, followed by other several divisions. After 2 days, the second division to 4 cells occurs. Successive cell divisions occurs more rapidly. After 4 days there are about 60-70 cells. By 72 hours zygote has developed into a ball of cells, which is transported along the fallopian tube toward the uterus. The ball of cells will continue to enlarge through cell division as it approaches and enters the uterus. Once in the uterus, it will continue to grow while it floats for another three to four days.

By the beginning of the second week, the hollow, fluid-filled ball of cells is referred to as the **blastocyst**. Seven to nine days after fertilization, the blastocyst embeds itself into the lining of the uterus (endomentrium). This process is called implantation. Once the blastocyst is embedded in the endomentrium, *the placenta* rapidly begins to form. The placenta is an organ that sustains the foetus by allowing oxygen and nutrients from the mother's blood across a semi-permeable membrane to the foetus and by returning waste product from the foetus to the mother. The two blood supplies do not actually mix. The placenta is attached to the foetus by the umbilical cord, a tube containing two arteries and one vein that is pressurized to prevent tangling. After implantation, the period of the zygote ends and the embryonic period begins.

Embryo Period

By the fourteenth day, the growing ball of cells begins to differentiate into three distinct layers of cells, each of which is destined to form specific organ systems. The outermost layer, called the ***ectoderm,*** develops into the brain, the spinal cord, the nerves, and the skin. The intermediate layer, the ***mesoderm,*** forms the skeletal system, muscles,

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heart, and kidneys. The innermost layer, or ***endoderm,*** becomes the digestive tract, the respiratory system, pancreas, and liver. The developing organism is now referred to as an embryo.

The Period of the Embryo extends from the beginning of the third week to the end of the eighth week of pregnancy. During these critical six weeks, the structure of all the major organ systems (except the genitals) is established, a process known as ***organogenesis***. Each organ is formed at a particular time. The time when an organ is developing most rapidly is its ***critical period***. The heart, for example, begins to emerge in the middle of the third week and is structurally complete by the end of the eighth week.

By the end of the end of the embryonic period, the organism takes on a distinctly human form and many of its systems have begun to function. For example by the end of the fourth week, the ectoderm has folded over to form the neural tube (rudimentary brain/spinal cord), and a primitive heart begins to circulate blood through a crude but functional circulatory system. By the end of the eight week, the nervous system, heart, kidneys and liver have begun to function. All of this has been accomplished in an organism that is approximately 3 centimetres long and weighs about onefifth of a kilogram.

How does growth and development proceed?

Two principles have been used to describe physical development during the period of the embryo. First, the ***cephalocaudal*** principle describes a growth trend from head to toe, with growth of the head proceeding more rapidly than the lower parts of the body. This is reflected in the disproportionately large sized head early in this period.

The ***proximodistal*** principle describes a second trend in which development proceeds from the center (or proximal) regions of the body out toward the extremities (or distal parts). Thus, arm buds and leg buds appear early in this period, followed progressively by the upper arms and finally by the development of the forearm and lower legs. Webbed fingers and toes appear toward the sixth week. The embryonic period ends at the eighth week. The next period is the period of the foetus.

Period of the Foetus.

The final and longest stage of prenatal development, the period of the foetus, extends from the beginning of the ninth week to birth. The major organ systems continue to develop in size and function throughout this period. The size of the foetus is multiplied by a factor of 10. Body movements increase steadily in both intensity and variety between 10 and 16 weeks and by 16 weeks the mother usually begins to feel the ***quickening,*** the sensation of the foetus kicking against her abdomen. By 18 weeks, this movement slows as a result of advances in regulatory functions of the higher centers of the foetal brain.

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By 26 weeks, the foetus has reached the age of viability, the age at which it would have a reasonable chance of survival if born prematurely. Despite modern advances in medical care, survival with an earlier birth date is impossible because of the immaturity of the lungs and nervous system prior to week 24 or 25 as shown in the following table.

12 Weeks The foetus weighs about 1 ounce and measures about 3 inches. The head is still quite large in relation to the body. The eyes have formed but they remain fused shut. The ovaries and tests have formed within the body, but the external genitalia are still indistinguishable. The foetus is quite active in the roomy environment of the uterus: kicking,	**16 Weeks** At 4 ounces and 6 inches in length, the foetus now takes on a distinctly human form. The face has been formed with the eyes large in relation to other features such as the chin. The fingers and toes are completely separated and the foetus' sex can be determined by external features. The body is now covered with downy hair or lanugo. The mother may detect the first movements
clenching its fists, and making facial expressions	
20 weeks The foetus now weighs 8 ounces and is 10 inches long. All of the nerve cells that the individual will have throughout life are now formed. The foetus now shows many reflex reactions and can suck its thumb	**24 weeks** Now I pound and 13 inches long, the wrinkled skin is covered with *vernix,* a waxy coating that protects the skin in the liquid environment. The foetus reacts to loud sounds and music. The immature lungs, kidneys, and liver cannot yet sustain life out-side the womb. If born, some infants may survive but only with the best of intensive care.

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28 weeks At 2 pounds, fatty deposits beneath the skin begin to smooth out the skin. The foetus begins to produce sufactant, a substance that prevents the collapse of the lungs between breaths. Babies born prematurely have 60 - 70% chances of survival. Foetal movements can be seen through the mother's abdomen as the uterine environment becomes crowded. The eyes open for the first time and the foetus may hiccup as it swallows amniotic fluid.	**32 weeks** At 4 pounds, 3 ounces and 16 inches in length, the lungs produce enough surfactant to give the foetus an excellent chance of survival if born prematurely. The foetus's movements can be quite vigorous, sometimes causing the mother to gasp for breath. The brain develops deep folds in its surface.
36 weeks The lungs are now mature, and the kidneys and liver are beginning to process wasteproducts. Most foetuses (96%) will turn head down and many will "engage" the head into the mother's pelvic during the last prenatal month	

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LESSON FIVE: THE BIRTHING PROCESS

Objectives

By the end of this session you should be able to:-

i. ***What happens in the birthing process***
ii. ***Explain types of birth complications***

Introduction

For 9 months, heredity and environment factors have interacted to shape a new life. At 38 weeks the foetus is now ready to leave the comfort of the mother's womb to join others in its next world. By this time the foetus has a strong chance of survival. The question that many ask is how this transition finally takes place. Like many other reproductive issues, many children find it a tricky puzzle. Questions like: How is a child born? Does the baby need to bite his way out? or Does the doctor cut through mummies tummy to access the baby? are not uncommon. In this unit I will describe the process of birth and its implications.

By the end of this session you should be able to:

Explain the importance of child development study.

Describe the significance of child development to parents and teachers

What Happens To The Baby Just Before Birth?

During the last month of pregnancy, the head of the baby turns down toward the mother's pelvis and the uterus descends into the pelvic cavity. The ideal position for the baby's head is down and facing the mother's back. This position allows the baby to move most easily through the cervix and birth canal.

What Are The Signs Of Labor?

Labor is the process by which a woman gives birth to her baby. The onset of true labor is marked by the appearance of any one or combination of three signs: (1) Labor pains-these are rhythmic contractions of the uterus. These contractions are perceived to come from the lower back. They occur at regular intervals but as labor progresses the intervals shorten and the intensity of the contractions increases. (2) The Showthis is a small blood-spotted mucus plug that is discharged from the Cervical canal as a result of the dilation of the cervix and (3) Discharge of the amniotic fluid. This discharge occurs when the amniotic sac ruptures.

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When any or a combination of these signs occur the mother is said to be in labor, a process which occurs in three stages. Let us look at each of these stages now.

Birth Process Model - Anatomical Model

Stages of Labour:

The process of labour unfolds in a series of three well-defined stages.

The First Stage: Dilation:

This stage begins spontaneously approximately 266 days after conception with a series of contractions that dilate or open the cervix to allow the baby to pass through. The muscular walls of the uterus contract pushing the baby against the cervix. The first contractions are relatively short and painless. However, they gradually become longer, stronger and more frequent. As the cervix approaches its widest aperture, the pain can reach extraordinary levels of intensity. Complete dilation to 10 centimetres takes between 2 to 20 hours for the first pregnancy and less for later pregnancies. The first stage is divided into three phases.

The early phase lasts until dilation reaches 4 centimetres, roughly the width of two fingers. The amniotic sac (bag of waters) may break, releasing a trickle or possibly a gush of liquid through the vagina. The active phase of labor lasts from two to six hours for first-time mothers. The transition phase is marked by the dilation of the cervix from 8 to 10 centimetres. This phase typically lasts about one hour. Bee (1995) likens this stage labor to putting on a sweater with a neck that is too tight. You have to pull and stretch the neck of the sweater with your head in order to get it on. Eventually the neck is stretched wide enough so that the widest part of your head can pass through. When dilation is complete, the baby" head moves through the cervix and begins to descend into the vagina, or birth canal. Contractions have increased to maximum intensity, lasting about two minutes each and occurring in rapid succession, often without a lapse between them. In a conventional hospital delivery, the mother is now moved into the delivery bed.

The Second Stage: Descent and Birth:

At the end of the transition phase, the mother will normally have the urge to push the baby out. The second stage of labor begins when the cervix is fully dilated and ends when the baby is born. The baby descends down the birth canal. The descent of the baby can take from a few minutes up to three hours. This depends on the relative size of the baby, the resistance of the vagina and the strength of the contractions. The descent is gradual and intermittent. When the top of the baby s head appears through the vaginal opening, the opening of the vagina is stretched to its limit Many women feel more clearheaded and have a renewed sense of optimism when pushing begins since the

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baby is almost born. Just before the baby is born a mother may feel a burning, stinging, stretching sensation at the vaginal opening- a sure sign that you're almost there! As the head crowns, mother relaxes and the head moves out through the vaginal opening. As the baby s head emerges it turns to one side to allow the shoulders to align, then with next push shoulder's and the rest of the baby's body slips out. The birth attendant may assist in turning foetus to facilitate this process.

If the opening is not adequate to accommodate the size of the head, the physician may administer a local anaesthetic and perform an ***episiotomy.*** This is a surgical incision that widens the opening to allow the head to pass. The episiotomy reduces the risk to the baby and prevents injury that would result from a tea of the skin between the vagina and the anus. The last few contractions expel baby out of the mother's body. The baby takes the first breath and the umbilical cord is clamped and severed.

The Third Stage:

The third stage of labour involves the expulsion of the placenta and umbilical cord or afterbirth, through the cervix. There are mild contractions that expel the placenta and the now-useless foetal membranes out of the mother's body.

This stage consists of the expulsion of the placenta. It generally takes 5 to 10 minutes and the stage also includes after birth recovery of the mother.

Should the placenta not easily come out, tugging or pulling should not be performed. Gentle uterine massage may be utilized to assist in the release The placenta should always be examined to be sure no parts remain within the uterus This can become detrimental to the mother causing haemorrhage and/or death.

During the after birth recovery phase, a mother is monitored to be sure no uterine bleeding or other complications occur.

Birth Complications

The labor and birth process may be complicated by lack of sufficient power or by inappropriate presentation of the baby

Delivery Presentations

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Delivery presentation refers to the orientation or the position of the baby as it presents itself for the birth process. It is often called by different names including "foetal attitude' and "foetal lie". The preferred foetal delivery presentation is "cephalic presentation. This is a head down presentation with the topmost part of the head emerging first and the face downward in the direction of mothers back and back of head upward in the direction of the mothers' front.

If the child is not in the face down head down position, the delivery presentation is referred to as a **malpresentation.** There are various types of mal-presentations:

- **Upside Down Presentation:** This is a presentation where the baby presents in an "upside down" or "sunny side up" position. If the baby does not turn to the proper position or turns late, the newborn will generally develop "cone head". This position also produces what is known as "back labour". This can be significantly reduced in women who receive regular chiropractic care during their pregnancy. The upside down presentation occurs in approximately 13% of births.
- **Breech Presentation**: In a breech presentation, the feet or buttocks present first as opposed to the head. This presentation occurs in about 1 out of every 40 births. The possible complications of this presentation can be serious and include:

  i. intracramal bleeding ii. neck dislocation iii. shoulder dislocation iv. hip dislocation
  v. clavicle fracture vi. internal organ disruption vii. premature placental rupture
  viii. prolapsed cord ix. uterine rupture

- **Face Presentation:** In this presentation, the baby presents face first with the neck in extension. Causes this kind of presentation includes a lax uterus, flat pelvis, multiple foetus, or- neck spasms of the foetus. This is stressful on the cervical spine. Chiropractic care by a chiropractor trained in adjusting newborns is crucial for the continued proper growth of the spine. Face presentations occur approximately every 3,000 births.

- **Shoulder Presentation:** The shoulder presentation is one where the shoulder emerges first. This delivery presentation occurs in every 200-300 births.

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- **Transverse Presentation:** The tail area is presented first with legs and head at opposite sides, Efforts are usually made to turn the baby in utero through massage, exercise or position of mother. If the baby's position does not change a caesarean operation maybe needed to safely remove the baby.

Insufficient Power

Sufficient power and coordinated contractions are essential for a smooth uncomplicated labour. When the contractions are weak or the pattern of contractions disorganized, the mother is more likely to become exhausted. This can cause foetal distress resulting in foetal harm. A caesarean section may be necessary in such a situation.

Passage Obstruction

Passage way obstructions in the uterus, pelvis, or cervix can cause serious complications in the birthing process. Causes of such complications may include:

i. tumours ii. cysts iii.
fractures iv. flat male-like
pelvis (android)
v. physiological changes due to degenerative joint disease, tuberculosis, rickets or osteomalacia.

Dealing with Labour Complications:

The natural process of labour is subject to a number of problems, some of which place the mother and/or the infant at risk. When serious problems arise, medical mterventions are necessary to reduce the risk to mother and infant. Commonly used procedures include inducing and speeding up labour, assistance through mechanical means and caesarean delivery.

Inducing and Speeding Up Labour .

A variety of medical circumstances may recommend that labour be initiated artificially in a process called induction. If the pregnancy is at or beyond the forty-second week, induction is recommended. If the amniotic sac has not yet ruptured, the physician can • induce labour by purposely breaking the ammotic sac. This procedure is called the ***Artificial Rupture of Membranes*** (AROM). If AROM fails, a hormone can be given intravenously to induce or speed up labour. The dose can be varied to regulate the rate and intensity of contractions throughout labour. If it is medically advisable tor the labour to be induced and the procedure fails, a caesarean delivery is the only reasonable alternative.

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Artificially induced labour can cause very strong and painful contractions, thereby increasing the need for pain medication. The equipment necessary to administer the drug restricts the mother's movement, adding to her discomfort.

Assisting Delivery through Mechanical Means

In some instances, after the baby's head has passed into the vagina, the contractions weaken and the baby stops or dramatically slows its descent. If procedures to induce or speed up labour have failed or are inappropriate, one remaining option is to assist the descent by mechanical means. The first procedure involves the use of forceps, a tong like instrument that is inserted into the vagina around the baby's head. The physician pulls if the mother pushes with each contraction. Forceps typically require the use of anaesthesia and an episiotomy. The forceps introduce the risk of injury to the mother and the baby.

A second mechanical procedure for assisting the descent of the baby is vacuum extraction 4 plastic suction cup, connected by a tube to a vacuum device, is placed on the top of the baby's head. Handles on the tube allow the physician to pull on the head as the mother pushes with each contraction. Compared to the use of forceps, vacuum extraction is less likely to damage the vagina, and can be applied higher in the birth canal.

Caesarean Delivery

If the baby or mother is thought to be at risk for instance if there is foetal stress and other techniques are either unsuccessful or inappropriate, the baby may be delivered surgically by caesarean section. Another common reason for doing a caesarean delivery is breech birth This complication occurs when a part of the body usually the buttocks, feet, or umbilical cord- other than the head is positioned to emerge first from the cervix. It the mother's pelvic bone is too small to accommodate the birth of a large baby. A caesarean delivery may also be performed. In this procedure, anaesthesia is administered. An incision is made in the abdomen and uterus and the baby is removed.

Even when a caesarean is clearly indicated, there are risks. This procedure is considered a major surgery and it prolongs the recovery of the mother. There is a higher rate of postpatum infection and a prolonged period of healing in the mother. It is important to use it only when necessary for the well being of the mother and the baby.

Summary: In this unit we have looked at the process of birth, we have seen that labour progresses through three stages. The first stage involves contractions of the uterus which cause the cervix to dilate until the baby moves out of the cervix. In the second stage, further uterine contractions push the baby down the vagina until out of the mother's

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body. The placenta and the umbilical cords are expelled out of the mother's body by mild contractions a few minutes after the baby is born. We have also looked at some mechanical interventions that become sometimes necessary to reduce the risks of the baby and the mother.

Definition of terms

Anoxia: Prolonged period of oxygen deprivation to the foetus.

Caesarean delivery: Birth of the baby through an incision through the abdomen and uterus

Episiotomy: A surgical incision that widens the vaginal opening to allow the foetus to

Activity

1. ***Discuss the implications of home births***
2. ***What leads to birth complication?***
3. ***What impact does complication in delivery have on a child?***

Summary

The session covered the three stages of birthing until the baby is born, it also highlighted different types of delivery and mechanical intervention necessary to reduce the risk of mother and child

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LESSON SIX: ENVIRONMENTAL INFLUENCES ON PRENATAL DEVELOPMENT.

Introduction

For a long time, it was commonly believed that the uteais and the tissues surrounding the embryo and foetus offered more than adequate protection to prenatal development. It was believed that the placenta provided an impenetrable barrier to the invasion of harmful substances. (Krantz, 1994). It is now known that some diseases, drugs, and other potentially harmful chemicals can be transmitted from the mother to the embryo. At best, the placenta may play a role in slowing down the transfer of some harmful substances and slightly reducing their concentration. However the organism is still vulnerable to environment influences.

Teratogens

As earlier discussed, the placenta provides a semi permeable membrane for the exchange of substances between mother and the embryo foetus. As such, it can restrict some substances in the maternal blood supply (for instance, larger proteins and the mother's blood cells) from entering the blood supply of the embryo. This has some benefits: The mother's antibodies, which protect against disease, are transmitted to the embryo and provide some degree of immunity to certain forms of infection. However, many other substances - including chemical agents, viruses and bacteria are capable of crossing the placenta and entering the embryo or foetus. Agents that enter into the biological system of the embryo or foetus and negatively influence development are called teratogens. This term comes from the Greek word *"teras"* meaning monster. The key elements determining the effects of teratogens are dosage, timing in relation to^ organ development, the embryos own resilience, and protective factors in the mother. There are various teratogenic agents: Let us go through the main teratogenic agents and have a look at their likely effects on the developing organism.

Effects of drugs and medication on the developing foetus

Although medication is at times essential for the pregnant mother, it is true that some drugs do affect the development of the foetus. They are teratogenic. The teratogenic effects differ from one drug to the other and depend on the time they are taken. This sometimes causes damage on the foetus leading to deformed babies.

Drugs effects on prenatal development

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Some pregnant mothers take drugs and medication without thinking about the possible effects on the foetus. Some smoke tobacco and bhang and other hard drugs or drink alcohol while others buy curative drugs over-the- counter without consulting a physician.

The most harmful medication

Thalidomide is the most harmful drug ever known. It is a tranquilizer that has the ability to alleviate morning sickness suffered by many mothers in their early stages of pregnancy.

Effects of thalidomide

In 1961, some 7,000 babies were born with severe physical deformities, most notably small, flipper like appendages in place of arms or legs. The birth defects were traced to the mother's use of the over-thecounter sedative thalidomide (Lenz, 1966). Although thalidomide was apparently highly effective in reducing a pregnant mother's feelings of nausea, its effects on the embryo were devastating. The effects depended on when it was taken. If the ears were in their critical period, the baby was born without ears. If the alimentary tract was in its critical period, severe deformity resulted in various locations along the tract. Defects of the heart, liver, and kidneys were also common (Krantz, 1994). Thalidomide is no longer available for pregnant women.

Are there other teratogenic medication?

Yes, there are other medications that may cause harm to the developing embryo and foetus. A good example is the synthetic sex hormones (oestrogen and progestin) commonly used in birth control pills. The pill works by suppressing ovulation and by artificially creating cervical hostility — but it is not always successful. In cases where a woman is unaware of the pill's failure, she will continue using the pill through the early weeks of her pregnancy, exposing the embryo to abnormally high levels of sex hormones. This may result in abnormalities of the baby's genitalia.

Synthetic hormones have also been prescribed to correct hormone imbalances that might otherwise result in miscarriage. Between 1948 and l969, for instance, the synthetic oestrogen diethylstilbestrol (DES) was routinely prescribed to prevent miscarriages in up to 2 million pregnant women. Babies born to the DES mothers appeared normal at birth but at adolescence, some girls developed a rare form of vaginal cancer. Approximately 59 percent show abnormalities of vaginal tissue and/or the cervix (Elliott, 1979). Moreover, when DES babies grow to adulthood and become pregnant, they are more likely to miscarry than other women. Male offspring, although less severely affected by DES, have been shown to have genital abnormalities and an increased risk of testicular cancer (Stillman, 1982). The DES example illustrates that some of the negative effects of apparently safe drugs introduced today may

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not show up for many years. We know almost nothing about possible long-term teratogenic effects of thousands of drugs that are commonly prescribed by physicians and many others sold over-the-counter. For some potential teratogenic medications, prescriptions are accompanied by appropriate warnings against use by pregnant women. Despite these precautions, however, there are many, instances, where mothers may take the medications because they are unaware they are pregnant.

In our society adults take medication for virtually every ailment, discomfort or irregularity. Many women continue this behaviour into their pregnancy, placing the embryo and foetus at risk.

Other teratogenic drugs

These include cigarettes, alcohol, narcotics and other street drugs.

Cigarettes effect on the developing foetus

Women who smoke cigarettes continue to smoke throughout their pregnancy. These mothers place their babies at a high risk for this indulgence. Babies born of smoking mothers have been found to have low birth weight. Smoking during pregnancy is also associated with a significant increase in the rate of miscarriages, stillbirths and death of babies soon after birth (Naeye, 1978). Foetal oxygen deprivation is associated with abnormalities of both the brain and the heart. The damage to the foetus is directly correlated with the number of cigarettes smoked per day. It also seems to be associated with the period of pregnancy when the mother smokes. Recent research shows that cigarette smoking can lead to asthmatic babies.

Alcohol affect the developing foetus

Women who drink heavily during pregnancy have a high risk of giving birth to a baby with ***Foetal Alcohol Syndrome*** (FAS). FAS is characterized by mental retardation as well as a combination of deformities of the face, brain, heart, genitals, bones and joints, and growth retardation. The head may be abnormally small with inadequate development of the brain. Alcohol ranks third after Down syndrome and fragile X chromosome abnormality as a cause of mental retardation (Milunsky, 1989). FAS baby usually have widely spaced eyes, a flat nose, and underdeveloped jaws. The period of greatest vulnerability to alcohol is in the first trimester (the first three months of pregnancy).The critical period is when the heads is forming, during the third and fourth weeks of gestation. At this stage the mother is unlikely to know that she is pregnant and is therefore unlikely to take precautions, particularly if she is an alcoholic.

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Prenatal growth retardation and damage to the foetal brain have enduring effects on a child. The child's growth remains stunted. Hyperactivity, distractibility, short attention span, and delay in perceptual and emotional development persist into childhood for children of alcoholic mothers (Krantz, 1994).There is no safe level of alcohol consumption for pregnant women. Even moderate use of alcohol does place the embryo and foetus at risk.

What are the effects of narcotics and other street drugs?

When a pregnant mother takes a psychoactive drug, the drug crosses the placenta and enters the bloodstream of the foetus within minutes. The foetus cannot metabolize drugs as efficiently as the mother can, so the effects of drugs stay with the foetus long after they have worn off the mother.

When a mother who is addicted to **heroin** or **methadone** (a potent narcotic used in drug rehabilitation) gives birth, her baby too will be addicted to the drug and will soon experience withdrawal. The withdrawal symptoms are severe, including tremors, vomiting, and fever. The newborn may die if not given the drug immediately. The addiction can be eliminated by gradual reduction of the drug. (Krantz, 1994)

Infants of narcotic-addicted mothers are very likely to be born preterm and at low-birth weight. Addicted newborns are highly-irritable-and engage in a characteristic high-pitched cry that cannot be soothed, even by the mother. Although the infant can be treated successfully for the addiction, this highly irritable behaviour pattern typically presents an overwhelming challenge, especially for a mother who is still addicted and is likely to have limited coping strategies. Consequently, such mothers and their children remain in an intense state of risk.

Cocaine poses greater threat than heroin. Cocaine is a highly addictive drug with pervasive teratogenic effects. Its widespread use by young women in some societies greatly expands its risk level.

Research has shown that pregnant mothers who use cocaine significantly increase the likelihood of spontaneous abortion and stillbirth. Babies who do survive are likely to be born preterm, at lower birth weight, and with abnormally small head circumference and other malformations. Cocaine-exposed newborns are irritable, easily over stimulated, and disorganized in their interaction with caregivers (Krantz, 1994)

A mother who stops using cocaine early in her pregnancy substantially reduces her baby's risk of preterm delivery and low birth weight. Cocaine causes a reduced head circumference, an indication of inadequate brain development. Cocaine-exposed children continue to have learning and cognitive difficulties into the preschool years.

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While prenatal exposure to cocaine has multiple negative effects on children's development, the problem is often exaggerated by the mother's continued involvement in a drug-related lifestyle after the child is born. Cocaineaddicted mothers are likely to physically and emotional neglect their infants and preschool children. Addicted mothers are likely to be chaotic and unable to provide for their children's needs. In comparison to non-addicted mothers, their children are likely to develop more slowly. They are less likely to succeed. Thus, children who are born to cocaine-addicted mothers will be exposed to multiple forms of risk to their development.

A woman is more likely to have sex, less likely to use birth control, and thus more likely to become pregnant when she is high on alcohol or drugs. Before she even knows that she is pregnant, she will be bathing her baby in these substances. They enter the baby's bloodstream just a few minutes after they enter the mother's. So when she gets high, the baby gets high. The baby's liver and kidneys are immature and far less efficient than the mother's organs are at breaking down the chemicals and cleansing the bloodstream. So the baby suffers more and is in much greater peril than the mother is.

Diseases harmful to Expectant Mothers

Not all diseases suffered by a pregnant mother are harmful to the foetus. However there are several common diseases that have teratogenic effects. Let us have a look at some of these diseases now.

Rubella

Rubella or German measles is an infection that causes mild cold symptoms and a rash in adults. If a mother contracts rubella during the first trimester of her pregnancy, the foetus is likely to be affected. ***Foetal rubella syndrome*** mayresult in spontaneous abortion or stillbirth; if the foetus survives, its problems may include mental retardation as well as deafness or malformation of the eyes, ears, and/or heart, depending on the period of pregnancy during which the disease is contracted. The rate of malformation is approximately 50 percent for mothers infected during the first trimester. There are reduced likelihood of effects from second-and third-trimester illness. Approximately 10 to 20 percent of infected infants die before age one (Bergsma, 1979).

Women who intend to become pregnant should have their immune status checked and updated at least six months before trying to conceive. **Herpes**

Two forms of herpes virus are epidemic. Both forms have severe effects on the foetus.

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Herpes simplex 2 (HSV) is a very common sexually transmitted disease that causes painful genital sores in adults. HSV is transmissible only when sores are present. If the mother has HSV, the foetus is most likely to contract it at delivery through exposure to maternal secretions. There is a 50-percent infection rate for vaginal deliveries. Although infection at the time of birth can often be avoided by Caesarean Section, there are possibilities that the foetus can be infected before birth. Regardless of the mode infection, the effects on the foetus are catastrophic. Most infected babies die within the first few months of life.

If a mother has been infected with herpes at any time in her life, she is still infected, even if she shows no symptom. The following precautions must be taken to avoid damaging the baby:

- If she is not yet pregnant but is sexually active, she should keep in mind that the only effective drug for the control of herpes (acyclovir) is a suspected teratogen. She should discuss this with a physician.
- When the mother becomes pregnant, she should inform her obstetrician immediately. Her pregnancy must be monitored closely. A caesarean delivery will be required to prevent possible infection of the baby during vaginal delivery.

A second common form of herpes vims, Cytomegalovitus or CMV is a highly, infectious disease with symptoms resembling a mild cold Unfortunately, the virus is not always accompanied by symptoms and CMV can hide in the mother's system for some time and affect the foetus when the mother becomes pregnant. Blood tests are available to determine the mother's vulnerability to the disease Since no vaccine or cure is presently available for CMV pregnant women-particularly those who have daily contact with young children should avoid exposure to others with the disease.

Acquired Immunodeficiency Syndrome (AIDS)

AIDS is a progressive weakening of the immune system by the Human Immunodeficiency Virus (HIV). In infected adults, the weakened immune system creates extreme vulnerability to infection and certain types of cancer. Children get AIDS in a variety of ways but the primary mode of transmission is from an infected mother to the foetus in uterus or at birth. A few newborns have been infected from their mothers' breast milk, and about 17 percent of known cases children resulted from receiving blood or blood products.

Infected infants suffer from frequent infection and diarrhea. They fail to thrive or gain weight. Brain damage is typical. In contrast to adults susceptibility to opportunistic infections - including cancer and pneumonia- infants fall prey to common bacterial infections. The prognosis for these infants is almost certain death within a few months.

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There is no cure for AIDS at any age.

The only way to prevent transmission of HIV to the foetus is for infected women to avoid pregnancy. The risk of HIV - positive mother infecting her foetus is approximately 30 to 40 percent. There is simply no safe way for an infected woman to bear a child. If she gives birth and her baby does not test positive, breast-feeding should be avoided. The virus can be transmitted to the baby by breast milk.

Syphilis

Syphilis is a common sexually transmitted disease. If contacted in the first months of pregnancy, the organisms that produce this disease cannot cross the placenta, so syphilis can be cured with antibiotics before the foetus is affected. However later in the pregnancy, the foetus is susceptible to bone, liver and brain damage or even death.

Maternal diabetes

This disease involves higher than normal amounts of blood sugar in the mother's blood. It causes babies to be large and mature looking at birth, but they are immature in their functioning.

Toxemia

This is a condition caused by toxic substances in the blood, causing swelling of the mother's limbs, sometimes malfunctioning of the kidneys and circulation systems. Toxemia causes cerebral palsy, epilepsy, mental retardation, and reading disability. The cause of this disease is unknown but it is thought to be due to malnutrition. If untreated toxemia may lead to a mother's death and can even lead to brain damage in the foetus due to lack of oxygen supply to the brain

Are There other Teratogenic Maternal Conditions?

Yes there are several other conditions of the pregnant mother that have been found to affect the developing foetus. These include genetic factors (Rhesus factor), maternal stress, nutrition and exercises during pregnancy, mother's emotions and the mother's age.

What Is Rhesus (Rh) Factor and what is its Effect?

Incompatibility between mother's blood and the infant's is another hazardous problem. If the mother has Rh-negative blood and the baby has inherited Rh-positive blood from the father, a reaction can occur in the mother and eventually in the foetus leading to miscarriage, or the baby may be born mentally retarded. Antibodies build up in the mother's

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blood stream and these antibodies fight the foetus as if it were foreign to the mother's system. This has a toxic effect on the foetus.

The first baby is not affected but subsequent babies can be damaged. A physician who detects the Rhesus- negative blood characteristic can prevent subsequent problems through injections of Rh immune globulin.

Maternal Stress and emotions During Pregnancy

Several factors can cause stress in the pregnant mother, for instance, mental conflicts, economic pressures, illness etc. Stress increases heart rate, blood pressure, respiration, and muscle tension. (Krantz, 1994). Mother's emotions also affect the foetus. A high a mount of psychological stress can affect the foetus leading to premature birth, prolonged pregnancy or restlessness in the newborn.

There are four compelling reasons to expect that maternal stress would have a negative impact on the foetus and/or on the birth process. Emotional reactions like extreme stress result in increase in the level of adrenaline and other hormones in the bloodstream. These substances pass through the placenta and affect the foetus.
Stress hormones released into the bloodstream also increase blood flow to the muscles and brain and decrease blood flow to the uterus, reducing the supply of oxygen to the foetus. Stress can also have negative effects on the mother's approach to prenatal care, diet, and exercise. These in turn affect the foetus.

Stress can promote substance abuse. Stress may result in the mother's increased use of alcohol, drugs, and cigarettes, all of which have been shown to harm the foetus.

Stress may also affect mother's feeding habits leading to a low birth weight infant.

Maternal Nutrition

Mother's nutrition and diet play a significant role in the growth and development of her foetus. The mother needs adequate diet during pregnancy to support the foetus developing body. Malnourished mothers give birth to premature or low-birth weight babies.

Malnutrition also affects brain development. Lack of sufficient vitamins in mother's diet also affects the development of the baby's nervous system multivitamins and neural tube defects. A recent landmark study (Mulusry et al., 1989) discovered that mothers who take Multivitamins during the early stages of pregnancy sharply reduce the risk of

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having a baby with neural tube defects. Such defects, occurring at about the sixth week after conception can cause paralysis and even death.

It is a common belief that pregnant mothers should eat more food than they do normally. Although there is some truth in this, it does not mean that she should simply double her normal intake of food. Their nutritional needs change. For example, although the mother's energy requirements increase by only 15 percent, her need for protein increases 60 percent and her need for iron may increase up to 300 percent. Individual women differ, so each woman should consult with a certified dietician as an essential aspect of prenatal care

Unfortunately, many low-income mothers cannot afford to maintain adequate nutrition during their pregnancy, increasing the likelihood of low birth weight babies.

Exercise during Pregnancy:

Many women who are concerned about physical fitness ask whether they can continue to exercise during their pregnancy. What effects does the mother's exercise have on the pregnancy and on the foetus? There has been very little research on the effects of exercise on pregnancy. In on study, researchers examined the effects of vigorous bicycle peddling on foetal heart rate in the second trimester. Foetal heart rate declined only when mothers exhausted themselves; it then returned to normal within a half hour after the exercise was terminated (Carpenter et al., 1988). More research is needed on this important aspect of prenatal care.

The limited research available suggests that physically fit women in low-risk pregnancies may continue moderate exercise into the first six months, but only under the guidance of a physician. However, pregnancy is not the time for the pregnant woman to begin to get in shape, nor the time for her to take on new challenges even if she is already in good shape.

Mother's Age:

Mortality rates and incidences of retardation are higher in children born to mothers under 20. This can be explained by social, economical and physical factors. Most teenage mothers are not socially and economically stable hence they are likely not to feed well and afford other facilities necessary for proper development in pregnancy. Physically, the young mother is not adequately developed and this may introduce problems during birth process.

Women who delay childbearing until they are over 35 years have higher risks of premature, low-birth weight babies, and stillbirths. They have a higher risk of contacting diseases such as high blood pressure and rapture of placenta.

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Older women also have higher risks of giving birth to children suffering from a genetic disease called Down's syndrome (mongolism).

What about Environmental Hazards?

Massive doses of radiation from the atmosphere cause congenital problems. Evidence of this comes from the results of the atom-bomb explosion in Hiroshima and Nagasaki in 1945. All serving pregnant mothers within a mile of the centre of explosion had still births.75 percent of pregnant women within 1-4 miles had spontaneous abortions, still born babies and severely handicapped infants.

Use of x-rays (especially during the first trimester) increases the risk of Leukemia during childhood.

Another form of environmental hazard is pollution. Exposure to several pollutants can be teratogenic. For example carbon monoxide, lead and mercury can lead to low birth weight and slow neurological development. Carbon monoxide can easily lead to death.

Maternal Prenatal Care:

It is important that the expectant mother seek prenatal care as soon as she suspects she is pregnant. Signs of pregnancy include one or several of the following symptoms: a missed period, a need for more rest, nausea and swollen sensitive breasts. As mentioned earlier, the first eight weeks of pregnancy are critical to the developing foetus, since this is the time that all major organ systems develop. Early detection of problems is important in ensuring the development of a healthy baby.

During prenatal visits examination by the medical personnel provide basic information about the overall health of the mother and the foetus. Blood test are taken to determine if the mother is anaemic, has diseases such as syphilis, or has had rubella or if there could be Rh blood incompatibility. Questions are asked about the mother's and father's past medical history, family history, and personal health habits.

Expectant mothers are usually counseled about the importance of proper diet and the things they need to avoid. It is important for the mothers to have constant medical checkups at regular intervals throughout pregnancy.

Conclusion.

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After the long journey of prenatal development, birth has to take place. Once the 38 weeks of gestation are over, the foetus is ready to be born, an event that holds some anxiety as well as a lot of joy for most parents. I will look at this event in the next unit.

Definition of Terms

Anoxia :	Refers to a condition that is caused by the lack of oxygen in the brain of infant during labor and delivery and can cause brain damage.
Cephalocaudal:	A pattern of development that proceeds from head to tail.
Critical period:	The time when an organ is developing most rapidly.
Ectoderm:	The outermost cells of the embryo which develops into the brain, the spinal cord, the nerves, and the skin.
Endoderm:	The innermost layer of cells in the embryo which becomes the digestive tract, the respiratory system, pancreas and liver
Mesoderm:	The intermediate layer of cells in the embryo which forms the skeletal systems muscles heart and kidneys.

Activity

1. ***What are teratogens?***
2. ***Explain their impact on unborn baby?***
3. ***What can mothers do to reduce their effects on unborn child?***

Summary

The session covered human development from conception to birth it involved the development from zygote to an embryo until foetus stage and the developmental stages until the baby is born. The factors influencing prenatal development were discussed

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LESSON SEVEN: BRAIN DEVELOPMENT AND ITS FUNCTIONS

Objectives

By the end of the session you should be able to:

a) ***Explain parts of the brain and their functions.***
b) ***Explain the process of brain development.***
c) ***Analyze parental and caregivers contribution towards brain development.***

A newborn's brain is about 25% of its approximate adult weight. By the age of 3, it has grown dramatically by producing billions of cells and hundreds of trillions of synapses between these cells. Much of the increase occurs before the age of 2 years at which time the brain weighs 75% of adult's weight. During the prenatal period and early infancy, the number of brain cells increase rapidly. By the time the child is five years old, the brain has attained 90% of adult's weight and by age 10, it is 95% of adult's weight.

Parts of the Brain and Their Functions

- **External structures:** They are visible by mere inspection of the brain. They include the cerebellum, brain lobes and hemispheres, cranial nerves, midbrain, medulla, spinal cord, olfactory bulbs and tract.

- **Internal regions of the brain:** They are viewed in a cross-section. They include the forebrain (telecephalon) diencephalons, which includes the hypothalamus, pituitary, and thalamus among others. Other parts of the brain include the midbrain (mesencephalon) which consists of basic brain nuclei fibre tracts and neurons.

- **Medulla:** This is an oblong-shaped area of the hindbrain involved in heartbeat and respiration. The medulla regulates vital functions such as heart rate, blood pressure and respiration. The medulla also plays a role in sleep, sneezing and coughing.

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- **Cerebellum:** This means “little brain” in Latin. The 2 hemispheres of the cerebellum are involved in maintaining balance and in controlling motor (muscle) behaviour. Injury to the cerebellum may lead to lack of motor coordination, stumbling and loss of muscle tone.

- **Thalamus:** This is located near the centre of the brain. It consists of 2 joined lobes that have egg or football shapes. The thalamus serves as a relay station for sensory stimulation. Nerve fibres from our sensory systems enter from below; the information carried by them is then transmitted to the cerebral cortex by way of fibres that exist from above. For instance, the thalamus relays sensory input from the eyes to the visual areas of the cerebral cortex. The thalamus is also involved in controlling sleep and attention in coordination with other brain structures, including reticular activating system.

- **Hypothalamus:** This is a tiny collection of nuclei located beneath the thalamus and above the pituitary gland. The hypothalamus is about the size of a pea and weighs only 4 grams yet it is vital in the control of body temperature, the concentration of fluids, the storage of nutrients and various aspects of motivation and emotion.

- **Cerebrum:** The surface of the cerebrum is wrinkled, or convoluted with ridges and valleys. It comprises a large proportion of the brain. The convolutions allow a great deal of surface area to be packed into the brain.

- **Somatic nervous system:** This is the division of the peripheral nervous system that connects the central nervous system with sensory receptors, muscles and the surface of the body. The somatic nervous system consists of our sensory (afferent) and motor (efferent) neurons. It transmits messages about sights, sounds, smells, temperature, posture and so on to the central nervous system. As a result, we can experience the beauties and the horrors of the world, its physical ecstasies and agonies. Messages from the brain and the spinal cord to the somatic nervous system control purposeful body movements, like raising a hand, winking, running, breathing, and movements which we hardly attend to – movements that maintain our posture and balance.

- **Pituitary gland:** The pituitary gland is located below the hypothalamus and it is referred to as the master gland. It circulates hormones that regulate growth metabolism and sexual development. The gland secretes many hormones such as growth hormone which causes growth of muscles, bones and glands; prolactin

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hormone which stimulates production of milk and inhibits production of urine; oestrogen hormone which regulates menstrual cycle; progesterone which promotes growth of female reproductive tissues and maintains pregnancy and adrenaline which increases metabolic activities of the heart and respiration rates and blood sugar. These are just but a few of the hormones secreted by the pituitary gland.

Process of the Brain Development

- **Myelination:** This is the process by which a fatty substance called myelin forms a sheath around nerve fibres. At the same time, the size of the glial cells that support the nerve cell increases.

- **Lateralization:** The brain is organized into right and left hemispheres. The right hemisphere controls the left body part giving directions to the left eye, hand and leg. The left hemisphere controls the right side of the body giving directions to the right ear, eye, hand and leg. The left hemisphere holds the brain centres responsible for receiving, processing and producing language. The right hemisphere has the centres for spatial information, visual tactile or visual imagery. The functional centres responsible for language skills begin to develop early in life. Their maturation, which takes several years to complete, is associated with mylogenetic cells during which myelination occur to particular functional centres within the brain. The protective myelin sheath that coats the nerve fibres enhances their ability to send and receive signals and impulses.

Factors that Positively Influence the Brain Development of Children

- **Parent-child attachment:** Attachment is the desire to stay close to and feel with a specific person, resulting in an internal model of relationships. Research shows that a child's first relationship will have profound longlasting effects on subsequent development. Secure attachments result from a child's sensibility and responsiveness to the parent's behaviour. Children's behaviour is an understandable, logical adaptation to care during the early months of their lives. Children who are secure are cooperative, persistent, competent, empathic, and wellliked by peers and teachers among others. Parents, who believe their children are responsive to them, are more likely to invest time in effective parenting practices. Those with greater selfefficacy (sense of control and confidence) over their own lives may be more responsive and engaged with their children. Likewise, those with strong support networks are also likely to create secure attachments.

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- **Nutrition/diet:** A healthy well balanced diet, especially one containing a lot of proteins, will promote brain development.
- **Early stimulation:** Children, who are talked to, cuddled, clung to, have the company of caregivers, and have objects to play with, will experience better brain development.

- **Health:** Children, who are immunized, have proper environmental sanitation, have personal hygiene and are treated when sick, are likely to have better brain development.

- **Safety/protection:** Young children, who re provided with a safe environment, are protected from accidents that cause injury to the brain or head.

Factors that Negatively Influence Children's Brain Development

- Poor nutrition
- Poor health and sanitation
- Disobedience and lack of self-control, triggered by insecurity in children, influence the children's brain development negatively
- Lack of play, interaction and exploration suppress brain development

Activities Parents and other Caregivers can Carry out to Promote the Brain Development

There is need for parents and caregivers to provide appropriate activities to facilitate brain development; for example, talking to the infants, providing a variety of coloured safe play and learning materials, encouraging role or makebelieve play for older children, exploration and discovery learning. Play is a critical element in early childhood because it provides the context of experiences that are vital for the development of neural pathways.

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2. Process of the Brain Development
3. The chapter analyses factors that positively Influence the Brain Development of children

LESSON EIGHT: PERCEPTUAL AND PHYSICAL DEVELOPMENT DURING INFANCY

Activity

1. ***Explain factors that contribute towards negative brain development***
2. ***What contributions do mothers and caregivers have on a child's brain development.***

Summary

1. Parts of the Brain and Their Functions

INTRODUCTION.

Objectives

By the end of this session, you should be able to:

1. ***Describe physical characteristics of a newborn***
2. ***Explain physical changes in infancy***
3. ***Explain factors that impact on physical growth and motor development, Perceptual and physical development during infancy***

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Many changes take place in a child after infancy causing amazement to those that watch as they grow. This session will discuss the physical and perceptual changes that take place in the child from birth to about 2 years of age.

Changes in Physical Characteristics

Physical development follows a predictable and orderly sequence. This is governed by the interaction between genetic and environmental factors. Most infants follow a normal course of physical growth and development. Some are born with physical disability or are identified as disabled shortly after birth. Information on what normal physical growth constitutes helps us to identify children with disabilities and to develop programs to help them.

Characteristics of the Newborn:

The newborn has perfect physical features e.g. hands, toes, fingernails etc. He or she is relatively tiny. However there is a perfectly formed human being capable of responding to the environment. The following characteristics are evident.

Appearance of the Newborn

Many adults are naturally attracted to a new born. A newborn's appearance surprises many people Some aspects of physical appearance are odd for instance the newborn appears fragile and has a short neck. Sometimes this makes other people hesistant and insecure in caring for them. An oily substance called ***vernix caseosa*** covers the body. This dries a few days after birth. Some have a growth of fine textured hair called ***lanugo*** all over the body. The abdomen appears to be bulging. They have dissapropriately small arms and legs. A large head that may be misshapen and the bones are flexible. Both boys and girls may have swollen breasts and genitals. This is due to maternal hormones that crossed over to their bodies from the mother before birth. The newborn has large eyes, bulging forehead and a small nose.

Individuality

Babies vary greatly in size, shape, degree of physical maturity and personality traits They differ in the way they cope with stress, hunger, thirst etcetera. It is not unusual to hear adults saying a baby is cool, irritable, quiet, calm, tense etc. Babies have different patterns of responding to the environment (temperament). Children range from easy to difficult. Some of these temperament traits persist to adulthood.

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Dependence and Competence

For several months the infant is totally dependent on adult care for instance she must be fed cleaned protected and cared for in every way. The infant on the other hand, is born with remarkable capacities for survival such as he or she is able to eat, eliminate body waste and regulate body temperature etc

Newborns actually cry less of the time than you might think. Crying seems to increase over the first 6 weeks of life and then decreases. Initially, infants cry most in the evening and then shift their crying more toward times just before feedings. The basic function of the child's cry, obviously, is to signal a need. Because babies can't move to someone to get care. They have to bring someone to them, and crying is the main way they have to attract attention. Infants have a whole repertoire of cry sounds, with different cries for pain, anger, or hunger. Infants cry in different ways depending on their needs. So each parent must learn the specific sounds of his or her own baby.

Fifteen to twenty percent of infants develop a pattern called **colic**, which involves daily, intense bouts of crying, totaling 3 or more hours a day. The crying is generally worst in late afternoon or early evening. Colic typically appears at about 2 weeks of age and then disappears spontaneously at 3 or 4 months of age. It is not known why some babies experience colic.

States of Consciousness

Researchers have found that there are five states of sleep and wakefulness in the baby. These states and their characteristics are as follows:

- **Deep Sleep**: Eyes closed; there is regular breathing and no movement except occasional startles.
- **Active Sleep**: Eyes are closed, breathing is irregular and there is no gross body movement.
- **Quiet Awake**: Eyes open, no major body movement and there is regular breathing.

Active Awake: Eyes are open, with movements of the head, limbs and trunk and irregular breathing.

Crying and Fussing: Eyes may be partly or entirely closed. There is vigorous diffuse movement with crying or fussing sounds.These five states tend to occur in cycles.

Sleeping behaviour

Newborns sleep about 15 to 16 hours each day, with little day- night rhythm. They sleep equal lengths of time at any time of day or night. By 6 or 8 weeks of age, the total amount of sleep per day drops and the first signs of *circadian rhythm* (day-night rhythm) merge. Babies vary a lot in their sleep patterns with the average hours of sleep ranging from 88 to 22 hours. Marked irregularity of sleep patterns may be a sign of disorder or problem.

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Another interesting thing about sleep in newborns is that they show the external signs that signify dreaming in adults and older children. This is seen in the fluttering of eyeballs under closed lids, a characteristic referred to as ***Rapid Eye Movement*** or REM Sleep. It is a recurring sleep stage during which the baby and adults seemingly dream. About one half of the infant's sleep is REM sleep.

Infants Reflexes

Nature equips the newborn with reflexes to ensure survival. A reflex is a specific involuntary response to stimuli. A baby is born with a host of reflexes. Many of them disappear after the first 2 or 3 months of life. These reflexes include:

- **Breathing reflex**: Normal newborns take their first breath soon after birth. Breathing is usually irregular for the first few days and reflexive hiccups, sneezes and spit-ups are common as the newborn tries to coordinate breathing, sucking and swallowing.
- **Sucking Reflex**: Newborns suck anything that touches their lips -fingers, toes, blankets and nipples.
- **Rooting Reflex**: Babies turn their heads wherever anything touches their cheeks. This reflex helps them to find the nipple.
- **Swallowing Reflex**: They swallow anything that gets into their mouth.
- **Babinski reflex**: When their toes are stroked, their toes fan upward.
- **Stepping Reflex**: When their feet touch a flat surface, they move as if to walk.
- **Swimming Reflex**: When they are held horizontally on their stomachs, their arms and legs stretch out.
- **Grasping Reflex**: When something touches their palms, their hands grip tightly.
- **Moro Reflex**: When someone bangs on the table they are lying on, they fling their arms outward and then bring them together on their chests as if to hold on to something and they may cry and open their eyes wide.

Most of these reflexes disappear in the first months of life. Preterm babies usually develop and lose these reflexes later than full term babies. These reflexes are useful as signs of normal brain and baby function.
Tests of reflexes are used to assess the newborns physical condition.

Size and Shape

The average newborn measures about 50 centimetres in length and about 3 kg in weight, (this vanes from infant to infant). In the first days of life, most newborn lose a little of their weight as their bodies adjust to sucking, swallowing, digesting their own food etc.

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Once they have made these adjustments, they grow rapidly, doubling their weight by the 4th month and tripling it by the end of their first year. "They increase by about 2.5 centimetres in height every year for the first 12 months. Growth in the second year proceeds at a slower rate. Boys by the end of the end year are slightly heavier and taller than girls. By the end of the second year the child is about one fifth of the adult weight and half their adult height. Much of the weight gained in the first months of life is fat providing both insulation and energy that can be drawn on in case of teething or other problems..

Brain Growth and Maturation:

The newborn's skull is relatively large. This is because it must accommodate the brain. The brain is about 25% it's adult weight at birth. By the age of 2 years, the infants brain is about 75% of its adult weight.

Neurons and Their Function

There are significant changes in the maturing nervous system (which consists of the brain, spinal cord and the nerves). The nervous system is made up of long thin, nerve cells called neurons. At birth it contains most of or perhaps all the neurons it will ever have.

Further development consists of the growth and branching of these cells into increasingly dense networks that transmit messages in form of electrical impulses between the brain and the rest of the body. As the nervous system matures the neurons become coated with a fatty insulating substance called myelin. This helps to transmit messages faster and more efficiently. This process is called ***myelination*** and the process continues until adolescence. The development of the nervous system allows children to gain increasing control over their motor functions and to experience refined perceptual abilities.

Axons are covered with a fatty coating of cells called myelin. It insulates the axon and helps it transmit electrical impulses faster and more efficiently. When myelmation occurs in certain regions of the brain, the child gains specific forms of motor skill and muscle control that are associated with those regions. Consequently the timing of an infant's milestones - the visually directed grasp, rolling over, sitting up, standing, and walking etcetera are directly related to developments in that child's nervous system.

Anything that interferes with myelination affects motor development.

Structure of the Brain

Different regions of the brain develop at different rates before and after birth. The motor regions, which control gross body movements, develop first. The sensory regions, which receive information from the sense organs (nose, eyes, ears, mouth, and skin) develop next. These are the only areas of the brain that function at birth. The sensory and

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motor regions continue to mature up until the eighth month of life. The association regions, which mediate thought, are the last to develop and their growth continues well into adulthood (Suomi, 1982).

Sensation and Perception

How does a newborn know that her mother's skin is soft rather than rough? How does she know the smell of her mother's milk? Infants know these things because of their senses. All information comes to the infant through senses. These senses include: sight (vision), hearing, touch, taste and smell. Sensation occurs when information contacts the sensory receptors-the eyes, ears, skin, tongue and nostrils.

Perception is the interpretation of what is sensed. For example sounds that are received by the ears may be interpreted as musical sounds or the sound of a human being,

At birth sensation and perception, are apparently present. Neonates (infants) see, hear, smell and taste. Newborn perception is very selective. They pay attention to certain things and not others. For example they pay attention to bright lights, loud noises and objects within a foot of their eyes and they almost screen out everything.

Vision

At birth, vision is the least developed of the senses. Newborns focus both eyes on the same spot focusing well on objects that are about 8 inches away (Bee 19995).It not easy to tell whether they perceive these objects or whether they just stare at whatever they see. Within a few weeks the baby can at least roughly follow a moving object with his eyes.

By the age of 3 months, babies recognize the mother's facial expressions and they can recognize their mother's photograph. Newborns do not use their eyes together but by age 4 months they are able to use both eyes together. This makes them better observers since they can differentiate the photos of men from those of women. They show preference for happy faces over sad or angry ones.

Hearing

Unborn babies can respond to loud noises even 4 months before birth. At birth the newborn's auditory canals are filled with anmiotic fluid. Thus for the first day or two after birth, they have difficulty hearing. As soon as the fluid left in the ears is discharged or absorbed, their hearing improves and they can hear relatively well, Newborns can *localize* sounds that is detect the direction of a sound. They are especially sensitive to high pitched sounds. Sudden noises startle babies and make them cry, While awake they turn their heads to the direction of a noise and pay

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attention to the sound of a conversation. By the end of first month, the baby prefers the mother's voice to other voices. By 4 months, the baby can identify the voices of most familiar people in their lives. Infants are born with the ability to respond to all speech sounds.

However, they eventually tailor their speech perception (attention) to the speech sounds they hear.

Touch and pain

Newborns are sensitive to touch (this is evident in their reflexive response. We can soothe a baby by holding or picking them up. Research shows that the experience of being touched has a direct effect on the infant's growth and development. Premature infants who are massaged gain more weight than those who are not massaged but given the same calorie intake.

Infants respond to pressure on the skin by certain reflexive behaviours; If you press their palm with your finger, they will respond by grasping it, and if you stroke the sole of their foot, they curl their toes outward. The mouth shows considerable touch sensitivity during early infancy (Kislevsky & Muir, 1984), followed by increasing sensitivity in the extremities (arms, hands, and legs). We also know that infants feel pain. During the neonatal period babies respond to injections by fussing and crying,

Taste and Smell

There is considerable evidence that sensitivity to taste is present soon after birth (Haith 1986). Newborns have been found to suck harder and longer to obtain sweetened liquids than clear water (Crook, 1978). This innate preference for the sweet taste attracts the infant to breast milk, thereby promoting the bond between mother and child.

The sense of taste develops quite rapidly during the first month. It's easy to tell which tastes infants prefer by observing their facial expressions. Neonates display a slight smile and sucking movements to sweet substances. They purse the lips, wrinkle the nose, and blink to sour substances and they spit up and choke on bitter substances (Steiner.)

Infants continue to enjoy sweet substances and, after four months, they show interest in salty liquids (Beauchamp & Cowart, 1985). Infants' preferences and parents approaches to food selection ma y have long-term effects on food preferences later in childhood. For example, infants who had been regularly fed with sugar water in the first year showed stronger preference for sweet liquids at age two than infants who had not had sugar water (Beauchampo & Moran, 1985). Moreover, infants who experienced a greater variety of foods developed more diverse food preferences than infants who experienced less variety (Frietas, 1984).

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Infants perception of faces

The human face is perhaps the most important visual pattern for infants to perceive. At about 4 weeks the infant, is fascinated with the eyes. At 2 months and older, the infant begins to differentiate facial features; the mouth is noticed and movements of the mouth draw attention to it. By 6 months of age the infant distinguishes familiar faces. For example he or she can distinguish mother from stranger, masks from real faces and so on.

Infants depth perception

Another question about what babies see and know is whether they have depth perception, that is, the ability to tell how tar away something is in terms of depth. In one study (Gibson & Walk. 1960) placed 6 to 14 months old infants on a special surface that appeared "shallow" on one side and "deep" on the other side. The deep side was covered with glass so babies could crawl across it just as they had crawled the shallow side. Their mothers stood on the far side and encouraged them to crawl across them. The babies in this study could not crawl across the deep side even after they felt the glass. This study shows that these infants were somehow aware of depth. However it is known exactly when depth perception is acquired.

Motor Development

Motor development deals with control of movement of various parts of the body. At birth the baby is a dependent creature who stays in one spot when laid down. As the primary motor area of the brain matures the motor skills emerge. The sequence of motor skills is generally the same. At the beginning the motor abilities follow the same cephalocaudal aad proximodistal patterns as in physical growth e.g. the infant lifts the head, then their shoulders, before they can sit up. They sit up before they can stand.

Hand Skills

The specifics of arm, hand and finger control follow the proximodistal pattern. At birth infants have a reflexive grasp, but they have no control of it. Then they start moving their arms and hands when they see dangling objects -but they can't hit them until they are about 3 months old, They still cannot grab the object because they close their hands too soon or too late. By 6 months, most babies can reach, grab and hold on to dangling objects. They still have a new problem –they can’t let go. A toy seems stuck in their hands until interest is lost in it then their hands relax and the toy drops out, letting go is mastered by about 7 months. Another activity to be mastered is picking about small objects by fingers. At first babies use their whole hand usually the palm and slowly they learn to hold with fingers by about one year of age.

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Locomotion: Most babies first learn to move from place to place by lying on their stomachs and pulling themselves ahead with their arms. Later they learn to crawl on hands and knees at around 7 to 8 months. Some babies do not crawl at all.

On average a child can stand with support at 7 months, can walk with support at 9 months and can walk unassisted at 12 months. In recognition of their walking accomplishment, the babies at this age are called toddlers. This is because of the way they use their legs toddling from side to side. Since their heads and stomachs are relatively heavy and large, they spread out their little legs for stability.

One-year olds cannot run and they fall frequently. By age 2 years, most children can walk and run quite well still placing feet wide apart.

Note that although healthy infants develop the same motor skills in the same sequence there are variations in timing. The age at which these skills are acquired varies from infant to infant.

Can Motor Control in Infants be facilitated?

Research has shown that training will facilitate the development of motor skills.

Changes in Bones and Muscles

Changes in the nervous system are paralleled by changes in other body structures, including bones and muscles. However, changes in bones and muscles occur gradually from infancy through adolescence.

Bones

The hand, wrist, ankle and foot ail have fewer bones at birth than they will have at full maturity. For example, in an adult's wrist there are nine separate bones. In the one-year old, there are only three. The remaining six develop over the period of childhood, with complete growth by adolescence.

The skull of a newborn is made up of several bones separated by spaces, called **fontanels.** Fontanels allow the head to be compressed without injury during the birth process, and they also give the brain room to grow. In most children, the fontanels are filled in by hone by 12 to 18 months, creating a single connected skull bone.

All of the infant's bones are also softer, with higher water content, than adults' bones. The process of bone hardening is called **ossification**. It occurs steadily from birth through puberty, with bones in different parts of the body

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hardening in a sequence that follows the typical proximodistal and cephalocaudal patterns. For example, bones of the band and wrist harden before those in the feet.

Bone hardening has some direct practical relevance. Soft bones are clearly needed if the foetus is going to have enough flexibility to fit into the cramped space of the uterus. But because of this flexibility the newborn human is relatively helpless, Newborns cannot even hold their own heads up, let alone sit or walk, As the bones stiffen, the baby is able to manipulate his body and this increases the range of movements making him much more independent.

Muscles: Muscle fibers are initially small and watery, becoming longer, and less watery at a fairly steady rate until adolescence. The sequence is again both proximodistal and cephalocaudal. So the baby gains muscle strength in the neck fairly early, but does not have enough muscle strength in the legs to support walking until some months later.

Nutrition.

Adequate nutrition is a pre-requisite for the changes and mastery of skills. At first infants are unable to eat or digest food, but their rooting, sucking, swallowing and breathing reflexes enable them to consume the quantities of liquid for nourishment they need.

Appropriate nutrition and feeding habits help to ensure healthy development of the infant. Infant's nutrient needs differ from those of adults in both quantity and proportion.

Breast Milk important

Breast milk alone will meet the infant's needs during the first 4 to 6 months. In early months breast milk is the most ideal infant food. It has several advantages which include:

- It is sterile, that is, not contaminated.
- It's at body temperature. It is not too c old or too hot. Thus it doesn't need to be healed or cooled for consumption,
- It contains proper nutrients needed by the growing baby for instance contains more iron, vitamin C and vitamin A than cow's 'milk
- It contains antibodies that provide the infant some protection against disease that the mother herself has had, or has been inoculated against such as chickenpox, smallpox, tetanus etc.
- It's more digestible than cow's milk or formula - hence breast babies have fewer allergies and digestive upsets than bottle fed babies.

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Weaning

Parents often want to know when to introduce new types of food. In the second half of the year breast milk continues to be the infant & primary nutritional source, in order to thrive during the first two years infants need about 50 calories per day per pound of body weight. These calories can not be supplied from breast milk alone. By 5 months or so babies need solid food. Hence solid foods should gradually be added to the diet. For example, cereals for iron & fruits for vitamins A & C, By the lime the infant is one year old, the diet should include all the nutritious foods the rest of the family consumes, The process of giving up one method of feeding for another is called ***weaning.***

Malnutrition

During the first year of life, severe protein- calorie deficiency causes ***Marasmus,*** growth stops, body tissues waste away and can lead to infant death. During toddler hood (from 1 year on to 2 years} protein calorie deficiency is 05t)re likely to cause ***Kwashiorkor.*** A condition in which the child's face, Legs and abdomen swell with water - the child may appear well fed to anyone who does not know the real cause of the bloating. In this condition, the essential body organs take the available nutrients at tile expense of other it. Child's hair become thin, brittle and colourless (may appear brown for the African child).

The primary cause of malnutrition in developing countries is early cessation of breastfeeding. Malnutrition can lead to diseases or death. Malnourished infants who survive carry handicaps that sometimes remain throughout their life span- e.g. seriously intellectual inferiority is due to slower brain development caused by lack of essential nutrients. Malnourished infants may become sickly and hence fail to explore the environment adequately. Most motor skills are developing during the first two years- and these can be affected by serious malnutrition.

Both mother and baby need to be properly fed with a proper diet to avoid serious malnutrition in the baby.

Factors that impact physical growth & motor development.

Genetics

Family hereditary factors contribute to:

- Rates and range (genotype and phenotype) of size and weight.
- Specific learning styles, types of intelligences, and learning disabilities.
- Development of physical diseases and allergies.

Nutrition

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Amount and types of foods intakes impact and growth.

- Excessive caloric intake leads to obesity: baby formulas.
- Improper diet results in malnutrition and reduced health with underweight wasting and stunting and underdevelopment of individual brain cells and thus brain's functioning capacity.
- Chemicals in specific foods may enhance concentration and memory in brain (choline in egg yolk, meat, fish, cereals and legumes): high carbohydrate diet increases serotonin levels and tryptophan, an eurotrasnsmitter actives when we sleep)
- Absence of specific minerals and vitamins relate to development of specific diseases (Example iodine: goiter)

Emotions

High stress impacts ability of body to produce growth hormone (somnetropine) during sleep so growth is retarded (failure to thrive)

Stimulation

Environmental stimulation enhances genetic predispositions to encourage early and stronger physical capacities, including enhanced muscle development and coordination (Practice makes perfect)

Gender

Boys tend to be heavier and longer at birth and have less dense (cartilage) at birth.

Both boys and girls grow at very similar rates, but boys initial growth in first few months is just slightly slower than girls so by the end of first three years they are approximately the same height and weights all other things being equal.

Activity

1. ***Briefly describe the perceptual process in an infant.***
2. ***What physical changes occur in an infant?***

Summary

1. The session covered the perceptual and physical development during infancy

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2. It explained the characteristics of new born, states of consciousness, neurons and their functions, infants' level of fine senses.
3. It also dwelt on the physical growth and motor development.

LESSON NINE: LANGUAGE DEVELOPMENT IN INFANTS

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Objectives

By the end of this session you should be able to:-

a) Describe the process of language development during infancy

b) Explain adults' role in language acquisition·

INTRODUCTION

Crying represents the infant's earliest vocalization. There are different varieties of cries. Each type of cry usually has a meaning and mothers usually learn to distinguish among the different varieties. Crying is not a language but it represents a type of communication, as it is the means by which infants convey their basic needs. During the first six to eight weeks of life, variations of the cry are the only vocalization that take place. Adults through experience learn to recognize the various types of cries.

Probably you have heard mothers comment about to infants cries. For example, "she cries this way when she wants to sleep" and indeed the baby sleeps almost immediately- This is evidence that adults are able to recognize an infants specific need by their cries. This enables them to respond to the child in appropriate ways.

Cooing and Babbling

Cooing emerges from the second month, it consists of a repetition of vowels .For example, "aaaah" or "ooooo". Cooing though not language is a form of communication because it generally indicates that the infant is pleased happy or excited. It is expressed alter a certain degree of maturation is attained. It requires muscular movements of the **tongue** that were not possible at birth. Cooing occurs m both normal and deaf children- It diminishes at approximately 8 months of age.

Babbling emerges at about 6 months, It consists of a repetition of a combination of consonants and vowel sounds, for instance dadada, mainama, bababa etc. Infants babble in a similar way irrespective of their linguistic background- Babbling is important because it gives the infant an opportunity to exercise their vocal organs and enables them to hear the range of sounds that they are capable of making. Babbling is the tint vocalization with resemblance to speech. Infants also begin to vary the intonation of their vocalizations during the second hall of their first year For example, they sometimes babbles with a raised or lowered pitch. These intonation patterns correspond to the signals older children and adults use for asking a question versus making a statement (Schwkedanz, Hansen & Forsvth WO). Variations in intonation increase during the second year(Bruner 1975). Babbling is accompanied by excitment and motor movement e.g they swing their hands and legs, move head etcetera.

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Infants make sounds that arouse their interest. Often they lie quietly while listening to sounds other times they babble in response to the verbal stimulation around them.

Holophrase stage (12 to 18 months)

This is the emergence of the first word. Children usually learn words related to food, toys, animals and people. These words are usually concrete nouns. The child does not understand all the words learnt. Most words are acquired through imitation hence the child may not know what they represent. Words may not be properly pronounced.

Two-word stage

By approximately 18 months children start using two word expression. There are usually utterances consisting of two single words. For example, “mama milk”, “me go” etcetera

Telegraphic sentences

These are short and simple sentences consisting mainly of nouns and verbs. They are referred to as telegraphic because the sentences lack some words, tense endings, plural endings on nouns and other grammatical omissions e.g. “*mama me milk*” meaning mama give me milk and *“soup hot"*- meaning this soup is hot.. These sentences can be likened to the messages we send by telegram which are usually shortened and grammatically incorrect in order to reduce posting costs. There is a considerable progress in sentence structure by the age of 3 years. Children's vocabulary increase. Their knowledge of syntax {knowledge of grammatical rules) and semantics (understanding of word meanings) also increase daily as they continue using and listening to language. By the age of 3-years they can participate in conversations.

As young children learn words, they often use them differently than adults do. Sometimes they do not use words us broadly as an adult do. This is called **under extension**. For example a young child might use the word "bag" only to refer to the mother's bag because it was in this context that she first learned the word. In this case she has under extended the word's meaning. Other times they **over extend** the meaning of a word. An urban child might call his grandmother's goat a dog because it has four legs and a tail like a dog.

In this case the child has overextended the meaning of a word.

Milestones in Receptive Language

The other side of children's language behaviour is the receptive side. This involves the child's ability to understand the meaning of individual words. This is also called language perception.. Together with human speech sounds,

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children hear all kinds of sounds; the moving vehicles, the sound of cows, the burking of dogs, the cracking of the chicken for instance. However the human sounds have special significance for the baby. (Schickedanz, Hansen & Forsyth 1994). According to a study by Wolff (1969), by the second week of life, a crying infant is quieted by a voice more effectively than by other sounds.

Children develop language perception before they are able to produce words. That is children understand before they can speak. They understand quite several words when they can only say only two or three words.

Socio cultural and environmental influence on children's language development.

According to Snow(1989), early exposure to language is necessary for acquisition of competent language skills. As a result of this exposure, children acquire their native language without explicit training or teaching. However, there are factors that provide facilitative effects. Santrock (1996) discusses some social supports that enhance language development.

Social supports for language

During these early stages of development, children need the benefit of adult language social supports. Such support improves the development of language. Children therefore need to hear speech around them. The adult speech reinforces the child's vocalization. Parents provide support to their children in different ways. The unique communication style among adults is known as **motherese.** It's so called because it's observed more frequently among mothers of infants. Motherese is simple redundant language. In motherese, adults often raise their voice and use brief sentences. This mode of communication is designed to teach babies to talk.

Other than motherese there are strategies that adults use to enhance the child's acquisition of language. These include recasting, echoing, expanding and labeling. **Recasting** involves phrasing the same or a similar meaning of a sentence in a different way like for instance turning it in to a question. For example, if a child says "mama- me-milk", the adult can respond by asking, "who wants milk?" **Echoing** refers to repeating what the child says to you. **Expanding** involves restating what the child has said in a linguistically sophisticated form. In the above example, a mother can respond by saying "mama give me my milk" **labeling** is giving the names of objects.

FACTORS THAT IMPACT ON LANGUAGE ACQUISITION.

Exposure to Language

The more a child is exposed to good language models and to each of the four areas of language the more proficient the child becomes in that language. The opposite is also true. So the two most important aspects of exposure are; the

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quantity of time of exposure and the quality of language models. Since learning a language take time for mastery the child can't learn a language if he/she doesn't hear it spoken or see it written or if the langue model whom they are to imitate is not proficient in the language.

Stimulation to Motivate Learning.

Cognitive Stimulation enhances the probability of children developing concepts that they want to communicate about. Hence, they want the label that they will obtain from those around them.

Linguistic Stimulation is the use of language by those around the child. In hearing the language being used by significant persons in their lives, children hear the sounds/grammar and meaningful language units that can be used in all of its potential functions and forms, the more easily and rapidly they are able to learn the language.
If caregivers use motherese it simplifies the process of language learning for the child. **Motherese** is a simplified version of a language that has shorter sentence structures (grammar) and simpler words (vocabularly lexicon) . Used within a language learner to assist in communication and learning of the language.

Positive Emotional Stimulation ensures that the children feel loved and secure so that they are able to learn freely without anxiety. Providing love and nurturing care (meeting all of their physical needs quickly) provides this needed emotional security.

Health (and Nutrition) Status of the child.
If children are sick and malnourished they are less likely to be able to attend the language around them or have the motivation to learn to communicate in that language. Consequently, the children are less likely to learn the language.

Physical Health includes the status of the child's health physically in terms of presence of illness or brain damage caused during birthing or by illness or accidents. If the child is physically well, he or she is highly motivated and able to learn the languages of important people in their lives (significant others) such as the primary caregiver and immediate family members. When a child suffers from severe malnutrition there is energy depletion and learning is retarded. The same applies to illness especially severe illness. Usually after nutritional and health status improves there is a period of **catch up growth spurt** that in this case applies to the language usage.

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Brain damage to the areas of language learning that occurs at these young ages causes the brain to adapt to other areas nearby or those in the other hemisphere (side of the brain) to language learning. Thus, language learning is only shortly delayed unless the damage is severe. The brain is very placid (flexible) at these ages and is somewhat flexible as to where language (and other capabilities) are processed.

Psychological Health refers to the development of positive emotional states and social relationships so that the child fees secure and capable. When children are exposed to excessive negative stimulation (excessive yelling, physical abuse etc) the front part of the brain is excessively stimulated and appears to interfere with the stimulation to other area needed for learning. This explains why children in abusive households appear jittery, withdrawn and very cautious even if they have never been abused themselves.

Individuals Capacity and Individual Differences.

Children are not the same and there are differences in capacity and learning characteristics that impact their ability to learn languages.

Rate of learning varies from child to child depending upon genetic codes. Some biological learn very quickly, while others learn moderately fast and others more slowly. This could be generally in reference to learning anything or specific to language learning.

Type of Intelligence also impact. Children who have high Linguistic Intelligence tone of the multiple intelligences identified by Howard Gardner they are more likely to learn language(s) more rapidly and with less effort.

Level of Overall Intelligence also contributes to language learning. Children with low overall intelligence (low Intelligence Quotient or IQ) require many more language experiences and opportunities to learn. They require more time and many more experiences with motherese forms of language. They will learn but at a different rate. The more serious the deficit in intelligence, the slower the rate of learning and the lower complexity of the language will be learned.

Gender

Boys tend to learn language slower than girls. This appears to biological due to brain differences.

Multiple Language or Single Language Learning.

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The more language a child is exposed to impacts the rate of language learning. There is naturally some cross learning

Activity

a) Explain ways you would use to help a child develop language

b) Which mistakes would an adult avoid that can hamper language development.

but the brain is capable of identifying the different language structures and forms if it is exposed to adequate language models for some time. Any "mistakes" in language learning at this age should not be considered as errors but as evidence of the breadth and depth of the languages being learned and the status of the language learning.

Summary

The session dealt with the development of language in a child during infancy. Children need social interaction with adults to facilitate development of language.

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LESSON TEN: SOCIAL AND EMOTIONAL DEVELOPMENT IN INFANCY

Objectives

By the end of this session you should be able to

1. ***Explain the theories of infant attachment***
2. ***Discuss whether a half year old have any peer interaction***
3. ***Explain the role of caregivers in attachment process.***
4. ***State the factors that influence emotional development.***

WHAT IS SOCIAL AND EMOTIONAL DEVELOPMENT?

Social Development: This refers to changes in a person's capacity to relate interpersonally with others. After the child is born he/she finds him/herself in a social context. The infant begins to relate with other people they grow they learn to interact with other people in the environment.

Emotional Development: Changes in a person's capacity to control and express feelings. As an infant grows, he makes adjustment in the way be expresses his/her emotions. There are also changes in the variety of emotions that children express as they grow and develop.

Attachment: Attachment is the strong emotional tie that babies and their mothers (or other primary caregiver) develop for each other. The quality of attachment is a crucial factor in his social and emotional development. When a child interacts with his mother, he learns patterns of interacting with others. From the way he feels about her, he learns ways of feeling about him/herself and others.

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There are many theories about how and why an emotional tie develops between parent and child.

Theories of infant attachment

The Psychoanalytical Explanations: Both psychoanalytical and behavioural theories purpose that babies become attached to adults who satisfy their basic needs, especially hunger. Traditional Freudian psychoanalytic theory is based on this notion of **drive reduction.** According to this theory, inner needs (hunger) create tension, which in turn generates a drive to reduce the tension. The individual seeks to reduce the drive by satisfying the need (by eating, for example). The adult who gratifies a baby's needs becomes associated with the pleasurable sensations that result. Psychoanalytical theory also assumes that each person has a limited amount of mental energy for attachments, so the child comes to identify with the parent of the same sex, but only after having worked through conflicts associated with his first emotional tie with the mother.

The first relationship with the mother is also assumed to be extremely important for personality development and as a model for later love relationships. According to Freud, the mother and father continues to be present in the individual's mind as objects of love, admiration and fear long after the actual parent child relationship is over.

Behaviouoral Explanation

Early behavioural also explained attachment in terms of drive reduction. The adult who feels a baby becomes a *secondary reinforce.* Babies become attached to their mothers because they associate them with being fed.

Unlike psychoanalytic theory, behavioural theory has never used ideas like "mental energy" and thus has never claimed that the mother is the baby's only attachment figure. A baby might become attached to several adult caregivers if all of them are involved in feeding. In addition, in behavioural theory the first attachment is not considered central to all others.

The Ethological Explanation

The ethological explanation for attachment, first proposed by John Bowlby in 1969, claims that infants are preprogrammed that is, genetically "wired" to form attachment to the adults who best ensure their survival. **Ethology** is the study of animal behaviour. The ethological explanation for attachment focuses o the way young animals, including young humans, keep in touch with their mothers, and mothers (Water & Deane, 1982). Babies do this by sending signals to their mothers, and mothers respond by reacting to the signals. Babies prefer adults who read their signals accurately and respond to them appropriately. Although signals often revolve around feeding, attachments

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are not necessarily based on this situation. Babies may develop strong attachments to adults who have never fed them or provided them with other basic physical care.

The ethological explanation accounts both for the way babies try to stay near their mothers and for the way separate to explore. Babies must be able to explore if they are to learn about their environments and become independent, but they also need to be kept safe from danger.

The ethological theory predicts that babies will become attached most strongly to the adults who respond the best to their signals. This theory suggest is that babies are born with certain capabilities for building relationships with adults. It also assumes that adults have to be able to respond to the subtle signals babies send if these relationships are to work.

How do we tell if a baby has become attached to a particular adult, and how can we judge the quality of the attachment?

A standard method has been devised to study and describe attachment behaviour in babies between 1 and 2 years of age. It is called **strange situation.** It was developed by Mary Ainsworth and collegues in 1969. It involves seven episodes and three participants- mother, baby and stranger.

The experiment procedure begins with the mother and the baby being introduced to the laboratory playroom. Then the mother and baby are left alone in the room, but the mother doesn't play with the baby (episode 1). Next, a stranger enters, talks to with the mother a bit, and then focuses her attention on the baby (episode 2). The mother leaves introducing episode 3 in which the baby is separated from the mother, leaving the stranger and the baby alone. Several features of infant attachment become apparent in the strange situation. The researcher or observer can see to what extent the baby used the mother as a secure base for exploration, how much the baby prefers the mother to the stranger, and how likely the baby is to consider the mother a strange situation, researchers have identified and described three major patterns of response. These patterns, which indicate the quality of infants' attachment to their mothers, have been labeled; secure attachment anxious am bivalent attachment and anxious/avoidant attachment. In this pattern, the baby separates from the mother to explore toys in the playroom but periodically touches base with her. When she leaves, the baby is moderately distressed and either cries or plays in a more subued way. When she returns the baby greets her warmly. In the second pattern, Anxious/ambivalent, also called resistant), the baby stays close to the mother and doesn't explore the toys. When the mother leaves, he or she reacts with great distress. On her return, he or she seeks her and rejects her at the same time. That is the reaction of ambivalent.

In the third patter (anxious/avoidant), the baby readily separates from the mother to explore the toys and doesn't seem to prefer the mother to the stranger. When the mother leaves, the baby shows no concern and when she returns he or she ignores or avoids her.

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Each pattern seems to reflect different patterns of behaviour by parents. When parents respond sensitively to the baby's signals, the baby tends to become securely attached. When they respond inappropriately, or not at all, the baby forms an anxious attachment of one kind or the other.

Infants signal For Attention

The most powerful; signal is **crying.** Adults find infant crying unpleasant and they usually move quickly to do something about it. Many parents can tell from listening to the infant's cries whether the baby is hungry, angry in pain. Most of them respond to a crying baby by picking her up and holding her. The baby now uses another very powerful signal, **gazing.** This steady intent attention to another's face indicates that the baby wants to engage in social interaction. The parent will probably gaze back at the baby and smile or speak. The baby smile or coo in return. Parents like this eye contact- it makes them feel that the baby is enjoying their company. They will often gaze back at the baby as long as the baby is gazing at them. Adults like looking at the faces of babies because they find them cute. Babies signal their wish to reduce stimulation or end the interaction by looking away. Sensitive parents do something different when the baby looks away. Of course, starting to cry again is another way the baby says,

"that enough"

Besides crying and gazing, babies have other inborn ways to make adults pay attention and interact with them on their terms. When they are eating for example, babies suck in rhythmical bursts and then **pause.** Mothers often jiggle babies during their pauses, and in the first few weeks they learn that short jiggles make the baby start to suck again. This biologically based sucking pattern, with it's alternating bursts and pauses, keeps the mother actively involved in the feeding. She realizes that her attention is important to the feeding process, and she watches the baby's face closely. The baby often gazes back at her, and this too keeps her attention. When the baby is done with eating, the mother may wish to continue this pleasant interaction in some other situation such as playing. The turn taking nature of the feeding interaction may help mothers to learn to observe their baby closely and adjust their responses to the baby's behaviour.(kaye, 1982)

As the newborn matures, another powerful signal appears the **social smile.** It begins to apparent on the baby's previously expressionless face by about 2 months of age. One day the baby's eyes light up and her face breaks into a smile. News that "the baby smiled today" is thrilling for parents and siblings. Smiling is the baby's way of saying that all is well with the interaction that is going on at the moment. When the smile starts to fade and the baby sobers, something is going amiss. Smiling and sobering, gazing and gaze aversion are the signals with which the baby regulates her interactions with her parents in the early months (Schikedanz, Hansone & Forsythg 1992)

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The development Smiling

Newborns are often seen smiling in their sleep, but these spontaneous smiles are triggered by changes in their internal sleep states and are known as **endogenous smiles (**Emde & Gaensbaurer, 1980**).** Between 6 and 8 weeks of ages, the social smile appears- a smile given in response to something outside the baby. The baby, smiles more and more frequently over the next few months (Wolff, 1969; Anisfeld, 1982; Korner, 1982)

At first, the social smile is in discriminate. The baby smiles at familiar and unfamiliar adults alike, as long as they make some friendly gesture such as gazing, smiling or talking. But at about the fourth month, things begin to change. The baby begins top search the faces of unfamiliar people and to sober a bit towards them. By 6 to 8 months of age, thebaby's reaction may be so intense that he cries if a stranger approaches (Koner, 1982: Lamb, 1982). The baby's smile is now a very selective social smile; reserved only for familiar people. Unfamiliar people are likely to make the baby cry. When a baby reacts this way to a stranger (usually between 6 and 18 months of age) he is showing ***stranger anxiety.*** Babies can be real partners in even the earliest social interactions, actions and responses with their repertoire of signals. Parents must be able to read and respond to the signals. What happens to the relationship when parents are insensitive to their baby?

Relationship between infant attachment and parent's behaviour.

The parent's overall pattern of behaviour towards the baby determines the kind of attachment the baby will develop for the parent. Babies become **securely** attached when their parents give the **responsive feedback securely** attached when their parents give them **responsive feedback.** This means that they (1) pay attention to the baby's signal;(2) interpret the signals accurately: (3) give appropriate feedback; and (4) respond contingently, that is, they respond promptly enough for the baby to feel that his signals caused the response (Ainsworth et al, 1978). Responsive parents also cooperate with the baby's on going activity rather than intruding or interfering. Mothers of securely attached babies "are more responsive to their infant's cries, hold their babies more tenderly and carefully, pace the intersection contingently during face to face interaction, and exhibit greater sensitivity in initiating and terminating feeding"(Crockenberg, 1981, p. 857)

Parents of anxiously attached babies act quite differently. They may ignore the baby's signals, or they may respond but inappropriate ways. For example, a mother may take the opportunity to read the newspaper every time she feeds the baby, holding him loosely and not noticing his gaze. A father may consistently ignore the baby's hungry cries in the middle of the night to get some extra sleep. A parent might bounce a baby vigorously when the baby prefers to be cuddled or end a feeding when he baby only wants to rest or, continue to stimulate the baby when the baby wishes to stop.

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More serious problems in attachment occur when a parent's behaviour is more disturbed. For example, some mothers dislike physical contact with their babies or treat them in an angry or threatening way (Bretherton et al, 1986) .Some mothers have "restricted affect", meaning that they express fewer emotions that other mothers, especially positive joyful ones (Main, 1981). Some mothers even mock their babies' behaviour, speak sarcastically to them, handle them roughly, and "stare them down". Babies treated this way are likely to have severe problems in their emotional and social development.

Sensitivity in parenting has two dimensions: **responding versus not responding** and **responding appropriately versus responding inappropriately.** Babies whose parents respond to their signals learn that what they do matters. Children whose parents are insensitive to their signals may come to exhibit "learned helplessness", the feeling that they have little effect on the world.

Babies whose parents don't respond contingently or consistently may show the **ambivalent (or resistant)** pattern of anxious attachment. The response they receive is appropriate when it comes, but they are not sure when it's going to be given or if it will some at all. Because they can't be sure the parent will give the desired comfort, they seek it but then resist it, as if anticipating disappointment.

The second dimension of sensitivity in parenting- responding appropriately versus responding inappropriately-in combination with the first may affect the development of the child's trust in others. Babies whose parents respond to them appropriately learnt to expect that people will react to them in desired ways. Babies whose parents respond inappropriately, who find they can get a response but aren't sure what it's going to be, may tend to show the avoidant pattern of attachment. They stop sending signals and may even try to avoid the parent. Perhaps as a protection against getting an inappropriate response.

Kinds of attachment reactions in babies

Between 6 and 12 months-babies develop a whole new set of actions that they did not have before they begin to move around. They learn to cling, creep, crawl and finally walk. Now they don't have to wait for their parents to come to them or just watch them as they move away. They can follow them around. At about the same time, babies start to discriminate sharply between familiar and unfamiliar adults. When approached by someone they don't know, the sober and may even cry, a response known as **stranger anxiety.** Babies also become capable of gaining information about new people and situations by **social referencing,** for example by asking questions, "is she good mummy?"

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Somewhere between 8 and 10 months of age, babies start to be able to express on their faces emotions such as joy, surprise and anger. Now they can put together facial expressions, sounds and motion to produce the full-blown attachment reactions that parents or other caregivers know so well. When they see their parents after a separation they greet them with joy and stretch out their arms or crawl to them. When they're separated from their parents, or when they are concerned that a separation may be coming, they show **separation distress-** they sober, cry and cling or try to follow. The older infant and the toddler show attachment by waling or running to keep in contact with the parent. With the acquisition of language, the child adds words to the protest e.g. mom doesn't go. These kinds of scenes are practically the hallmark of the toddler years, when separation distress is at its height. Another important development is social referencing.

Social referencing

The strategy of using information about how another person feels about a third event is called social referencing. It involves scanning the face of a familiar and trusted person to pick up in formation about a new situation. From that person's emotional reaction to the situation, the child can form his response. Babies 8 or 9 months of age and older often use this strategy. as they get older, they get better at interpreting and using the information their parent provides. When babies engage in social referencing, it indicates that they recognize and can respond to the facial expressions that reflect various emotional states.

Develop attachment

There are various explanations (theoretical explanations) or how infants develop attachment with other people.

Biological explanation.

Parental history explanation.

The social system explanation.

The Biological Explanation.

The biological explanation, ,also known as the critical period explanation, claims that maternal attachment is based on hormones. Hormones released at the end of pregnancy or soon after birth cause in the mother to bond with her baby. **Bonding** refers to the dramatic and immediate emotional tie that mothers feel towards their newborn babies. It is triggered in the mother by contact with her baby within a critical period immediately after birth, when the hormones are at their highest level. If the mother bonds with the baby during this early period, her love will continue

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to grow, and she will be able to provide the responsive care needed to ensure the baby's attachment to her. The biological theory of maternal attachment originated in research on lower animals. Studies of sheep, goats and rats showed that mothering was disrupted when the young were separated from their others immediately after birth, even for only one or two hours (Collais, 1952). The identification of a critical period for maternal attachment emerged from these studies.

The Parental History explanation

Other theories and researchers have suggested that maternal attachment is closely related to the mother's history, that is her earlier experiences in life. A mother's ability to form a close, nurturing tie is closely related to her personality and on the way she was cared for when she was a child. Mothers who have no positive attachment to their children are seen as lacking in parenting skills, knowledge about children, or the ability to act responsibly because of their known immaturity. Sometimes these mothers have psychological problems or personality disorders. However, parents' childhood histories don't completely explain maternal attachment behaviour. After all, not all abused parents abuse their own children. Parental history and personality characteristics may provide part of the explanation for maternal attachment, but they don't account for it completely (Parke & Lewis, 1981)

The socials system explanation.

Because no single factor completely explains maternal attachment, some researchers have considered that many factors might be working together. They have proposed a model, known as the interaction/social system explanation, suggesting that maternal attachment is a function of several interacting factors:

(1) Characteristics of the mother (such as personality traits and psychological well-being):
(2) Characteristics of the baby (such as health, degree of development at birth, activity level etc);
(3) Characteristics of the family (such as number of children, role of the father in child care, economic resources etc) and
(4) Social supports beyond the immediate family (such as extended family, friends, medical resources etc.

These factors are thought to interact to determine the kind of attachment the mother forms as well as the extent of her ability to nurture attachment in the baby. It's the combination of circumstances-not one or another single factor-that determines the quality of the mother-baby attachment.

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Since many factors are involved in the development of attachment, it follows the infant **temperament-** the inborn characteristics that account for differences in crying, cuddliness, activity level etc among babies is not a critical factor in the relationship. Temperament characteristics at birth often don't last, and they don't predict the baby's later behaviour nearly as well as the mother's parenting does.

The social system model is currently the favoured explanation for maternal attachment. Studies have shown that the quality of the early attachment has long-lasting effects on the child's social and cognitive development. Children who were securely attached as infants grow into toddlers and preschoolers with a positive attitude, a friendly interest in other people, and a confident approach to problem. Children who were anxiously attached as infants, on the other hand, have a much harder time getting along in the world. They tend to become extremely difficult children who are negative and aggressive with people and tasks. They evoke reactions from others that keep them in negative relationships and that prevent them from developing more positive social behaviour.

Do babies interact with other babies?

Babies notice and interact not just with adults but with other babies as well. They are curious and interested in what their peers might be like.

Babies' peer interactions are different from parent-child interactions. Babies start to watch other babies and send them social signals as early as 10 months of age. They might smile, laugh, or vocalize to them, more often than not from a distance or they might cry or fuss while watching another baby. Occasionally they might touch or strike another baby. Many times the interactions among infants and toddlers involve a toy. Toddlers often offer a toy to another child, reach for the toy the other child is playing with, or attempt to play with toy while the other child is playing with it. Some of the signals observed in a group of toddlers are listed and defined.

Category	***Definition***
Offer	Child holds out a toy to peer.
Deposit	Child places an object in peer's lap or at peer's feet.
Offer-withdraw	Child holds out a toy to peer, then retracts it as peer starts to accept it.
Join gesture	Child acts on peer's toy e.g. child hits ball held by Peer.
Play gestures	Examples included throwing a ball towards a peer, taking a book to a peer.
Affiliative gestures	Child kisses, pats, hugs peer.
Point to peer.	

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Reach for peer's toy. Touch
peer.
Touch peer's toy. Smile/laugh.
Verbalizations.

Real social interaction requires not just a signal but also a response. Infants and toddlers aren't very adept at responding. By far their most frequent response to a social gesture from another infant or toddler is to ignore it. They also might look at the other child or passively grasp an offered toy.

Reciprocation of the social intention of the child are fairly rare in the first or second year of life. When a child of this age does respond to a social gesture. It's most often a single response with no further interaction following.

Children up to about 2 years of age don't have much ability to sustain social interaction with peers.

Interactions do increase when children are brought together more than once and have a chance to become acquainted with each other. They also sometimes get caught up in social games, group activities that involve taking turns, repetition and imitation, along with lots of smiling and laughing.

Emotional Development

It was once thought that young infants do not have emotions (feelings). Their cries and smiles were thought to be simply reflex actions that were not truly related to emotions feelings. It has however been found that infants show and respond to many emotions e.g. joy, surprise, anger, fear, disgust, interest, sadness etc.

Birth- 6 months.

One of the first emotions seen in infants is fear. Infants a few months old (2 months) will cry, look surprised and afraid if they hear a loud noise or if they suddenly loose support or see an object fall towards them. Slightly older infants have more pronounced fear reactions and seem angry at times. Babies are also sensitive to sadness e.g. in an experiment. Infants between 1 month and 3 months responded by turning away when their parents pretended to look sad and depressed. When surprised they open their eyes wide. They smile when happy or when smiled at. This is a clear indication that these emotions are developed at least to a certain extent, in the infants.

8 Months-2 years.

By 8 months, infants emotions become much stronger e.g. many new experiences produce fear. There are different types of fears that are universal among children fear of strangers (Stranger anxiety) and separation anxiety- fear of

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being left by mother or caregiver. Another emotion that becomes much common in toddler hood is anger. Babies also laugh and smile more quickly and more selectively.

Personality Development.

Personality has to do with the way a person behaves. It refers to the person's moods, emotions, actions and responses. When referring to children, we talk of **temperament.** It refers to the basic dispositions inherent in the person that underlie and govern the expression of activity,. Reactivity, emotionality and sociability. Many aspects of temperament are innate (the result of genetic & prenatal influences). As the person develops in the social context, the individual's experiences increasingly influence the nature and expression of temperament. For example, we shape the way a child expresses himself by discouraging some habits. Studies have shown that babies are active, they kick a lot in the uterus before.

a. **Activity Level:** Some babies are active ,they kick a lot in the uterus before they are born. They move around as toddlers and are always nearly running and others are less active.
b. **Rhythimicity:** Some babies have regular cycles of activity e.g. they eat, sleep and defecate at regular periods, others do not.
c. **Approach-withdrawal:** Some babies are delighted by every new thing. Others withdraw from nay new situation. E.g. the first bath makes some babies laugh and other cry.
d. **Adaptability**: Some babies adapt quickly to change; others are unhappy at every disruption of their normal routine.
e. **Intensity of reaction**: Some babies are calm and others seem to react intensely.
f. **Threshold of responsiveness**: Some babies seem to sense every sight, sound and touch. E.g they wake up at a slight noise, turn away from a distant light. Others are unaware of bright lights loud noises and wet diapers (nappies).
g. **Quality of mood:** Some babies seem constantly happy, smiling at almost everything. Others are constantly unhappy- they are ready to complain at any moment.
h. **Distractibility:** Some babies are easily distracted from an interest while others are hard to distract –they are single murdered.
i. **Attention Span:** Some babies play happily with one toy for a considerable period of time. Others quickly drop an activity for another. The attention span varies among children.

Factors that influence emotional development.

Research has documented the role of the following factors in emotional development:

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- **The child's temperament:** The inborn personality characteristics of a child determine how the child will express and control his/her feelings.
- **Quality of care giving:** The nature of care giving provided to a child shapes how the child feels, the emotional states he/she develops. Nurturing and gentle caregivers foster the development of the positive emotional states while inconsistent, indifferent and aggressive caregivers may stimulate the development of negative emotions more than positive ones.
- **Health status of the child and caregivers:** A child that is consistently ill will not often pay attention to or may become overly sensitive to emotional expressiveness of others. Caregivers that are unwell may also be extreme in the way they express feelings. This may impact the emotional stability and development of those around them, particularly young children.
- **Parental expectations and beliefs:** Parents beliefs and expectations about how emotions should be expressed determine how children show their feelings. Culture suggests the appropriate level and ways for expressing emotional states and specific emotions. These shape the parents expectations and beliefs about emotional; expressions. Sometimes there are differences in the parents view of how a girl child should express their anger, happiness, sorrow or curiosity. In some cultural groups, a boy child is not expected to express certain emotions openly. Boys for example are not encouraged to express emotions by crying. Crying is considered feminine and consequently girls are encouraged to express emotions freely by crying.

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LESSON ELEVEN: MORAL AND SPIRITUAL DEVELOPMENT

Introduction

As children develop their general thinking skills, they are expected to start to conform to the morality rules that society dictates. Psychological theories of moral development differ immensely in their opinions on how moral development occurs. Freud claims that the quality of relationship the child has with his/her parent/s greatly affects the way the child develops morally. On the other hand, the Social learning theory states that children initially learn how to behave morally through observing and imitating significant adult's behaviour. Cognitive development theories claim that a child's ability to reason morally depends on his/her general thinking abilities. Different theories help us to understand moral development.

Theories of moral development The Social Learning Theory

According to social learning theorists (such as Albert Bandura), children initially learn how to behave morally through observing and imitating adults in the environment, a process referred to as modeling. Children adopt behaviours that are exhibited by significant people in the environment such as caregivers, parents and elder siblings. Social learning theorists believe that children's moral development can be assisted by providing appropriate behaviour models that children could learn from.

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The Cognitive Developmental Theory

Cognitive developmental theories argue that a child's ability to reason morally depends on his/her level of general thinking abilities. Piaget assessed children's moral development using two methods: 1) He presented them moral issues to them and then analysed their responses to them and 2) He observed the way in children used rules in a game. He concluded that children's way of looking at and treating rules changed with age. Thus: ▪ Preschoolers play for enjoyment and disregard rules.

- Six year olds consider rules to be sacred and inflexible.
- Ten year olds realize that rules can be manipulated.

According to Piaget, children differ in response to moral dilemmas depending on their stage of moral development.

Stages of moral development

Moral realism: Children younger than ten years of age determine whether an act is good or bad depending on how much damage it causes. They do not put into consideration the motives or intentions behind the action.

Moral relativism: In this stage, children take into account the motives behind an action when making judgements about an action. They no longer believe that every wrong action should necessarily be punished. If for example, a girl breaks her mother's cup, she should only be punished if she intentionally broke the cup.

Kohlberg presented moral dilemmas to older children. He would for instance ask: "Should a man steal an overpriced drug that he can't afford, to save his wife from cancer?" in order to analyse their moral reasoning (development). Using this method, Kohlberg concluded that there were three distinct levels of moral reasoning.

- **The pre-conventional level**: At this level, the child's actions are primarily motivated by the desire to gain a reward or to avoid punishment.
- **The conventional level:** A child's moral decisions are largely influenced by society. The child is motivated by the desire to obey the rules and laws of the society. At this stage the child thinks that rules can not be changed.

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- **The post-conventional level:** The child is able to determine which rules can be bent, but the child also considers some values such as human dignity to be sacred.

Children's development of the understanding moral and pre-social development

The child's understanding of moral and pre-social development increases with age. The following table shows how this understanding changes with age.

1 – 2 years: Children help others when they are upset by providing physical comfort.
2 – 3 years: Starts to show guilt for undesired behaviour.

3 – 6 years: Actions are motivated in order to gain a reward or to avoid punishment.

7 – 11 years: Children's actions are primarily motivated by what other people in society will think.

11 – 15 years: Judges moral dilemmas according to the intentions of the perpetrator.

16 years and above: Reasons on the basis of internal principles.

Foundations of moral development

At this age children are considered moral; they have no knowledge of right and wrong and do not behave according to rules. However, this is not completely true.

A. Moral Development In The First Three Years

Children during this age respond to differences in tone of parent's voices that suggest certain actions are "good" and others are "bad" those that are accepted and those that are not. They may even stop behaviour if a parent says "No" with a sharp tone or they may attempt to test their limits and express their autonomy by continuing the behaviour.

It is important for the caregiver to give reasons for the "No". These reasons should refer to the social and emotional consequences of the action on others. The children will begin to internalize these ideas and develop norms of behaviour.

If explanations for the "No" are not given, a child will only stop the behaviour because of fear of being seen and thus being punished. They may develop the idea that the behaviours are only bad because of the when one is caught or seen doing it. The behaviours are not thought of as bad because of their consequences to the child that does it.

B. Factors that Impact Moral Development

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Culture: Culture often determines moral beliefs, culture may dictate what is considered right or wrong. Consequently, people in different cultures may have quite different moral beliefs. For example, some cultures allow to call parents by their names while others don't.

Media: Media has a strong influence on children's moral development. Some prosocial and antisocial behaviours are transmitted to children via the media. A good example of such behaviours is violence. Some television broadcast in Kenya and elsewhere contains violence. Young children are more susceptible to violence because they do not fully understand the link between actions and consequences. These children also find it difficult to separate real life from television. That exposure to television violence results in more frequent aggressive acts by children. This is evident in research. The more violent television a child watches, the more violent he/she becomes.

Temperament: Research suggests that child temperament is a valid variable which affects child development. This has implications for discipline strategies.

Parenting: House (1997) found that positive attention and communication practices between parents decrease the likelihood of child aggression towards peers. Also harsh punishment and criticism from parents result in increased aggression. To assist early child moral development, it is suggested (Kochanska 1995) that parents should adapt their strategies of child rearing to fit their children.

Temperamentally inhibited children respond to gentle reasoning because this develops their conscience.
Fearless impulsive children require a strong attachment bond with the care-giver.

C.

1. Presence of parents and other caregivers to act as role models of moral behaviours.
2. Caregiver's talking to the child about right and wrong behaviours.
3. Caregivers consistently directing and redirecting the child to right behaviours. This suggests that no punishment of wrong behaviours occurs, but the child is redirected to and then reinforced for right behaviours.

Foundations of spiritual development

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The development of a personal relationship with the higher power (God) occurs over years. Virtues are not known nor is knowledge of the spiritual power developed until after this age group. However, these are the foundation years in which the child begins on the journey of spiritual development. At this age they come to know that there is "someone" bigger than their parents/caregivers who are very important.

A. Extent of spiritual development before three years

In addition to developing an understanding norms of social behaviour children under 3 years are developing awareness of a greater power, a higher person (God). As caregivers demonstrate by their actions the existence of a higher power. Children imitate the actions and eventually develop some basic spiritual awareness. It caregivers pray or spend time reading the bible. Koran or other religious materials children see and are likely to imitate the actions. At this age they will not develop spiritual values, but by age five they will have internalized the spiritual values their caregivers have demonstrated in their behaviours.

B. Factors that Impact Spiritual Development Some of the factors include:

1. Presence of significant caregivers to act as role models for the child to observe and imitate in terms of prayer and other forms of devotion.
2. Caregiver's talking to the child about the higher power (God).
3. Caregiver relating daily actions of the child and others to the wishes of the higher power (God).

Activity

a) What is the role of mothers and caregivers in moral development?

b) How can mothers and caregivers lay foundation for infant attachment?

Summary

a) The session covered theories of infant attachment, social and emotional development
b) Some explanation on theories on attachment and relationship and caregivers and its roles in infant attachment development.

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LESSON TWELVE: COGNITIVE DEVELOPMENT - PIAGET'S SENSORI-MOTOR SUBSTAGES.

Objectives

By the end of the session, you should be able to:-

- ***i) Explain the theories of moral development***
- ***ii) Describe factors that impact on moral development***
- ***iii) Explain cognitive development of an infant***
- ***iv) analyze the stages of moral development***
- ***v) Explain spiritual development in an infant.***

Piaget began to describe the differences in the ways infants, preschoolers, school-age children, and adolescents understand their environments. At each of these ages, different internal structures are operating. The internal

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organizations that infants exhibit are limited. Unlike adults, infants are not constantly thinking about their environment. Their knowledge is based on actions. These actions become organized into patterns of behaviours called schemes. For older children and adults, schemes are like concepts, but for these infants they are organizations of behaviours.

Cognitive growth is the result of the infant actively exploring the environment and developing more organized schemes. During the first two years. Piaget outlines six different milestones based on the child's rapidly changing schemes.

Stage 1: Exercise reflexes and built-in patterns of behavior: (Birth to 1 month):

The newborn's beginning equipment, as we have seen, is primarily a set of reflexes and some loosely patterned behaviours. These reflexes and beginning sensory skills constitute the initial organization of the child. During the first month, through the dual processes of assimilation and accommodation, these initial skills become more stable, more useful, and more organized. After perfecting the sucking reflexes, the infant may suck fingers, blankets, stuffed animals and his parent's shoulders. The same is true of the "looking" reflex. In the first week, the infant's eyes are not always coordinated. By exercising the eyes, the pattern of behavior that is called "looking" becomes well established. During active alert states, the infant does a great deal of looking. Even the grasping reflex changes during the first month of life and is gradually transformed into a grasping scheme.

Stage 2: Extending reflexes (1 to 4 months): Before infants can really understand the world, they must coordinate the information that is being gathered through the schemes of looking, grasping, or sucking. Although this coordination takes time, the beginning of this stage, the infant notices the association between some action and a consequence. For example, Laurent's hand somehow gets into his mouth. When he loses contact with his hand, he tries again to get his hand in to his mouth. His arms, instead of gesticulating aimlessly, constantly move towards his mouth. There is no longer any doubt that coordination exists. His right hand may be seen approaching his mouth. But as only the index finger was grasped, the hand fell out again. Shortly after, it returned. This time the thumb was in the mouth. I then remove the hand and place it near his waist. After a few minutes, the lips move and the hand approaches them again. (Piaget, 1952, p. 52-53)

The importance of the second stage of sensory motor development is that the schemes we saw in the first stage no longer exist in isolation. The infant begins to relate these behaviours Laurent's struggle to get his hand in to his mouth illustrates the gradual changes that take place in the sucking and grasping schemes so that ultimately infants can direct their hand to their mouth quickly and effectively. Laurent is practicing two schemes-sucking with his

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mouth and swiping with his hands. Initially, these two actions are isolated is established. Now he can manoeuvre his hands into his mouth. This new action reflects the infant's revised organization or structure.

In addition to coordination, we see that the infant takes a more directive role in exploring. Looking is one scheme that demonstrates this increased control. When looking was merely an exercise, it was not particularly directed. Now Laurent explores by looking.

In this second stage of the sensory motor period, there is a definite change from the reflexive behavior of the first stage. The skills that the child had at birth have been practiced and organized into higher level schemes that involve two senses. More importantly, infants have much control over these behaviours.

Stage 3: Intergrating senses (4 to 10 months): Each stage builds on the one before, and comparing behaviours in two adjacent stage makes the limits of the earlier stage and the progress of the later stage clear. In this period, all the senses become fully coordinated. Not only can infants get their thumbs into their mouths, they can see or hear an object, direct their arm to it, grab it, and put it in their mouths. The child has a more flexible and more sophisticated organization to guide the exploration of the environment. In this stage, the intentions and goals that were missing before become more evident. The child observes an action, then tries to reproduce it. For example, rhythmical kicking causes some toys attached.

The combination of fully coordinated schemes and the ability to carry out simple goals helps the child explore everyday objects. Towards the end of this stage, infants understand that objects exist independently of them. This realization, fitting called the object concept, is rally a monumental accomplishment because it signals the beginning of symbolic processes. To understand that an object is permanent and still exists when it cannot be seen or heard or felt means that the infant has developed some way to imagine or remember the object. Acquiring an object concept signifies a transition from relying on actions and simple schemes to representing the world in the mind. Hence, we need to examine the object concept in some detail. To form an object concept, infants must first understand that an object is the sum of their expressions. Mother is no longer an isolated voice, a particular face, or a pleasant as she was in Stage 1. Now these schemes are integrated, the child forms a multifaceted representation of mother. It takes many hours of contact with an object and a lot of active exploration to understand all of its facets. Infants gave this kind of contact with their mothers. Hence, it is not surprising that the first object that infants seem to understand as a collection of different pieces of information is their mother.

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Stage 4: Coordination of schemes: 10 to 12 months. During this stage, infants begin to understand other objects in the same way they under-stand their parents' permanence. Prior to developing an object concept, an infant seems to think that any change in the object makes it a different objects.

Another experiment called the disappearing toy illustrates the child's tenous understanding of objects. This is an easy experiment to conduct with infants in the 6- to 11 month age range: Older children seem to be able to hold an image of the toy in memory long enough to conduct a search. They have developed a concept of an object that endures even when the object disappears.

There might be another interpretation for this change in behavior. It could be that the six-month-old does not have the ability to remove the towel. If this case, it is a replay of the flea experiment mentioned earlier. Psychologists are including that the infants cannot hold an object in memory, but that is not really the problem. It might be that the towel is too big for the infant to manage. To check this possibility, Gratch covered the toy with a see-through cover. The hide and seek technique has been used to explore facets of the child's memory. Some researchers have tried to fool the children, they hide a set of keys while the children watch.

The development of an object concept is a landmarks in the development of memory. As infants develop more expensive with objects, they can represent objects in their minds. This representation is the basis of later symbolic behavior. Once infants understand that objects and people exist independently, they can engage in much more thoughtful behaviours. An infant can plan actions to accomplish more complex goals. Laurent demonstrates his ability to coordinate different schemes in the following vignettes:

As in stage 3, the child is behaving intentionally and has a clear goal in mind. But the coordination of schemes that is required in Stage 4 is quite different. The behaviours that Laurent must combine now to use his coordinated schemes to get beyond obstacles. This takes thought.

Stage 5: Search for novelty (12 to 18 months): Infants who have a concept of an object and can remember objects, who can plan a course of action, and who are mobile are well prepared to investigate their world. They begin their exploration in earnest during the fifth stage. Using their full repertoire of coordinated schemes such as throwing, dropping, rubbing and banging, toddlers scrutinize every facet of their environment. Infant explores like to watch tissues come out of the box one at a time.

Jonas Langer, a Piagetian scholar, believes that the action sequences produced during stage 5 are the roots of abstract logic. He points out that people sort the world into categories on an abstract level. Objects are identified by color, mass, weight, density, volume and friction. The logic that underlies these categories can be seen in action sequences. Infants learn that when they drop different objects, some float downward, others bounce, while still others break.

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Their action sequences help them form rudimentary classes that will later be transformed into a symbolic classification system.

Stage 6: Beginning of though (18 to 24 months): The emphasis through the fifth stage of sensory motor developed is on action. Although we can see the beginnings of representation in the child's memory of objects, the child still relies on manipulation. However, during the sixth stage, the role of action begins to recede and we see thoughtful assessments assume more importance. The ability to remember something that was not present was acknowledged with the object concept. At that point, the child's ability to reconstruct objects was very limited. Now infants not only reconstruct images of people and objects that are absent, they can manipulate these images in their mind. As a result, they can think about problems. The experimenter of stage 5 gives way to the philosopher of stage 6. During this final period of sensory motor development, infants, who now know a great deal about the environment, begin to use their information to solve problems.

In reviewing the changes in the child's conception of the world newborns exercise their reflexes and builtin patterns of behaviour until they gradually gain control over their eyes, hands and mouth. With few more weeks of effort, these isolated patterns become coordinated into action schemes. Exploring objects- by seeing them, touching them, and sucking on them – helps infants realize that the people and objects- by seeing them, touching them, and sucking on them-helps infants realize that the people and objects in the environment have an independent existence.

Once children understand that objects have a permanence of their own, they can begin to represent these objects.

Now hide-and-seek games reveal the child's conceptual understanding of the world. Equipped with intergrated senses and the ability to remember-at least for short periods of time- the infant investigates the environment with gusto. At the culmination of the sensory motor period, the infant is able to devise new approaches to problems by manipulating the world symbolically rather than physically.

Piaget's observations, first published in 1936, have stood the test of time. Almost 50 years later, this description still provides an accurate overview of the changes in the infant's thought patterns. The coordination of schemes, the formation o early memories, the development of the object concept, and the relationship between language and thought are all new areas of research that have grown out of Piaget's work with infants have been conducted in the areas of research that have grown out of Piaget's description of the sensory motor period. In addition, extensions of Piaget's work with infants have been conducted in the areas of vision, memory, learning and concept development. With these extensions of Piaget's ideas, our understanding of an infant's cognitive development has become much more elaborate and detailed.

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Activity

a) ***Explain the role a mother or caregiver in moral development***

b) ***Describe the significance of the levels of moral development of an infant.***

Summary

The session covered theories of moral development.

It analyzed the factors that impact on moral development were examined and also foundations of spiritual development

CHILD DEVELOPMENT II

Credit hours: 3hrs

Pre-requisites: ECD 102

Purpose

To understand middle, late and adolescent psychology

Course content

Review of early and middle childhood development, late childhood (adolescence), explore achievement and major milestones in: physical, mental, language, socio-emotional and spiritual.

Theories of development, development of creativity and critical thinking, factors influencing creativity, transitional issues among teenagers.

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Adolescent and adult psychology, definition of personality, theories of personality development, factors influencing personality, attitudes, emotions and motivation, categories of personality, personality assessment.

Teaching/learning methodologies

Group discussions: lecturing, individual assignments, micro-teaching.

Instructional materials and equipment

Chalk board, overhead projectors.

Course assessment

Examination – 70%, continuous assessments (exercises and tests) – 30%; Total – 100%

MOUNT KENYA UNIVERSITY

SCHOOL OF EDUICATION COURSE OUTLINE

UNIT CODE: BEC 123 UNIT TITLE: CHILD GROWTH & DEVELOPMENT II LESSON ONE: EARLY CHILDHOOD

- Cognitive domain
- How personalities develop
- Socialization and play
- Moral development during childhood
- Gender identity
- Spiritual growth
- Action research in early childhood development
- Perspectives on child development

LESSON TWO: MIDDLE CHILDHOOD

- Physical and cognitive domains
- Socio-emotional domain
- Issues and challenges in childhood
- Looking through the eyes of the system of support
- Spiritual growth

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- Research lessons and issues
- Perspectives on child development

LESSON THREE: LATE CHILDHOOD (ADOLESCENCE)

- Developmental changes during adolescence
- Looking through the eyes of an adolescent
- Physical development during adolescence
- Cognitive development during adolescence
- Socio-emotional development during adolescence
- Looking through the eyes of the systems of support
- Spiritual growth
- Action research in late childhood development
- Child development theories

LESSON FOUR: CHILDHOOD DEVELOPMENT THEORIES

- Sigmund Freud
- Erik Erikson
- Cognitive Child Development Theories
- Behavioral Child Development Theories
- Social Child Development Theories
- John Bowlby
- Albert Bandura
- Lev Vygotsky

LESSON FIVE: ADULTHOOD

- Development in middle and early adulthood
- Development in late adulthood
- Theories of successful ageing
- Physical changes
- Cognitive changes
- Death and dying

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- Darling's spiritual growth paradigm

LESSON ONE: EARLY CHILDHOOD

Lesson Overview

We have termed this stage in the developmental process of humans, the "Play Years", because this aspect, as much as any other, seems to characterize the most important context in which development occurs. While we examine the physical development during this stage that moves from an emphasis on the Gross and McIlveen to the fine motor skills, we consider in greater detail the cognitive aspect of development as the child begins to think about its world. We examine three perspectives on the cognitive development of children including representation of experiences, learning and language development. In addition, we also consider the emotional and personality development of children particularly in play situations, in contexts ranging from the home to daycare and pre-school. **Lesson Objectives**

Knowledge

- To understand biological changes during early childhood.
- To understand cognitive changes during early childhood.
- To understand socio-emotional changes during early childhood.

Teacher Information

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The roles nature and nurture play in the development of a young child

The template Nature/Nurture and Human Development depicts the interactions between heredity, the environment and the organism. Note how heredity and experience combine to influence the organism. The next level of interaction happens between the current organism and the current situation that results in the current behaviour. This behaviour, in turn, influences the genetic dispositions and preferences of the organism and the cycle or process is repeated.

How the various domains influence development during early childhood

Physical development

- The average child grows six centimeters in height and gains two to three kilograms a year during early childhood. Growth patterns vary individually though. By age five, the brain has reached ninetenths of its adult size. Some of its size increase is due to the number and size of nerve endings. Gross motor skills increase dramatically during early childhood. Young children are more active than at any other period in the lifespan. Fine motor skills also increase substantially during early childhood (Santrock, 1999, p. 261).

Cognitive development

- Piaget's stage of pre-operational thought is the beginning of the ability to reconstruct at the level of thought what has been established in behaviour and a transition from primitive to more sophisticated use of symbols. The child does yet think in an operational way. Pre-operational thought consists of two sub stages: symbolic function (two to four years) and intuitive thought (four to seven years).

  The child's attention improves dramatically during the early childhood years, as doe's short-term memory. Young children develop a curiosity about the nature of the human mind. Advances in language development also occur during early childhood. Vygotsky's theory emphasizes the zone of proximal development, the merging of language and thought from three to seven years of age and the socio-cultural contexts of cognitive development. Child-centered kindergarten and developmentally appropriate education are important dimensions of early childhood development, as are quality Head Start programs. A current concern is that too many preschool and early childhood education programs place too much emphasis on academic achievement (Santrock, 1999, p. 261).

Socio-emotional development

- Authoritative parenting is associated with children's social competence. Parents need to adapt their strategies as the child grows older using less physical manipulation and more reasoning.

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Crosscultural and ethnic variations occur. In some cases, siblings are stronger socialization agents than parents. Children live in changing families; more children today grow up in working mother and divorced families. Depression in parents is associated with adjustment problems in children. Peers are powerful socializing agents who provide a source of information and social comparison outside the family. Play also is an important aspect of the young child's development. Parten year developed a number of categories of social play. Among the most important types of children's play is sensor motor practice play, pretence/symbolic play, social play, constructive play, and games. Television is another socializing influence in children's development. Children watch a huge amount of television; preschool children watch an average of four hours a day. A special concern is the television violence children see. Erikson believes that early childhood is a period when the self involves resolving the conflict between initiatives versus guilt. In early childhood, the physical and active self becomes a part of self-understanding. Gender identity is the sense of being male or female which most children acquire by three years of age. Identification, social learning, cognitive development and gender-schema theories have been proposed to explain children's gender development. Peers are especially adept at rewarding gender-appropriate behaviour. Piaget distinguished between the heteronymous morality of younger children and the autonomous morality of older children. In addition to moral thought, moral behaviour and moral feelings are important dimensions of children's moral development (Santrock, 1999, p.
261).

The socio-cultural influences on child development

- Vygotsky's portrait of children's development suggests that children actively construct their knowledge, and that it is inseparable from social and cultural activities. In Vygotsky's theory, knowledge is situated and distributed among people and environments which include objects, artifacts, tools, books, and the communities in which people live, and collaborate, advanced through interaction with others in cooperative activities (Gross and McIlveen, 1998, p. 365).

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Teacher Information

How the brain develops during early childhood

One of the most important physical developments during early childhood is the continuing development of the brain and nervous system. While the brain continues to grow in early childhood, it does not grow as rapidly as in infancy. By the time children have reached three years of age, the brain is three-quarters of its adult size. By age five the brain has reached about nine-tenths of its adult size. Part of the increase in brain weight is due to continued proliferation of communication pathways (via the growth of dendrites and axons) among the brain's various specialized areas in response to the child's specific experiences. Another part of brain growth is due to ongoing myelination - the insulating process that speeds up the transmission of neural impulses. Finally, several areas of the brain undergo notable expansion, in particular those areas dedicated to control and coordination of the body, the emotions, and thinking processes.

How important nutrition is to healthy and proper childhood development

With their growth slowed, children need fewer calories per pound of body weight during the preschool years than they did from birth through toddler hood - especially if they are modern, sedentary children who spend most of their time indoors. As a result, their appetites become markedly smaller, a fact that causes many parents to fret, threaten, and cajole to get their children to eat more. A related problem is that many children like most adults, eat too few fruits and vegetables and consume too much fat. No more than 30 percent of daily calories should come from fat, but six out of seven preschoolers in the United States exceed that limit. Interestingly, one North American study found that both children whose family income is below the poverty level and children whose family income is three times above it are more likely to exceed the 30 percent fat limit, compared to those whose income lies somewhere in between.

Early Childhood Development: Cognitive domain

Perhaps more so than at any other stage of development, early childhood is a time of phenomenal cognitive growth and development, highlighted in particular by language development. It may take several class periods to cover all the information presented on this LESSON

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Lesson Objectives

- The developmental changes in children's cognitive abilities
- The process of language development in early childhood
- The socio-cultural environment influence cognitive development
- How the theories of Piaget and Vygotsky compare
- The influence does culture have on cognitive development during early childhood

The process of language development in early childhood

The social learning view suggests that speech is acquired through a combination of conditioning and imitation. Presumably, children are praised or otherwise rewarded by their parents for making sounds approximating those of their native language. In addition, parents often model sounds, words or sentences for them.

A sharply different view has been proposed by linguist Noam Chomsky (1968): the innate mechanism view. According to Chomsky, language acquisition is at least partly innate. Human beings, he contends, have a language acquisition device – a built-in neural system that provides them with an intuitive grasp of grammar.

Cognitive theory recognizes the importance of both innate mechanisms and learning. This theory suggests that children possess certain information-processing abilities or strategies that they use in acquiring language.

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The method of discipline that works

There are several forms of discipline for helping children as they grow into socially capable people:

- *Spanking*. While most parents have experienced the urge to spank children, and many have resorted to spanking their kids, there are problems with this form of discipline. Spanking may give children a clear message about the unacceptability of their behaviour and sometimes stops the behaviour in the short run. However, in the long run, it teaches children that it is all right to hit, and that it is all right to be hit. Even children are confused by the irony of the statement,

  "This spanking will teach you not to hit your brother."

- *Time-outs*. Giving children a short time-out can give them the chance to reflect on their behaviour, and also a clear message that a certain behaviour would not be allowed. This can be an effective method of teaching. Time-outs can also give an angry, frustrated parent a chance to calm down and respond more rationally. It is suggested that a child be given a time-out equal in minutes to her age (a four-minute time out for a four-year-old child). It is not recommended to use time-outs with children under the age of three. The problem with time-outs is that they take a child away from a valuable learning experience. A child who hits another child can begin to

  learn empathy from watching the other's child's response to being hurt, and if he stays around, he may also be able to participate in helping the other child feel better.

- *Talking with children*. Talking with children offers rich opportunities for learning alternatives to hitting. It is often necessary to give children some specific suggestions about what they can say when they have strong feelings. Sometimes the suggestion, "Use your words," doesn't give children enough of an idea of what they could say.

- *Help children come up with alternative solutions*. What children usually want when they take toys from each other is a chance to play together. If you can offer them suggestions for other ways to

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play together, they may be able to let their conflict go. "It looks like Jeffy wants to play trucks with you. Can you find him a truck he could use?" Or, "Jeffy looks really interested in what you are doing with your truck. Could he help you make a road for the truck?"

- *Set limits and follow through*. It is important that children know that hitting is unacceptable. If your child is unable to stop himself from hitting his brothers after you have offered alternative ways for him to express his feelings and communicate his ideas, you can offer him a final choice. "Can you be safe with Jeffy or shall I help you move to the other room to play away from Jeffy until you can be safe with him?"
- *Remember that it takes time, repetition and modeling*. Children do not learn communication and problem-solving skills quickly. It takes time and repetition in many different circumstances before they really get it. If you stay focused on teaching your children these skills and gently model the ways you want them to interact, slowly they will begin to acquire the skills (Davis and Keyser, 1997, pp. 218-239).

The Self and Personality

This lesson addresses how young children develop from a basic temperament, to an increasing selfawareness and self-concept, to the beginnings of personality. **Teacher Information**

How, and when, young children develop a sense of self

Toward the end of the second year of life children develop a sense of self. During early childhood, some important developments in the self take place. Among these developments are facing the issue of initiative versus guilt and enhanced self-understanding (Alcock et al., 1998, p. 69).

How personalities develop

The *psychodynamic approach* as advanced by Sigmund Freud, proposed that personality and behaviour are determined more by psychological factors than by biological conditions or current events. He suggested that people may not know why they think, feel or act the way they do because they are partly

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controlled by the unconscious portion of the personality - the part of which people are normally unaware (Bernstein and Nash, 1999, p. 408).

The *trait approach* to personality makes three main assumptions:

Personality traits remain relatively stable and therefore predictable over time.

Personality traits remain relatively stable across diverse situations, and they can explain why people act in predictable ways in many different settings.

People differ with regard to how much of a particular personality trait they possess; no two people are exactly alike on all traits. The result is an endless variety of unique personalities (Bernstein and Nash, 1999, p. 413).

A *cognitive-behavioural approach* views personality as a set of behaviours that people acquire through learning and then display in particular situations. It defines personality as the sum total of the behaviours and cognitive habits that develop as people learn through experience in the social world (Bernstein and Nash, 1999, p. 421).

The *phenomenological (humanistic) approach* maintains that the primary human motivator is an innate drive toward personal growth that prompts people to fulfill their unique and natural potential. Like the planted seed that naturally becomes a flower, people are inclined towards goodness, creativity, love and joy. Proponents of this theory emphasise that each individual perceives reality somewhat differently and that these differences, rather than traits, instincts or learning experiences, are central to creating personality differences (Bernstein and Nash, 1999, p. 424).

What aspects of a child's world influence personality development?

Biology

- Every individual is born with a distinct, genetically-based set of psychological tendencies, or dispositions. These tendencies, which together are called temperament, affect and shape virtually every aspect of the individual's developing personality. Temperament, and therefore personality, is not merely genetic. It begins in the multitude of genetic instructions that guide the development of the brain and then is affected by the prenatal environment (Berger, 2000, p. 219).

Birth order

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- Refer to birth order chart, Lesson Objectives and Teacher Information, Lesson 3.5 Infancy Development: socio-emotional domain. **Temperament**
  - Researchers who study personality have found what are called the "big five" dimensions of temperament:
  - *Extroversion*: the tendency to be outgoing, assertive, and active
  - *Agreeableness*: the tendency to be kind, helpful, and easygoing
  - *Conscientiousness*: the tendency to be organized, deliberate, and conforming
  - *Neuroticism*: the tendency to be anxious, moody, and self-punishing
  - *Openness*: the tendency to be imaginative, curious, artistic and welcoming of new experiences (Baron et al., 1998, p. 336).

Culture

- In every culture, preschool education not only includes but goes beyond cognitive preparation for later schooling. As an example, Japanese culture places great emphasis on social consensus and conformity. Therefore, Japan's preschools provide training in the behaviour and attitudes appropriate for group activity; children are encouraged to show concern for others and to contribute cooperatively in group activities. These social attitudes and habits prepare young children for both the formal school system and later work settings. In China, similarly, learning how to be part of the group is combined with creativity in self-expression, both drawn from the culture's Confucian ethic of disciplined study. In the United Sates, by contrast, preschools are often designed to foster selfconfidence and self-reliance and to give children a good academic start through emphasis on language skills. Since most North American preschools are private, they vary a great deal in rules, curriculum and values (Berger, 2000, p. 296).

Parenting

- Refer to Curriculum Support Materials, Memorandum from your child.

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Family

- Children's emotional health is closely related to the emotional relationship between their parents. When these relationships are warm and constructive, such that the husband and wife feel loved, admired, and encouraged to act in ways that they themselves admired, the children are happy and healthy. Couples who are emotionally close, meeting each other's needs and encouraging positive self-images in each other, become good parents. Since they meet each other's needs, they do not use their children to live out their needs; since they are happy and satisfied, they can support and meet their children's needs; and since their own identities are clarified, they see their children as distinct from themselves. All this helps the children become emotionally healthy people (Schlesinger, 1998, p.8).

Economics

- No matter how maltreatment is defined or counted, it occurs more frequently as family income falls. This particularly true for neglect and physical abuse, which fall most heavily on children under age six who live in families with an income below the poverty line, an unemployed father, and four or more children. In such families, children obviously add to the financial pressures and are likely to become victims because of it (Berger, 2000, p. 302).

Maltreatment

- The more we learn about child maltreatment, the more we see that its causes are many and its consequences extend far beyond any immediate injury or deprivation. Compared to well-caredfor children, chronically abused and neglected children tend to be underweight, slower to talk, less able to concentrate, and delayed in academic growth. Deficits are even more apparent in social skills; maltreated children tend to regard other children and adults as hostile and exploitative, and hence they are less friendly, more aggressive, and more isolated than other children. The longer their abuse continues, and the earlier it started, the worse their relationships with peers are. As adolescents and adults, those who were severely maltreated in childhood (physically or emotionally) often use drugs or alcohol to numb their emotions, choose unsupportive relationships, sabotage their own careers, eat too much or too little, and generally engage in self-destructive behaviour (Berger, 2000, p. 258).

Self-concept

- Infants are not "given" a self by their parents or the culture. Rather, they find and construct selves. It is generally believed that awareness of "the self" develops from early experience. Indeed, one of the first

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stages in the development of thinking in the child is the capacity to distinguish what is “me” and what is “not me” (Alcock et al., 1998, p. 69).

Emotional regulation

- The most important emotional development during early childhood is not the emergence of new emotions such as pride or guilt, but the growing ability to inhibit, enhance, maintain and modulate emotional arousal to accomplish one’s goals. This ability, called emotional regulation, is developed in response to society's expectations that preschoolers manage frustration and modulate emotional expression (Berger, 2000, p. 308).

Attachment and care-giving

- The results of past care-giving are clearly exhibited by children’s reactions when another child cries in pain. Children who have been well nurtured and have formed secure attachments are able to regulate their own emotions and can express empathy, comforting the hurting child, reassuring the frightened child, or getting help if need be. Longitudinal research finds that those with insecure attachments respond abnormally to other children's distress (Berger, 2000, p. 309).

Social behaviour

- One of the most important functions of the peer group is to provide a source of information and comparison about the world outside. Children receive feedback about their abilities from their peer group. Good peer relations may be necessary for normal socio-emotional development.

  Social isolation, or the inability to “plug in” to a social network, is linked with many problems and disorders ranging from delinquency and problem drinking to depression.

Media

- According to Neilson Media Research, in 1996 children between the ages of two to five in the United States watched 23 hours and 21 minutes of television each week. This is more than three hours a day, and it is at least three hours more a week than the viewing time of any other age group. Among the criticisms of television are the time it takes away from active, interactive and imaginative play; the faulty nutritional messages it sends; and the sexist, racist, and ageist stereotypes it provides that are particularly harmful for inexperienced, vulnerable viewers. Prosocial behaviour depends on emotional regulation, which is best learned through active social relationships. Television, passive

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observation, undercuts the very attributes, skills and values that lead to pro-social activity. However, the most compelling and convincing criticism of television concerns the antisocial behaviour it encourages, especially in children. The effect is interactive and cumulative; children who watch a lot of television are likely to be more aggressive than children who do not, and children who are already inclined to be aggressive are likely to watch a lot of violence. Television desensitizes children to violence in real life, making physical aggression seem normal. For all these reasons, children who watch substantial quantities of violent television are more likely than others to be bullies, more likely to retaliate physically for any perceived attack, more likely to be passive victims, and more likely to be passive onlookers rather than mediators when other children fight (Berger, 2000, p. 313).

Socialization and Play

Early childhood is often referred to as the "play years". This stage of development establishes the foundation for all social behaviours and social interactions. It is, literally, the first step in determining, Who I Am.

Lesson • Why children play

Objectives • The different kinds of play behaviours

• How parents can encourage nonviolent play

Why children play

Play is pleasurable activity that is engaged in for its own sake. Play is essential to the young child's health. Play increases affiliation with peers, releases tension, advances cognitive development, increases exploration and provides a safe haven in which to engage in potentially dangerous behaviour.

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- For Freud and Erikson, play is an essential, useful form of human adjustment, helping the child master anxieties and conflicts. Because tensions are relieved in play, the child can cope with life's problems.
- Piaget believes that play advances children's cognitive development. Play permits children to practise their competencies and acquired skills in a relaxed, pleasurable way.
- Vygotsky also believes that play is an excellent setting for cognitive development, especially the symbolic and make-believe aspects of play, as when a child substitutes a stick for a horse and rides the stick as if it were a horse.
- Play encourages exploratory behaviour by offering children the possibilities of novelty, complexity, uncertainty, surprise and incongruity (Santrock, 1999, p. 240).

The different kinds of play behaviours

Mildred Parten (1932) developed the following classification of children's play:

- *Unoccupied play* occurs when the child is not engaging in play as it is commonly understood. The child may stand in one spot, look around the room, or perform random movements that do not seem to have a goal.
- *Solitary play* occurs when the child plays alone and independently of others. Two- and three- yearolds engage more frequently in solitary play than older preschoolers do.
- *Onlooker play* occurs when the child watches other children play. The child's active interest in other children's play distinguishes onlooker play from unoccupied play.
- *Parallel play* occurs when the child plays separately from others, but with toys like those the others are using or in a manner that mimics their play.
- Associative play occurs when play involves social interaction with little or no organization. In this type of play children seem to be more interested in each other than in the tasks they are performing.

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- Cooperative play involves social interaction in a group with a sense of group identity and organized activity. Little cooperative play is seen in the preschool years (Santrock, 1999, p. 241).

Parten's categories represent one way of thinking about the different types of play. However, other types of play are important in children's development. Whereas Parten's categories emphasize the role of play in the child's social world, the contemporary perspective on play emphasizes both the cognitive and social aspects of play.

- *Sensorimotor play* is behaviour engaged in by infants to derive pleasure from exercising their existing sensor motor schemas. Infants initially engage in exploratory and playful visual and motor transactions in the second quarter of the first year of life. By nine months of age, infants begin to select novel objects for exploration and play, especially objects that are responsive such as toys that make noise or bounce. By 12 months of age, infants enjoy making things work and exploring cause and effect. At this point in development, children like toys that perform when they act on them.
- *Pretense/Symbolic play*. Between nine- and 30 months of age, children increase their use of objects in symbolic play. They learn to transform objects - substituting them for other objects and acting toward them as if they were these other objects. Dramatic play or "make-believe" often appears at about 18 months of age and reaches a peak at about four- or five- years of age, then gradually declines. In the second year, infants begin to understand the social meaning of objects. For example, two-year-olds may distinguish between exploratory play that is interesting but not humorous, and “playful” play which has incongruous and humorous dimensions.
- *Social play* is play that involves social interactions with peers. Parten's categories are oriented towards social play.

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- *Constructive play* combines sensorimotor/practice repetitive play with symbolic representation of ideas. Constructive play occurs when children engage in self-regulated creation or construction of a product or a problem solution.
- *Games* are activities engaged in for pleasure. They include rules and often competition with one or more individuals (Santrock, 1999, p. 241).

How parents can encourage non-violent play

Four-year-olds are busy trying to master their fears, understand mortality, and gain a sense of power in a big, often frightening world. These factors, which naturally converge in four- and five-year-olds, lead to a fascination with guns. The fact that your son is fascinated with guns right now does not mean that he will grow up to be a violent person. Rather, it means he is developmentally on target, trying to understand the power of guns, what makes people get hurt, and what makes them die, why everyone on TV is so obsessed with guns and killing, and why his parents get so upset when he points his finger at someone and pretends to shoot them.

Families have a wide range of responses to gun play. At one end of the spectrum, kids are equipped with realistic Uzi imitations, and at the other, gun play is banned altogether. Yet even in households where toy guns are not allowed, children still find ways to explore their interest in weapons; they build guns out of Legos, pick up sticks and say, "bang! Bang!" or eat their peanut butter sandwiches in the shape of a gun.

Here are some guidelines for responding to your son's fascination with guns while encouraging nonviolence:

- Set parameters for the play. When we forbid a form of play, that play often goes underground and we lose an opportunity to help our children figure out answers to their questions. Make it clear to children that any play which intimidates hurts or frightens another child is unacceptable. Then clarify the rules about guns at your house: "In our family, you are not allowed to point guns at any people or pets without their permission." Or, "I want you to keep your gun play outside."
- Provide props for play that have multiple uses. Giving children open-ended props encourages flexibility and creativity. When a child makes a gun out of plastic blocks, ten minutes later, those plastic blocks

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can be transformed into a container ship. However, when children play with highly realistic guns, those guns can only shoot and kill.

- Make your values clear. Children care deeply about how we see the world. Share your perspective with your son, "When I see you playing with guns, it really upsets me because people can be hurt by real guns."
- Help children deal with their fears. Often gun play increases when children are feeling fearful. Children look for symbols of power (such as guns and swords) to help them feel more secure. Helping children deal with their fears can take some of the intensity out of their gun play.
- Let your kids know that people are working to stop violence in the real world. Kids sometimes use gun play to cope with fears about violence in the real world. If your child seems scared of real violence, talk about people who are working to stop fighting and end war. This can help kids feel safer and therefore, less driven to engage in gun play.
- Provide kids with alternatives to gun play that help them feel powerful. Four- and five-Year-olds love to be competent. Providing them with real work experiences such as carpentry, cooking and gardening can channel some of the energy being directed into gun play in a new, more creative direction (Davis and Keyser, 1997, p. 339).

Moral development during childhood

Moral development begins in early childhood. This lesson describes the theories of moral development, knowing what is "right" and "wrong", and the processes by which children learn basic values.

Lesson	• How children develop moral behaviour
Objectives	• How children learn values

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How children develop moral behaviour Theories of

moral development:

- *Social learning* theorists argue that moral behaviors are acquired through classical and operant conditioning. We resist temptation because we have been reinforced for so doing and punished for transgressions. Social learning theorists also recognize the importance of modeling or observational learning. The development of self-control is strongly influenced by models and by patterns of direct reinforcement children encounter, that is, adults' disciplinary measures.
- **Cognitive developmental**

  - Kohlberg's theory states that the child is in the first level of moral development (preconventional morality) characterized by:

    - Stage 1: Punishment and obedience orientation, what is right or wrong is determined by what is punishable and what is not.
    - Stage 2: What is right and wrong is determined by what brings rewards and what people want.

  - Piaget's Theory

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- Heteronomous morality is the first stage of moral development in Piaget's theory, occurring from approximately four to seven years of age. Justice and rules are conceived of as unchangeable properties of the world, removed from the control of people. The heteronomous thinker judges the rightness or goodness of behaviour by considering the consequences of behaviour, not the intentions of the actor. For example, the heteronomous thinker says that breaking twelve cups accidentally is worse than breaking one cup intentionally while trying to steal a cookie (Gross and McIlveen, 1998, p. 382).

How children learn values

Every interaction with children provides an opportunity to teach values. While no parent tries to make every kiss goodnight a lesson, it is useful to think about the opportune times for teaching in families.

- Children learn about our values through daily interactions with us. When we think about teaching values to kids, we often think about taking them to church or having a talk with them about lying, teaching them about sharing or encouraging them to give during the holiday season. Yet we teach values every day in our ordinary daily encounters.
- Children learn through our example.
- Children learn through the values we strive towards. While it is true that children learn through what we model, it is <u>not</u> true that you need to have mastered a value before you teach it to your children. All of us have some values that are woven into the very fabric of who we are. At the same time, most of us have values we are newly adopting, that we haven't practiced or integrated. Even if we move toward our values in tiny increments, children will pick up on our intention and commitment, and learn that they, too, can strive toward a vision they haven't yet attained.
- Children learn values through the way we do things as a family.

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- Children learn values and beliefs through their exposure to the larger world. Through friends, extended family, books, TV and the experiences they have in their community, children absorb values and societal norms.
- Children learn values through our explanations of the world (Davis and Keyser, 1997, p. 6).

Gender Identity

Gender, by definition, refers to the state of "femaleness" or "maleness". This lesson addresses how young children develop a sense of gender identity and gender roles.

Lesson Objectives

- How a gender identity develops
- The differences between the genders
- The socio-cultural influences on gender development

How a gender identity develops

Even at age two, gender related preferences and play patterns are apparent. Children already know whether they are boys or girls, can identify adult strangers as mommies or daddies, and apply gender labels (Mrs., Mr., lady, man) consistently. That simple cognitive awareness becomes, by age three, a rudimentary understanding that male and female distinctions are life-long. By age four, children are convinced that certain toys are appropriate for one gender but not the other. Four year-olds criticize peers who choose toys that are not appropriate for their gender and are proud of themselves when they act in gender-typical ways. When given a choice, children play with other children of their own sex, a tendency apparent at age two and clear-cut by age five. By age six, children have well formed ideas

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(and prejudices) about sex differences and also know which sex is better (their own) and which sex is stupid (the other). Stereotypes and taboos are also evident in fantasy play, where most of the other restrictions of daily life disappear (Berger, 2000, p. 326).

The differences between the genders

Genuine behavioural differences do exist between the sexes and people's stereotypes are not entirely inaccurate. But the differences are fewer in number, smaller in size and far more complex than stereotypes suggest.

- *Social behaviour and personality*: Studies indicate that males tend to be more aggressive than females, both verbally and physically. This disparity shows up in early childhood.
- *Aggression*: One of the most consistent gender differences is that boys are more aggressive than girls. Another is that boys are more active than girls. The aggression difference is especially pronounced when children are provoked. These differences occur across cultures and appear very early in children's development. Biological factors include heredity and hormones. Environmental factors include cultural expectations, adult and peer models, and social agents who reward aggression in males and punish aggression in females
- *Emotional control*: An important skill is to be able to regulate and control your emotions and behaviour. Males usually show less self-regulation than females, and this low self-control can translate into behavioural problems. In one study, children's low self-regulation was linked with greater aggression, teasing others, overreaction to frustration, low cooperation and inability to delay gratification (Santrock, 1999, p. 318).

The socio-cultural influences on gender development

Although parents do encourage “sex-appropriate” play, there is evidence that biological factors may play an initial role in children's preferences. Although fathers are less likely to give dolls to one-yearold boys than to one-year-old girls, the boys
who do receive the dolls are less likely to play with them (Snow, Jacklin and Maccoby, 1983). Perhaps adult expectations and encouragement build upon children's preferences, producing an amplifying effect.

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Then, because boys' toys provide more opportunity for developing motor skills, visuo-spatial skills, and inventiveness, and girls' toys provide more opportunity for nurturance and social exchange, some important differences in sex roles may become established.

Once children begin to play with other children outside the home, peers have a significant influence on the development of their gender roles. In fact, Stern and Karraker (1989) found that the behaviour of two- to six-year-old children was even more influenced by the knowledge of a baby's gender than was the behaviour of adults. By the time children are three years old, they reinforce gender-typed play by praising, imitating or joining in the behaviour. In contrast, they criticize gender-inappropriate behaviour (Langlois and Downs, 1980). Parents indirectly encourage gender-stereotyped play by seeking out children of the same sex as playmates for their own children (Buskist at al., 1997, p. 399).

Looking through the eyes of the system of supports

This lesson addresses the various roles and functions of the support systems that nurture and promote healthy development of the young child. If students come from a different culture, inviting them or their parents to share their experiences of parenting and child rearing would be most valuable.

Lesson How the socio-cultural context influence the development of a young child **Objectives**

Teacher Information

The influence does the media have on early childhood development

According to Neilson Media Research, in 1996 children between the ages of two and five in the United States watched 23 hours and 21 minutes of television each week. This is more than three hours a day, and it is at least three hours more a week than the viewing time of any other age group. Among the criticisms of television are the time it takes away from active, interactive and imaginative play; the faulty nutritional messages it sends; and the sexist, racist, and ageist stereotypes it provides that are particularly harmful for inexperienced, vulnerable viewers. Pro-social behaviour depends on emotional regulation, which is best learned through active social relationships. Television, passive observation, undercuts the very attributes, skills and values that lead to pro-social activity. However, the most compelling and convincing criticism

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of television concerns the antisocial behaviour it encourages, especially in children. The effect is interactive and cumulative; children who watch a lot of television are likely to be more aggressive than children who do not, and children who are already inclined to be aggressive are likely to watch a lot of violence. Television desensitizes children to violence in real life, making physical aggression seem normal. For all these reasons, children who watch substantial quantities of violent television are more likely than others to be bullies, more likely to retaliate physically for any perceived attack, more likely to be passive victims, and more likely to be passive onlookers rather than mediators when other children fight (Berger, 2000, p. 313).

The influence does economic status and conditions have on early childhood development

No matter how maltreatment is defined or counted, it occurs more frequently as family income falls. This particularly true for neglect and physical abuse, which fall most heavily on children under age six who live in families with an income below the poverty line, an unemployed father, and four or more children. In such families, children obviously add to the financial pressures and are likely to become victims because of it (Berger, 2000, p. 256).

The effects doe's social class has on parenting and child rearing

In most Western countries, social class differences in child rearing have been found. Working class and low-income parents often place a high value on external characteristics such as obedience and neatness. By contrast, middle-class families often place a high value on internal characteristics such as self-control and delay of gratification. There are social class differences not only in child rearing values but also in parenting behaviours. Middle-class parents are more likely to explain something, use verbal praise, use reasoning to accompany their discipline and ask their children questions. By contrast, parents in lowincome and working class households are more likely to discipline their children with physical punishment and criticize their children more (Santrock, 1999, p. 232).

Spiritual Growth

Children are more likely to become adults who identify with their cultural and spiritual heritage if they grow up feeling included in cultural and spiritual communities. Parents can take an active role in shaping children's cultural and spiritual life by providing them with frequent opportunities to interact with peers and adults of all ages (e.g., extended family members, friends, neighbors, and school staff) who share children's heritage or belief system, or who can otherwise function as cultural role models. Young children

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who see others practicing the same customs and rituals as they do themselves will feel a sense of belonging and pride.

Young children are very concrete. They learn through their direct experience and through observation of other's behavior, and not by way of abstract discussion. It is of little use to explain the philosophical, historical or religious basis for a given cultural or spiritual practice to young children. Instead, children will best learn particular practices by participating in them. Religious or traditional cultural festivals, holidays, and rituals are perfect teaching opportunities, as are opportunities to teach children about traditional foods and dress. Children's participation in holidays and rituals and cultural activities teaches them that they are valued members of a particular community. Such memories form the ongoing basis of children's cultural and religious identity, and become the building blocks of tradition.

Besides immersing children in the ongoing practice of cultural and religious traditions, parents can also foster children's appreciation of their heritage through celebratory games and crafts. For instance, children may enjoy making books about historical family members or filling out a family tree so as to learn about grandma's mother and father. Children can also be encouraged to make scrapbooks documenting their participation in family occasions, and special cultural or religious celebrations. Filling the pages of homemade books with special photographs, drawings, labels, and short stories will help children create cherished keepsakes that reinforce their familial, cultural and religious identity.
Making family movies or videos can achieve a similar effect.

Though learning about one's own culture is paramount, it is also important to teach children about other people's cultures. Neighbors, family friends, and school peers can be recruited to help teach small children about other cultural practices including music, language, food and dress (if applicable). Exposing children to other cultures at an early age helps develop their tolerance and appreciation of cultural differences and diversity; skills they will need to navigate the complex and culturally interdependent modern world.

According to Froebel, the end of education should not be reason, per se, but rather the unfolding of the divine essence within the child. He thus urged educators to respect the spontaneous and essentially creative nature of this unfolding (Miller, 1997). In his view, the role of education “is to build on the

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'living core' of the child's intrinsic spiritual capacities" (Dillon, 2000, p. 11).

Pestalozzi also advocated respect for the unique nature of each child and felt that the end or purpose of education was to foster the holistic development of the students. Education, he said, was not intended to make children into people "such as we are" (Miller, 1997). Rather the role of the teacher was to guide children to become the humans their natures required them to be – including what was divine and sacred in their nature (Miller, 1997). Pestalozzi also believed in the innate goodness and wisdom of the human being and felt that the focus of education should not be so much to learn something as to be something. As expressed by Miller (1997), Pestalozzi "was more concerned with the needs and characteristics of the learner than with the requirements of the subject matter" (p. 94).

Montessori also believed in a spiritual force that guides human development (Miller, 1997). She referred to this force as "an individual spiritual embryo" (Montessori, 1972, p. 109). According to Montessori, children will seek growth and development because that is consistent with their nature. She urged educators to respect this spiritual force and to follow the inclinations or nature of the child.

According to Montessori, the direction of a child's life is contained within his own soul (Miller, 1997). In her writings for educators, Montessori once said, "Our care of the child should be governed, not by the desire 'to make him learn things,' but by the endeavor always to keep burning within him that light which is called the intelligence" (Montessori, 1965, p. 240). Montessori believed that the first six years are especially crucial to the holistic and spiritual development of the child. During this time, she said, "The young child literally 'incarnates' the world around him. The things he sees are not just remembered; they form part of his soul" (Montessori, 1973, p. 63). The poet, Walt Whitman, expressed something similar when he said, "There was a child went forth every day; and the first object he looked upon, that object he became" (2001).

Rudolph Steiner also saw human development as an unfolding from within and believed that human nature is comprised of body (our physical being), soul (our personal inner life), and spirit (ultimate being). Steiner believed that while the role of education is to foster growth in all areas of human development, its primary focus should be on the unfolding of the individual's spirit (Miller, 1997, p. 171). Steiner's views are exemplified in Waldorf Schools where education "is not a pedagogical system but an Art – the Art of awakening what is actually there within the human being. Fundamentally, the Waldorf School does not want to educate, but to awaken" (Steiner, 1967, p. 23).

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Action research in early childhood development

This summary lesson will integrate all of the concepts covered in this unit, by actively engaging students in the design and conduct of research in developmental psychology

Lesson The issues and LESSONs for research in early childhood development **Objectives**

Suggested Review/Discussion: The scientific method of research

Instructional Refer to the template supplied in the Curriculum Support Materials.

Strategies Review and discuss the steps in the scientific method of research:- Develop a research question

- Direct
- Independent
- Interactive
- Indirect
- Experiential

- Describe the hypothesis
- Select a research method

Observation Experiment Interview Case Study Survey LESSON - Describe the process to be followed

- Gather the data
- Analyze the data
- Report the findings, account for reliability and validity
- State conclusions in relation to the hypothesis

- Discussion: A comparison of research methods in the social sciences
- Using the template supplied in the Curriculum Support Materials, complete the comparison chart of research methods.

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- Invite students, or groups, to defend its choice of the most appropriate research method based on its chosen LESSON.
- Discuss the many formats for presentation of the data and research findings. Depending on the LESSON chosen and the research method selected, presentation formats may include written reports, oral presentations, portfolios, role plays and drama activities, three-panel displays, posters, brochures, bulletin board displays, Power Point or other computer-based presentation software, web pages, etc.
- Review/Discussion: Issues in social science research
- Using the six templates supplied in the Curriculum Support Materials (Frequently Asked Questions about interviews, surveys, case studies, observation, experiments and LESSON research methods), review and discuss the issues and methods for each individual or group research project.
- Discussion: The research process
- Ensure that each individual or group research project has completed all documentation (The Scientific Method of Research, Permission to Conduct Research) before commencing the research.

Resources

- Curriculum Support Materials: Scientific Method of Research
- Curriculum Support Materials: Permission to Conduct Research Form
- Curriculum Support Materials: A Comparison Chart For Research Methods
- Curriculum Support Materials: Frequently Asked Questions: Interviews, Surveys, Case Studies, Naturalistic Observation, Experiments and LESSON

Lesson : Suggested issues and research methods

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Type of Research	Suggested Research LESSONs
Survey Conducting research using a survey involves going out and asking questions about the phenomenon of interest.	• Should parents be legally permitted to spank their children? • Are you afraid that television values are replacing family values? • Should the government be launching a national daycare program paid for by taxpayers? • Should parents be held accountable in some way for the behaviour of their children? • Should parental discipline of young children include spanking?
Naturalistic Observation In naturalistic research, the observer does not intervene at all. For all intents and	• Can you find evidence of Piaget's preoperational stage of development in young children? • Can you find evidence to support

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purposes, the researcher is invisible and works hard not to interrupt the natural dynamics of the situation being investigated.	Vygotsky's concept of scaffolding and the zone of proximal in young children's cognitive development? • How is playground safety for young children demonstrated? • What is the process of language development in young children? • How are basic human values developed and promoted in young children? • What is the process of language development in young children? • How is self-awareness demonstrated by young children?

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LESSON ▯ A LESSON research study involves the acquisition, synthesis, organization, and presentation of information. Typically, the LESSON research study will involve both paper based as well as web-based resources. Additional information may be gained through the other research methods and strategies.	▯ What is the nutritional value of fast food for young children?
Interview ▯ The interview method of research, typically, involves a face-to-face meeting in which a researcher (interviewer) asks an individual a series of questions.	• Are there basic temperament and personality traits in young children? • Are there gender differences in aggressive behaviour? • How is moral behaviour developed and promoted in young children? • Is there an awareness of gender role differences in young children?

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	• Are there gender differences in terms of aggression and emotional control in young children? • How are genders represented on television and in young children's books?
Case study A case study is an intensive study of one individual. Typically, the case study is based on interviews with the subject regarding his or her background, present thinking or actions; it may also utilize interviews of others who know the individual. Additional case study material may be obtained through observation,	• Children • values • impressions of media • mental schemas • play behaviours • toys
experiments and standardized tests, such as personality inventories and intelligence tests.	

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Experimental research	Research one of the following LESSONs:
Experimental researchers take care to create an environment in which they can make causal statements. They manipulate variables, randomly assign participants to various conditions, and seek to control other influences that could affect their research.	• Piaget's pre-operational stage of development • Vygotsky's concept of scaffolding and the zone of proximal development • Language development

Perspectives on child development

In this summary lesson, we consider how the lifespan approach to human development can be used as a lens through which we can view early childhood development.

How child development is described from the perspective of the lifespan approach

The lifespan perspective on human development has seven basic characteristics. Development is:

- Life-long
- No age period dominates development.
- Multi-dimensional
- Development consists of biological, cognitive, socio-emotional and spiritual dimensions.
- Multi-directional
- Some aspects of development increase, while others decrease.
- Plastic
- Depending on the individual's life conditions, development may take many paths.
- Historically-embedded

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- Development is influenced by historical conditions.
- Multidisciplinary
- Psychologists, sociologists, anthropologists, neuroscientists and medical researchers all study human development and share a concern for unlocking the mysteries of development throughout the lifespan.
- Contextual
- The individual continually responds to and acts on contexts, which include a person's biological makeup, physical environment, and social, historical, and cultural contexts (Santrock, 1999, p. 10).

How do the key issues and questions in developmental psychology relate to child development?

From the time of its establishment, several key issues and questions have dominated the field of developmental psychology. Among these issues are the nature of developmental change, the importance of critical periods, lifespan approaches versus the more focused approaches, and the nature-nurture issue.

- Continuous change versus discontinuous change: In continuous change, developmental change is gradual, with achievements at one level building on those of previous levels. In contrast, discontinuous change occurs in distinct stages or steps. Each stage brings about behaviour that is assumed to be qualitatively different from behaviour at earlier stages.
- A critical period is a specific time during development when a particular event has its greatest consequences. Critical periods occur when the presence of certain kinds of environmental stimuli are necessary for development to proceed normally.
- Lifespan approaches versus a focus on a particular period: Developmentalists now believe the entire lifespan is important, for several reasons. One is the discovery that developmental growth and change continue during every part of life. Furthermore, to understand fully the social influences on people of a given age, we need to understand the people who are, in large measure, providing those influences.

  For instance, to understand development in infants, we need to unravel the effects of their parents'

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ages on the social environment.

- Nature versus nurture: One of the enduring questions of development involves how much of people's behaviour is due to their genetically-determined nature and how much is due to nurture, the physical and social environment in which a child is raised. In this context, nature refers to traits, abilities and capacities that are inherited from one's parents. Nature encompasses any factor that is produced by the predetermined unfolding of genetic information - a process known as maturation. These genetic, inherited influences are at work as we move from the one-celled organism that is created at the moment of conception to the billions of cells that make up a fully-formed human being. In contrast nurture refers to the environmental influences that shape behaviour. Some of these influences may be biological, such as the impact of a pregnant mother's substance abuse on the fetus, or the amount and kind of food available to children. Other environmental influences are more social, such as the ways parents discipline their children and the effects of peer pressure on adolescents (Feldman, 2000, p. 10).

LESSON TWO: MIDDLE CHILDHOOD

Introduction

In middle and late childhood, children are on a different plane, belonging to a generation and feeling all their own. It is the wisdom of the human lifespan that at no time are children more ready to learn than during the period of expansive imagination at the end of early childhood. Children develop a sense of wanting to make things, and not just to make them, but make them well and even perfectly. Their thirst is to know and to understand. They are remarkable for their intelligence and for their curiosity. Their parents continue to be important influences in their lives, but their growth is also shaped by successive choirs of friends. They do not think much about the future or the past, but they enjoy the present moment.

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Learning Objectives

- To understand physical changes during childhood.
- To understand cognitive changes during childhood.
- To understand socio-emotional changes during childhood.

The roles do nature and nurture play in the development of a child

Nature refers to the changes in development as a result of the influence of the genetic code humans inherit from their parents. Nurture refers to the influences of nutrition, culture, the mass media, peers, stressors, families and parenting, in effect, the socio-cultural context that surrounds the developing person.

The socio-cultural influences on child development

At the Microsystems level, discuss with the students the relative influences of parents, peers and the classroom setting. In what ways do these aspects of the system of support influence our physical, emotional, cognitive and spiritual development? Do parents, peers or the classroom, dominate during childhood?

At the ecosystem level, discuss the relative influences of the school, the mass media (refer to LESSON 6.5.7 for more information) and the community. What school policies are in place that promotes healthy development? How has changing technology influenced child development? What community resources are available to promote healthy child development?

At the macro system level, discuss the influence that economic status and culture, in particular, have on child development. Are children as sensitive to the pressures of status as adolescents? How do different cultures (individualistic, collectivist and Aboriginal) view childhood?

Looking through the eyes of a child.

This overview lesson addresses the physical, mental, emotional and spiritual aspects of development during childhood.

The physical changes during childhood

During the elementary school years, children grow an average of five or eight centimetres a year. Muscle mass and strength gradually increase. Legs lengthen and trunks slim down. Growth is slow and

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consistent. Motor development becomes smoother and more coordinated. Boys are usually better at gross motor skills, girls at fine motor skills. Our nation's children are not getting enough exercise. Sociocultural factors, such as poverty, can place considerable stress on children. One of children's important buffers against stress is the long-term presence of a basic trusting relationship with at least one adult. A readily available support network is also important. A special concern involves children with disabilities, including children with a learning disability or attention-deficit hyperactivity disorder.

The cognitive changes during childhood

According to Piaget, the seven- to 11-year-old child's cognitive development is characterized by concrete operational thought, which involves operations. Concrete operations are mental actions that are reversible. Children's long-term memory, metacognitive knowledge, and cognitive monitoring improve during childhood. It is important for children to engage in critical thinking. Children's creativity should be encouraged. During middle and late childhood, children become more analytical and logical in their approach to words and grammar. Reading is a more central aspect of language in the childhood years

The socio-emotional changes during childhood

Parents spend less time with their children in childhood, but parents are still very important socializing agents in this period. New parent-child issues emerge, and discipline changes. In childhood, two major changes in many children's lives are movement into a stepfamily and becoming a latchkey child. Children spend considerably more time with peers in this period. In peer relations, children can be popular, average, rejected, neglected, or controversial. Friendships become more important. The internal self, the social self and the socially comparative self become more prominent in selfunderstanding during this period. Perspective-taking increases during this period. Self-concept and self-esteem are important dimensions of the child's socio-emotional development during this period. Gender is an important aspect of elementary school children's development, especially gender stereotypes, similarities and differences, gender-role classification, and ethnicity and gender.

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Childhood Development: Physical and cognitive domains

The lifespan period from ages six–12 is a time of slow, consistent physical growth before the onset of puberty.

Lesson Objectives

- The physical developmental changes during childhood
- The process of cognitive development during childhood

The physical developmental changes during middle childhood

During the elementary school years, children grow an average of five or eight centimetres a year. Muscle mass and strength gradually increase. Legs lengthen and trunks slim down. Growth is slow and consistent. Motor development becomes smoother and more coordinated. Boys are usually better at gross motor skills, girls at fine motor skills. Our nation's children are not getting enough exercise. Sociocultural factors, such as poverty, can place considerable stress on children. One of children's important buffers against stress is the long-term presence of a basic trusting relationship with at least one adult. A readily available support network is also important. A special concern involves children with disabilities, including children with a learning disability or attention-deficit hyperactivity disorder.

What is the process of cognitive development during childhood?

A nine- or 10-year-old child is a very different kind of thinker than say, a four- or five-year-old preschooler. Not only do older children know more, they also use their minds much better when they must solve a problem or remember a piece of information. By middle childhood, most children have acquired a sense of "the game of thinking" and they begin to enjoy an intellectual challenge as much as an athletic one. They begin to realize that good thinking involves considerable evidence, planning ahead,

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thinking logically, formulating alternative hypotheses, and being consistent. This is the Piagetian stage of concrete operational thought. They try to incorporate these qualities into their own reasoning and use them to evaluate the thinking of others. A marked improvement in memory occurs between ages seven and 11, particularly apparent in older children's ability to remember essential facts over a period of days or longer, with no forgetting. Selective attention, increases in processing speed and the development of thinking patterns are all factors behind the improvement.

Childhood Development: socio-emotional domain

As children grow and mature, the nature of their social relationships, their concept of self, and their gender identity and awareness all increase profoundly. This lesson addresses all aspects of the socioemotional domain.

Lesson Objectives

- The process of socio-emotional development in childhood
- How children develop a sense of self
- How children develop a sense of moral and ethical behaviour
- The influence do friendships have on healthy socio-emotional development in childhood
- How gender influences development during childhood

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The process of socio-emotional development in childhood

Erikson's fourth stage of the human lifespan, industry versus inferiority, appears during childhood. The term industry expresses a dominant theme of this period: Children become interested in how things are made and how they work. Children's social worlds beyond their families also contribute to a sense of industry. School becomes especially important in this regard.

Erikson agrees with Freud that childhood is a quiet period emotionally, a period in which 'the child becomes ready to apply himself to given skills and tasks." During Erikson's crisis of industry versus inferiority, children busily try to master whatever their culture values. On the basis of their degree of success, they judge themselves as either industrious or inferior or, in other words, competent or incompetent, productive or failing, winners or losers.

How children develop a sense of self

In childhood, self-understanding increasingly shifts from defining oneself through external characteristics to defining oneself through internal characteristics such as preferences, personality traits and qualities. In addition to the increase of psychological characteristics in self-definition, the social aspects of the self also increase at this point in development. Children's self-understanding also includes increasing reference to social comparisons. Many develop mentalists believe that perspectivetaking plays an important role in self-understanding. Perspective-taking is the ability to assume another person's perspective and understand his or her thoughts and feelings.

How children develop a sense of moral and ethical behaviour

According to developmental psychologist Lawrence Kohlberg, people pass through a series of stages in the evolution of their sense of justice and in the kind of moral reasoning they use to make moral judgments. Kohlberg suggests that moral development can best be understood within the context of a three-level sequence, which is further subdivided into six stages. At the lowest level, preconventional morality, people follow unvarying rules based on rewards and punishments. In the next level, that of conventional morality, people approach moral problems in terms of their own position as good, responsible members

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of society. Finally, individuals use postconventional morality to invoke universal moral principles that are considered broader than the rules of the particular society in which they live. Kohlberg's theory proposes that people move through the stages in a fixed order and that they are unable to reach the highest stage until adolescence.

Psychologist Carol Gilligan has suggested an alternative account of the development of moral behaviour in girls. She suggests that differences in the way boys and girls are raised in our society lead to basic distinctions in how men and women view moral behaviour. According to Gilligan, boys view morality primarily in terms of broad principles such as fairness and justice, whereas girls see it in terms of the responsibility towards individuals and willingness to sacrifice themselves to help specific individuals within the context of particular relationships. Compassion for individuals, then, is a more prominent factor in moral behaviour for women than it is for men.

The influence do friendships have on healthy socio-emotional development in childhood

Children's friendships are vitally important. They serve six functions: companionship, stimulation, physical support, ego support, social comparison and intimacy/affection. Two of friendships most common characteristics are intimacy and similarity. Intimacy in friendships refers to self-disclosure and the sharing of private thoughts. True intimate friendships may not appear until early adolescence. Also, through childhood, friends are more similar than dissimilar in terms of age, sex, race, and many other factors. Friends also play important roles in shaping children's and adolescents' well-being and development. In terms of well-being, all people have a number of basic social needs. These include the need for tenderness (secure attachment), playful companionship, social acceptance, intimacy, and sexual relations. If the need for social acceptance is not met, we suffer a lower sense of self-worth.

How gender influences development during childhood

Gender is a pervasive aspect of an individual's identity. Three areas of socio-emotional development where gender similarities and differences have been studied extensively are communication styles, social relationships, and aggression.

Sociolinguist Deborah Tannen (1990) distinguishes between rapport talk and report talk. Rapport talk is the language of conversation and a way of establishing connections and negotiating relationships. Report talk is talk that gives information. Males hold centre stage through report talk with such verbal performances as storytelling, joking and lecturing with information. By contrast, females prefer private, rapport talk and conversation that is relationship-oriented. Tannen says that boys and girls grow up in

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different worlds of talk – parents, siblings, peers, teachers and others talk to boys and girls differently. The play of boys and girls is also different. Boys tend to play in large groups that are hierarchically structured, and their groups usually have a leader who tells the others what to do and how to do it. Boys' games have winners and losers and often are the subject of arguments. Boys often boast of their skill and argue about who is best at what. By contrast, girls are more likely to play in small groups or pairs, and the centre of a girl's world is often a best friend. In girls' friendships and peer groups, intimacy is pervasive.

Turn-taking is more characteristic of girls' games than of boys' games. And much of the time, girls simply like to sit and talk with each other, concerned more about being liked by others than jockeying for status in some obvious way.

One of the most consistent gender differences is that boys are more aggressive than girls. Another is that boys are more active than girls. The aggression difference is especially pronounced when children are provoked. These differences occur across cultures and appear very early in children's development. Both biological and environmental factors have been proposed to account for gender differences in aggression. Biological factors include hormones and heredity. Environmental factors include cultural expectations, adult and peer models, and social agents who reward aggression in boys and punish aggression in girls.

Issues and challenges in childhood

This lesson addresses the issues of middle childhood using a research-oriented focus. Students are directed to analyze the research findings from a theoretical perspective to explain childhood issues.

Lesson Objectives

- How sports influence children
- The influence does exercise have on childhood development
- How learning disabilities influence childhood
- The effect Attention Deficit Hyperactivity Disorder (ADHD) has on children
- The role and nature of aggression in children
- How children become obese

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- Who bullies are, and what effects they have on other people • The effect does divorce have on children

How sports influence children

Participation in sports can have both positive and negative consequences for children. Children's participation in sports can provide exercise, opportunities to learn how to compete, increased selfesteem, and a setting for developing peer relations and friendships. However, sports can also have negative outcomes for children: too much pressure to achieve and win physical injuries, a distraction from academic work, and unrealistic expectations for success as an athlete.

The influence does exercise has on childhood development

Many of our patterns of health and illness are long-standing. Our experiences as children contribute to our health practices as adults. In summary, not only do children's school weeks not include adequate physical education classes, but the majority of children do not exercise vigorously even when they are in such classes. Furthermore, most children's parents are poor role models for vigorous physical exercise.

How learning disabilities influence childhood

Characteristics of children with mild, moderate and severe learning disabilities include:

- Mild learning difficulties: These children have some difficulties with normal school work, but can cope with the normal curriculum. If work is differentiated and support is available, their needs can be met.
- Moderate learning difficulties: These children make very limited progress in basic academic skills such as literacy and numeracy, despite support being given. As a result, they need additional help, achieved by a statement of their special educational needs.

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- Severe learning difficulties: These children function at a low level in terms of basic skills. Communication skills and academic attainment are normally very limited.

The effect Attention Deficit Hyperactivity Disorder (ADHD) has on children

It is estimated that from three to five percent of all school-age children in Canada – and elsewhere – suffer from attention deficit hyperactivity disorder. Found in most cultures and in most countries, ADHD is four or five times more common among boys, and it is first noticed, usually, between three and four years of age.

Regardless of gender, ADHD produces the same effects: children are unable to concentrate on any task for more than a few minutes. Moreover, they charge through each day with excessive energy. They talk during quiet periods, ignore social rules, leave their seats, and create disturbances in the classroom.

Basic to all these behaviours, they are moody, and very impulsive.
ADHD children have normal intelligence, but they show deficits in their ability to focus their attention.

They literally cannot seem to sit still, and are readily distracted by irrelevant information.

The role and nature of aggression in children

The specifics regarding when aggression is appropriate, in what forms, and to what degree depend on the particular society in which children grow up. Variation in the norms for aggression occurs by age, by ethnic and socioeconomic group, by neighbourhood, by gender, and by the specific social situation.

Customs vary about which kind of contact (being bumped up against, being shoved, having one's shoe stepped on) require retaliation, which types of insult (directed at one's relatives, one's physical appearance, one's intellect) should be ignored, and which children (friends, enemies, bystanders, the other sex, the physically impaired, smaller or younger ones) are fair targets and which are exempt. Generally, physical aggression declines with age – except for those children who are headed towards more serious violence and criminal activity. Relational aggression – verbal insults and social aggression – does not decline. In girls as well as boys, relational aggression becomes more hurtful, more isolating, and more important to deflect – with humour, a shrug, or a counter-attack when possible.

How children become obese

Obesity usually results from the interaction of a number of factors. Here are the most important of them:

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- Heredity. Body type, including height, bone structure, and the amount and distribution of fat on the body, is inherited. So are individual differences in metabolic rate and activity level. Certain combinations of body type and metabolism result in excessive storage of fat on the body.
- Exercise. Inactive people burn fewer calories and are more likely to be overweight than active people, especially in infancy and childhood, when many children seem to be on the move all day. A child's activity level is influenced not only by heredity but also by a willingness to engage in strenuous play, the availability of safe play areas, the parents' example, and weight itself, which slows down precisely those children who need more exercise.
- Television. While watching television, children eat more and burn fewer calories than they would if they were actively playing. In fact, they burn fewer calories than they would if they were doing ***nothing***. One study found that when children are "glued to the tube" they fall into a deeply relaxed state, akin to semi consciousness, that lowers their metabolism below their normal at-rest rate – on average, 12 percent lower in children of normal weight and 16 percent lower in obese children. Further, 60 percent of the commercials shown during Saturday morning cartoons on U.S. Television is for food products – almost all of them with high fat and sugar content.
- Cultural attitudes towards food. In some cultures, overeating is a sign of wealth and happiness, so parents urge their offspring to have a second helping. The implied message seems to be that a father's love is measured by how much food he can provide; a mother's love, by how well she can cook; and a child's love by how much she or he can eat.
- Precipitating event. For many children, the onset of obesity is associated with a crucial event or traumatic experience – a hospitalization, a parental divorce or death, or a move to a new neighbourhood. Generally such an event or experience creates a sense of loss or diminished selfesteem, along with a corresponding need for an alternative source of gratification.

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Who bullies are, and effects they have on other people

Researchers define bullying as repeated, systematic efforts to inflict harm through physical attack, verbal attack or social attack. Implicit in this definition is an imbalance of power. Boys who are bullies are often above average in size, whereas girls who are bullies are often above average in verbal assertiveness. Bullies' victims tend to be less assertive and physically weaker (boys) and more shy (girls). Bullying during middle childhood seems to be pervasive. It occurs in every nation, is present in small rural schools and in large urban ones, and is as prevalent among well-to-do majority children as among poor immigrant children. Contrary to public belief, victims are no more likely to be fat or homely or to speak with an accent than non-victims are. But they usually are rejected, withdrawn children, anxious and insecure, unable or unwilling to defend themselves, without friends who will take their side.

Bullied children are anxious, depressed and underachieving during the months and years of their torment. Even years later, they still have damaged self-esteem as well as painful memories.

The picture is even more ominous for bullies. Contrary to the public perception that bullies are actually insecure and lonely, at the peak of their bullying they usually have friends who abet, fear and admire them. They seem brashly unapologetic about the pain they have inflicted. Bullies become more hostile, challenging everyone who tries to stop them, getting into trouble not only with peers but also with the police. International research confirms that children who regularly victimize other children often become violent criminals later on.

The effect divorce has on children

The disruption and discord of divorce almost always adversely affect the children for at least a year or two. Immediately before and after a divorce, the children show signs of emotional pain, such as depression or rebellion, and symptoms of stress, such as having lower school achievement, poorer health and fewer friends. Whether this distress is relatively mild and short-lived, or serious and longlasting, depends primarily on the stability of the child's life and the adequacy of care giving. Divorce may not harm the children if the family income remains stable, if the conflict between the parents decreases, and if care giving by both parents is as good as or better than it was before the divorce.

Looking through the eyes of the systems of support

This lesson addresses the influence that the parents, family and peers have on middle childhood development.

Lesson Objectives

- How the socio-cultural environment influences child development

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The importance is the family in child development

As children move into the middle and late childhood years, parents spend considerably less time with them. In one study, parents spent less than half as much time with their children aged five to 12 in caregiving, instruction, reading, talking, and playing as when their children were younger. Parentchild interactions during childhood include whether children should be made to perform chores, and if so, whether they should be paid for them; how to help children learn to entertain themselves rather than relying on parents for everything; and how to monitor children's lives outside the family in school and peer settings.

The roles and influence do peers have on child development

During middle and late childhood, children spend an increasing amount of time in peer interaction. In one study, children interacted with peers 10 percent of their day at the age of two, 20 percent at age four, and more than 40 percent between the ages of seven and 11. Episodes with peers totalled 299 per typical school day. Researchers have devised five peer status categories to describe peer interactions:

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Popular children are frequently liked by their peers and are frequently nominated as a best friend. They show high rates of positive behaviours and low rates of negative behaviours.

- Average children are moderately liked by their peers and moderately often nominated as a best friend. They show moderate levels of positive behaviours and negative behaviours.
- Rejected children are actively disliked by their peers and infrequently nominated as a best friend. They show high rates of negative behaviours and low rates of positive behaviours.
- Neglected children are not disliked by their peers and are infrequently nominated as a best friend. They show low levels of positive and negative behaviours.
- Controversial children are frequently disliked by their peers but are often nominated as a best friend. They show high rates of both positive and negative behaviours.

Why are children's friendships important?

According to Hartup and Stevens (1997) there are some important developmental consequences to having friends. Those children and adolescents who have friends tend to be more socially competent than those who do not. Having friends provides someone to confide in, to be afraid with, and to grow with. Having friends sets the stage for intimacy with adults.

How to resist peer pressure

Realize that you are being manipulated. Others are deliberately trying to get you to change your mind.

If you are the only holdout, you are more likely to give in. Try to get just one other person on your side, and you will feel much stronger about your position.

People want their behaviour and attitudes to be in harmony. Consequently, even if they know of some negative effects of what they are doing, they probably would not own up to them, because to do so would cause cognitive dissonance. Thus, you should be aware that you are probably not getting the whole story from the group.

For all age groups, peers are more persuasive than other people. The more prestige particular peers have, the greater their influence. Use this tendency to your advantage by finding peers who have had a bad experience with whatever you are being urged to do and listen to what they have to say.

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Minority opinions can have an effect on the majority. One way to increase this effect is to give consistent and repeated statements of your position. Often, this technique has a private effect on others - one that is not publicly expressed.

If all the members of a group are on one side, doesn't that mean that they are more likely to be right than a single, lone individual? Actually, a group's decision making is usually not as accurate as an individual's, particularly if the individual is well informed and highly rational.

When attempting to persuade someone, group members often try to make that person feel embarrassed for not going along. Embarrassing people is an age-old tool for making them conform. If you know that ahead of time, perhaps it will have less impact on you.

One of the best ways to resist conformity is to be exposed to all the arguments in favour of some risky behaviour and then learn to refute these arguments, one by one.

How parents influence moral development

Parental discipline does contribute to children's moral development, but there are other aspects of parenting that also play an important role as well. These include providing opportunities for perspective-taking and modeling appropriate moral behaviour and thinking. Eisenberg and Murphy (1995) summarized the research on ways that parenting can influence children's moral development.

They concluded that in general, moral children tend to have parents who: are warm and supportive rather than punitive

- use inductive reasoning
- provide opportunities for children to learn about others' perspectives and feelings
- involve children in family decision making and in the process of thinking about moral decisions
- Model moral behaviours and thinking themselves and provide opportunities for their children to model such moral behaviours and thinking.

How parents influence pro-social behaviours

What can parents and teachers do to promote children's altruism and prosocial behaviour? Honig and Wittmer (1996) provided the following recommendations that focus on promoting children's prosocial behaviours:

- Value and emphasize consideration of others' needs. This results in children engaging in more helping activities. The morality of caring is one of teaching children to feel for others, which leads to empathy and concern.

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- Model prosocial behaviours. Children imitate what adults do.
- Label and identify prosocial and antisocial behaviours. Be specific in identifying prosocial behaviours.
- Attribute positive social behaviours to each child.
- Notice and positively encourage pro social behaviours but do not overuse external rewards.
- Facilitate perspective-taking and understanding others' feelings.
- Use positive discipline strategies. Reason with children when they do something wrong. Redirect antisocial actions to more acceptable actions.
- Actively lead discussions on pro-social interactions.
- Develop school and class projects that foster altruism.
- Use technology to promote pro-social behaviours.
- Invite moral mentors to visit the class.

The role the school plays in the healthy development of children

Schooling of some sort during childhood is available in every nation. But the specifics – who receives instruction, in what subjects, and how – vary enormously. In the past boys and wealthier children were much more likely to receive formal education than girls and poorer children, and some of that inequality is still evident today. In developing countries, more boys than girls attend elementary school. Indeed, girls are more likely to drop out of school before sixth grade, with the percent of boys and girls ages six - 11 in school ranging from 99 percent to 90 percent in Algeria to 42 percent to 14 percent in Afghanistan. In developed countries, less is generally demanded of girls and poor children, particularly in science and mathematics. Teaching techniques also vary widely, from the strict lecture method in which students are forbidden to talk, whisper or even move during class, to open education, in which students are encouraged to interact and make use of classroom resources – with the teacher serving as an adviser, guide, and friend more than as a subject-matter authority and disciplinarian.

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The role that economics play in the healthy development of children

One of the major advantages of two married parents is financial. Married couples are generally better able to provide for their children, for reasons that predate the child's arrival:

- Individuals who are financially secure are more desirable marriage partners.
  Couples who have good economic prospects are more likely to decide to marry and to have children.
- Couples who are financially secure are more likely to avoid divorce.
- Most contemporary two-parent families have two wage earners, so they tend to be more financially secure than other families.

Once a child is born, two-parent households in which only one parent works save money on childcare and household services for which a single parent might have to pay, and so two-parent households – even those with only one wage earner – have more money for housing, food, health care and education. Thus the general stability of their home and work situations tends to enhance the financial security of nuclear families.

Action research in middle childhood development

This summary lesson will integrate all of the concepts covered in this unit, by actively engaging students in the design and conduct of research in developmental psychology.

Spiritual Growth

Part of the child's development as an individual includes an emerging understanding of the lifecycle—of birth, growth, aging, and death. There is an increasing awareness that life fits into a larger scheme of relationships among individuals, groups of people, other living creatures, and the earth itself. School-age children become keenly interested in these topics, especially when confronted with personal experiences such as the birth of a sibling or the death of a grandparent. As children experience these events and learn to view their personal encounters as part of a larger whole, families and communities provide important structure. They define value systems that provide children with basic principles and encourage them to examine their personal actions in light of their impact on those around them. Children's ability to understand their place in the larger world leads to greater self-esteem and competence. The close link between competence and self-esteem is strengthened when a child is recognized for working hard in school; successfully completing chores and special projects; and participating in school, church, or community activities.

The relationship between values, competence, self-esteem, and personal responsibility needs to be modeled and affirmed by the child's parents, teachers, and community. Parents also need to support maintaining a balance between responsibilities at school and home, time spent with family and friends, extracurricular and community activities, and personal leisure.

Achieving this balance is essential for healthy development.

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Middle childhood is also a time of moral development and spiritual growth. Feldman states that children are now beginning to understand the moral principles behind behavior. Moral decisions are no longer based simply on rewards or punishments. An internal code of ethics begins to emerge, based in large part upon parental guidance. According to a 2009 article titled "The Role of Parents in the Spiritual Development of Children" by Leon Blanchette, parents have a unique opportunity to be spiritual mentors to their children. Parents help to develop a worldview in their children (either healthy or unhealthy), which will guide them in moral and spiritual decisions.

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Research Lessons and Issues

Type of Research	Suggested Research LESSONs
Survey Conducting research using a survey involves going out and asking questions about the phenomenon of interest.	• Who are your heroes? • What qualities do you admire in a friend? • What are your leisure activities? • Moral dilemmas: • Is it right to steal life-saving medicines that could save the life of a family member? • Is it right to tell the police if you know that a friend has committed a crime? • If you found a large sum of money in an unmarked envelope, should you keep it? • What jobs are men (women) most suited for?
Observation In naturalistic research, the observer does not intervene at all. For all intents and purposes, the researcher is invisible and	• What are the eating habits of school-aged children? • How is Erikson's stage of Industry versus Inferiority demonstrated in children?

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	works hard not to interrupt the natural dynamics of the situation being investigated.	• What are the different types of group behaviours in children? • How do the play behaviours of school-age children compare to the play behaviours of young children? • When and how do children develop a sense of gender roles? • What are the most common forms of leisure activities in children? • Are there gender differences in aggression in school-aged children? • How are school-aged children represented on television and in the movies? • Are there gender differences in watching television?

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LESSON A LESSON research study involves the acquisition, synthesis, organization, and presentation of information. Typically, the LESSON research study will involve both paper based as well as web-based resources. Additional information may be	What is Attention Deficit Hyperactive Disorder (ADHD)?
gained through the other research methods and strategies.	
Interview The interview method of research, typically, involves a face-to-face meeting in which a researcher (interviewer) asks an individual a series of questions.	• What was your parent's childhood like? What • are the parenting styles and strategies of your parents? • How does self-awareness develop in children? • What have been the changes in families and parenting since your grandparents were raising their children? • What is it like to teach?

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Experimental research Experimental researchers take care to create an environment in which they can make causal statements. They manipulate variables, randomly assign participants to various conditions, and seek to control other influences that could affect their research.	• Can you find evidence of Piaget's stages of cognitive development in children? • What is the process of morality development in children?

Perspectives on child development

This summary lesson considers how the lifespan approach to human development can be used as a lens through which we can view child development.

Lesson Objectives

How is child development described from the perspective of the lifespan approach?

How do the key issues and questions in developmental psychology relate to child development?

How would each of the six theoretical perspectives on human development explain child development?

How child development described from the perspective of the lifespan approach

The lifespan perspective on human development has seven basic characteristics. Development is:

- Life-long

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- No age period dominates development.
- Multi-dimensional
- Development consists of biological, cognitive, socio-emotional and spiritual dimensions.
- Multidirectional
- Some aspects of development increase, while others decrease.
- Plastic
- Depending on the individual's life conditions, development may take many paths.
- Historically-embedded
- Development is influenced by historical conditions.
- Multidisciplinary
- Psychologists, sociologists, anthropologists, neuroscientists and medical researchers all study human development and share a concern for unlocking the mysteries of development throughout the lifespan
- Contextual
- The individual continually responds to and acts on contexts, which include a person's biological makeup, physical environment, and social, historical, and cultural contexts.

How the key issues and questions in developmental psychology relate to child development

From the time of its establishment, several key issues and questions have dominated the field of developmental psychology. Among these issues are the nature of developmental change, the importance of critical periods, lifespan approaches versus the more focused approaches, and the naturenurture issue.

- **Continuous change versus discontinuous change:** In continuous change, developmental change is gradual, with achievements at one level building on those of previous levels. In contrast, discontinuous change occurs in distinct

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stages or steps. Each stage brings about behaviour that is assumed to be qualitatively different from behaviour at earlier stages.

- A **critical period** is a specific time during development when a particular event has its greatest consequences. Critical periods occur when the presence of certain kinds of environmental stimuli are necessary for development to proceed normally.
- **Lifespan approaches versus a focus on a particular period:** Developmentalists now believe the entire lifespan is important, for several reasons. One is the discovery that developmental growth and change continue during every part of life. Furthermore, to understand fully the social influences on people of a given age, we need to understand the people who are in large measure providing those influences. For instance, to understand development in infants, we need to unravel the effects of their parents' ages on the social environment.
- **Nature versus nurture:** One of the enduring questions of development involves how much of people's behaviour is due to their genetically-determined nature and how much is due to nurture, the physical and social environment in which a child is raised. In this context, **nature** refers to traits, abilities and capacities that are inherited from one's parents. Nature encompasses any factor that is produced by the predetermined unfolding of genetic information - a process known as maturation. These genetic, inherited influences are at work as we move from the one-celled organism that is created at the moment of conception to the billions of cells that make up a fully-formed human being. In contrast **nurture** refers to the environmental influences that shape behaviour. Some of these influences may be biological, such as the impact of a pregnant mother's substance abuse on the

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fetus, or the amount and kind of food available to children. Other environmental influences are more social, such as the ways parents discipline their children and the effects of peer pressure on adolescents.

How each of the six theoretical perspectives on human development explains child development

Lifespan development has produced a number of broad conceptual perspectives representing different approaches to development. Each broad perspective encompasses a number of theories, explanations and predictions concerning phenomena of interest. A theory provides a framework for understanding the relationships among an organized set of principles or facts. The six major theoretical perspectives include:

- **Psychodynamic Perspective: Focusing on the inner person.** Rooted in Freud's theory, the psychodynamic approach maintains that all behaviour and mental processes reflect the constant and mostly unconscious psychological struggles that rage silently within each person. Usually, these struggles involve conflict between the impulse to satisfy instincts or wishes and the need to play by the rules in society. Anxiety, depression and other disorders are outward signs of this inner turmoil.
- **The Behavioural Perspective: Considering the outer person.** As founded by John Watson, the behavioural approach views behaviour and mental processes as primarily the result of learning. Psychologists who take this approach see rewards and punishment acting on the raw materials provided by genes, evolution and biology to shape each individual. So, whether considering a person's aggression or drug abuse, behaviourists would look at that person's learning history. Since people learn problem behaviours, they can also learn to change or even prevent them by unlearning old habits and developing new ones.
- **The Cognitive Perspective: Examining the roots of understanding.** The cognitive perspective focuses on how people take in, mentally represent and store

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information. Cognitive psychologists then relate perception and information processing to patterns of behaviour. They study such areas as decision-making, problem-solving, interpersonal attraction and intelligence.

Aggression, for instance, might be viewed as a result of poor problem solving.

- **The Humanistic Perspective: Concentrating on the unique qualities of human beings.** According to the humanistic approach, our capacity to choose how to think and act determines our behaviour. Each person's unique perceptions – not instincts, cognitive processes, or rewards and punishments – dictate the choices made. Humanistic psychologists believe that people are essentially good, that they are in control of themselves, and that they seek to grow toward their highest potential.
- **The Evolutionary Perspective: Focusing on biology as the determinant of development.** Darwin's ideas on evolution and adaptation of species laid the foundation for the evolutionary approach. The evolutionary approach to 1psychology holds that the behaviour of animals and humans today is the result of evolution through natural selection. Psychologists who follow this approach are concerned with the adaptive value of behaviour, the anatomy and biology that make it possible and the environmental conditions that encourage or discourage it.
- **The socio-cultural Perspective: Emphasizing the systems of support.** Calling attention to the external influences on human behaviour such as the physical surroundings and social interactions that provide incentives, opportunities and pathways for growth, Urie

Bronfenbrenner (1979) began to emphasize the ecological approach to the study of human development. In Bronfenbrenner's application of this concept, human ecosystems include both the physical environment (the climate, the space per

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person, that arrangement of the dwelling) and the social environment (the people, the culture, and the economy).

LESSON THREE: LATE CHILDHOOD (ADOLESCENCE)

Introduction

The word "adolescence" comes from the Latin adolescere meaning "to grow into maturity". As well as being a time of enormous physiological change, adolescence is also marked by changes in behaviour and expectations. Traditionally, adolescence has been regarded as a prelude to and preparation for adulthood, a transitional period of life between immaturity and maturity. As well as having to deal with the question 'Who Am I?', the adolescent must also deal with the question, 'Who will I be?' .

This unit considers the physical, emotional, cognitive and social changes during adolescence from the perspectives of the developing young person, and the micro-, exo- and macrosystems of support as described by Urie Bronfenbrenner.

Learning Objectives

- To understand physical changes during adolescence.

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- To understand cognitive changes during adolescence.
- To understand socio-emotional changes during adolescence.
- To apply those concepts and understandings in a variety of practical, pertinent and contemporary issues. **Values**
- To appreciate the complex, dynamic, reciprocal and personal aspects of human development.

Lesson Overview: Developmental changes during adolescence

This introductory lesson examines the biological, cognitive and socio-emotional aspects of adolescence.

The roles nature and nurture play in the development of an adolescent

Brainstorm with the students examples of both genetic inheritance and environmental influences on adolescent development. Some examples for nature might include height, weight, temperament and personality traits, intelligence, multiple intelligences and so on. Some examples for the nurture side of the equation might include parenting, family, peers, socioeconomic status, schooling, nutrition, and self-concept and so on.

The socio-cultural influences on adolescent development

At the **microsystem** level, discuss with the students the relative influences of parents, peers and the classroom setting. In what ways do these aspects of the system of support influence our physical, emotional, cognitive and spiritual development? Do parents, peers or the classroom dominate during adolescence?

At the **exosystem** level, discuss the relative influences of the school, the mass media (refer to LESSON 6.5.7 for more information) and the community. What school policies are in place that promote healthy development? How has changing technology influenced adolescent development? What community resources are available to promote healthy adolescent development?

At the **macrosystem** level, discuss the influence that culture has on adolescent development. Is there such a thing as an "adolescent culture"? If so, how would you

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describe an adolescent culture? Has that culture changed? Why or why not? In terms of the influence that economics has on adolescent development, discuss LESSONs such as the cost of clothing, transportation and entertainment. How many students have a part-time job? How many hours a week does that job involve? Why do you need to work? What skills are you learning?

Looking through the eyes of an adolescent

This overview lesson addresses the physical, mental, emotional and spiritual aspects of development during adolescence.

Lesson

- What are the developmental changes during adolescence?

Objectives

- How do adolescents deal with change in their lives?

What are the developmental changes during adolescence?

Adolescence is the period of transition from childhood to adulthood. It is probably the most challenging and complicated period of life to describe, to study, or to experience. The biological changes of puberty, which is considered to begin adolescence, are universal, but their expression, timing and extent show enormous variety, depending on gender, genes, and nutrition. Cognitive development varies as well; many adolescents are egocentric, while others think logically, hypothetically and theoretically. Psychosocial changes during this second decade of life show even greater diversity, as adolescents develop their own identities - choosing from a vast number of sexual, moral, political and educational paths. Most of this diversity simply reflects differences in social and cultural contexts. But for about one adolescent in four, fateful choices are made that handicap, and sometimes destroy the future.

Yet there is also a commonality to the adolescent experience. All adolescents are confronted with the same developmental tasks: they must adjust to their changing body sizes and shapes, to their awakening sexuality, and to new ways of thinking. They all

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strive for the emotional maturity and economic independence that characterize adulthood.

How adolescents deal with change in their lives

Researchers have looked at how and why people change either on their own or with help. Prochaska, Norcross and Diclemente (1992) have proposed a wheel concept that illustrates the six stages of change:

- Precontemplation: At this point a person is not even thinking that there is a problem or that there is a need to make a change. A person requires information and feedback to raise their awareness that they can make changes.

  This is the entry point into change.
- Contemplation: At this point the person swings between considering a change and rejecting it repeatedly.
- Determination or preparation: The person determines that they have to do something about the problem and questions what they can do and explores options.
- Action: A person engages in actions in order to bring a change.
- Maintenance: A person continues previous action and perhaps tries new skills.
- Relapse: There is a return to old behaviour and the task is to get on the wheel again.

Adolescent Development: Physical domain

This lesson addresses the most profound stage of physical change since the prenatal stage.

Lesson Objectives

- What are the physical changes during adolescence?
- How important is body image to adolescents?
- What are the general theories about eating disorders?
- How can self-appraisal influence adolescent development?

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The physical changes during adolescence

During adolescence, the body is moving upward and outward rapidly in what is called the growth spurt. When growth spurts occur, development is not orderly. Arms and hands may grow at different rates from legs, for instance, while other parts of the body develop at yet another rate. Although they may look gangly, teenagers have exceptionally good coordination.

Growth spurts occur earlier for girls than boys, but for both sexes, early adolescence is the time of maximum physical development. Between the ages of nine and 12, girls may grow as much as eight centimetres taller in one year. For boys, this growth occurs between 11 and 15, when they may gain as much as 10 centimetres in height in a year.

How important body image to adolescents

Concern about weight is a common problem for adolescents. Weight and body image are more often problems for females than for males. Society places more importance on physical appearance for women than for men.

The general theories about eating disorders

Why would individuals torture themselves with such a destructive means of relieving emotional distress? Each theory offers an explanation:

- One psychoanalytic hypothesis is that women develop eating disorders because of a conflict with their mothers who provided their first nourishment and from whom the daughters cannot psychically separate.
- Learning theory notes that both anorexia and bulimia are typically associated with psychological problems, including low self-esteem and depression. These psychological disorders precipitate extreme dieting and then perpetuate the destructive pattern, thus becoming part of a stimulus-response chain. Fasting,

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bingeing and purging have “powerful effects as immediate reinforcers - that is, as a means of relieving states of emotional distress and tension.”

- One cognitive explanation is that as women compete with men in business and industry, they want to project a strong, self-controlled, masculine image antithetical to the buxom, fleshy body of the ideal woman of the past.
- socio-cultural explanations include the contemporary cultural pressure to be slim and trim and model-like - a pressure that seems to be particularly felt by unmarried young women seeking autonomy from their parents. This would help explain why these eating disorders were once rare; society expected women to be dependent, first on parents and then on a husband, with no need to prove one's selfhood.
- An ethological explanation notes that girls who are overwhelmed with the stresses of puberty may discover that self-starvation makes their menstrual periods cease, their sexual hormones decrease, and their curves disappear - all of which remove the sexual pressures that normal maturation compels adolescent girls to experience.

How self-appraisal can influence adolescent development

Negative self-appraisal can have a major impact on self-esteem. Although self-esteem is obviously influenced by success in athletics, academics, friendship or other areas that the adolescent considers significant, a teenager's assessment of his or her appearance is the most important determinant. At one time or another, almost every American girl under eats, sometimes drastically, to be thinner. In one study, girls age 14 to 18 typically wanted to be about 12 pounds (five kilograms) lighter than they were. Boys are also vulnerable. Roughly five percent of high school seniors use steroids to build up their muscles. Boys who do not take drugs are nonetheless concerned, lifting weights, doing push-ups, and so on, in an attempt to change their physiques. The concern with physique dominates the peer culture, particularly during adolescence. Unattractive teenagers tend

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Lesson Objectives

- What is the process of cognitive development during adolescence?
- What is meant by "adolescent egocentrism", and what role does it play in cognitive and social development?
- What is intelligence?
- What are the different types of intelligence?

to have fewer friends - of either sex. And attractiveness is a sexual lure, especially during adolescence. Adolescents also receive powerful messages from the broader social environment. Media images of handsome faces and beautiful bodies are used to sell almost everything, from clothes and cosmetics to luncheon meats and auto parts. These images reinforce the cultural stereotype that men should be tall and muscular, and that women should be thin and shapely.

Adolescent Development: Cognitive domain

This lesson addresses the rapid and complex growth in cognitive abilities during adolescence.

The process of cognitive development during adolescence

Since adolescents must deal with a staggering number of issues, it is fortunate that their thought processes and moral reasoning have reached their highest level. In Piaget's theory of cognitive development, the formal operations stage includes the ability to reason in abstract ways – to consider the possibilities instead of thinking only in terms of concrete realities. The ability to think abstractly helps the future become more real. This high level of reasoning ability helps adolescents consider how the world could be a better place and why personal principles are sometimes more important than external rules and laws (McMahon and Romano, 2000, p. 335).

"Adolescent egocentrism", and the role it plays n cognitive and social development

Adolescents frequently practise their new thinking skills on themselves, a process that makes them lose some of their detachment. They worry about how they are regarded by others; they try to sort out their own conflicting feelings about parents, school and close friends; they think deeply but not always realistically about their future possibilities; they

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reflect, at length, on each day's experiences. Analyzing their private thoughts and feelings, forecasting their future, and reflecting on their experiences underlie the greater reflection and self-awareness - and enhanced capacity for selfcenteredness - that distinguishes adolescence. However, these new ventures in introspection are often distorted by adolescent egocentrism, a self-view in which adolescents regard themselves as much more socially significant as they actually are. Adolescent egocentrism can lead to false conclusions:

- One is the invincibility fable, by which young people feel that they will never fall victim, as others do, to dangerous behaviour.
- Another false conclusion resulting from adolescent egocentrism is the personal fable, through which adolescents imagine their own lives as unique, heroic, or even mythical. They perceive themselves as different from others, distinguished by unusual experiences, perspectives, and values.
- A third false conclusion stemming from egocentrism is called the imaginary audience. This arises from many adolescents' assumption that other people are as intensely interested in them as they themselves are. This results in acute self-consciousness.

Social Cognition

This overview lesson describes the general concept of social cognition, or the ways in which we make sense of the thoughts, feelings and behaviors of other people through analysis and interpretation.

Lesson Objectives

- How do we make sense of the behaviour of other people?
- How do we form impressions about people?
- What are some biases on how we form impressions of other people?
- How do we make judgments and decisions about other people?

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How we make sense of the behaviour of other people

Social cognition is the thought process of making sense of events, people, oneself, and the world in general through analyzing and interpreting them. Social cognition focuses on social information in memory, which affects judgments, choices, evaluations, and ultimately, behaviour .

Social cognition focuses on the way in which our thoughts are affected by the immediate social context, and in turn, how our thoughts affect social behaviour. The approach can be summarized as follows:

- People have a limited capacity to process information about the social world and will take cognitive shortcuts (such as stereotyping) in order to minimize the load.
- We develop schemata that represent our knowledge about ourselves, others, and our roles within the social world. These schemata, once formed, bias our judgements about ourselves and others.
- Schemata become more complex and organized over time, and also harder to change.

Often, to save time, people use mental shortcuts to make sense of the world, developing rules of thumb. Some of these rules of thumb include:

- Representativeness, (individuals or events that appear to be representative of other members of a group are quickly classified as such)
- Availability (ease of association with existing knowledge)
- False consensus effect (people tend to believe that others agree with them)

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- Framing (the way in which information is organized and presented helps determine whether it is will be accepted, rejected or ignored).

How we form impressions of people

The evidence indicates that we follow a weighted averaging model in combining information about people. That is, we keep a rough "running average" of our trait ratings in our heads, as we discover more characteristics about people.

Of course, we know that every person is unique, and that no two classes, no two hockey matches are exactly alike. Yet there are similarities among certain types of people or events. Thus, we tend to organize our view of the world in terms of categories. People are generally categorized in terms of easily observable characteristics, such as gender, ethnic group, occupation or age. Similarly, we construct categories of events, activities, objects and even ideas.

Cantor and Mischel (1979) suggest that we often use prototypes - mental images of a typical example of that category - for example a dog as a prototype of the category, mammal. The extent to which a particular object resembles the prototype, and the extent to which you allow for variations, will determine how readily you identify the object with the category.

Each of us uses a particular organizing schema to make sense of the information about others. We do this by using constructs:

- Physical constructs classify people according to their appearance: male or female, beautiful or ugly, fat or thin, young or old, and so on.
- Role constructs use social position: student, lawyer, wife, and so on.
- Interaction constructs focus on social behaviour: Friendly, helpful, aloof, and sarcastic.
- Psychological constructs: curious, nervous, insecure and so on.
- Membership constructs help us to identify others according to the group in which they belong: teacher, Liberal, Environmentalist, and so on (Adler, Towne and Rolls, 2001, p. 91).

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Some errors in forming impressions

Researchers have noticed two interesting biases in how first impressions of people are formed. First, people tend to form impressions of others which are positive rather than negative, a positivity bias. But while we have this bias towards being positive in our impressions of others, those impressions will be influenced more by negative than by positive information. This is called the negativity effect.

How we make judgments and decisions about other people

In addition to schemas, people use judgmental heuristics to help us deal with the large amount of social information with which we are faced. Heuristics are rules of thumb people follow in order to make judgments quickly and efficiently. The *availability* heuristic refers to the ease with which we can think of something, which has a strong effect on how we view the world. The *representative* heuristic

helps us decide how similar one thing is to another; we use it to classify people or situations on the basis of their similarity to a typical case. When using this heuristic we have a tendency to ignore base rate information - that is, the prior probability that someone or something belongs in that classification. People also rely on the anchoring*/adjustment* heuristic, wherein an initial piece of information acts as an anchor, or starting point, for subsequent thoughts on the subject.

Social Perception

This lesson introduces the second of the three major aspects of this unit, social perception, or the ways in which we infer other people's motives and intentions from observing their behaviour and deciding whether the causes of the behaviour are internal or situational.

Lesson Objectives

- What social perception is
- How social perceptions are formed and changed
- How we select information about others
- What some problems and issues in forming impressions of people and situations
- What the relationship between attribution, stereotypes, prejudice and discrimination is
- Who some prominent social rights activists are, and what they achieved

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Social perception

Social perception is the process by which someone infers other people's motives and intentions from observing their behaviour and deciding whether the causes of the behaviour are internal or situational. Social perception helps people make sense of the world, organize their thoughts quickly, and maintain a sense of control over the environment. It helps people feel competent and masterful, maintain a sense of balance, because it helps them predict similar events in the future.

How social perceptions are formed and changed

Research has shown that people form impressions of each other in two ways. Sometimes people make quick and effortless judgments based on others' physical appearance, facial expressions, or body language. Studies have shown, for example, that people who are physically attractive are perceived to be happy, warm, friendly, successful, confident, and well-adjusted. At other times, however, people form impressions based on a careful observation of a person's behaviour. According to this latter view, people act like amateur scientists, gathering and analyzing behavioural evidence before evaluating others. The explanations for behaviour that people come up with are called attributions, and the theory that describes the process is called *attribution theory*.

How we select information about others

Because we are exposed to more input than we could possibly manage, the first step in perception is the selection of which data we will attend to. There are several factors that cause us to notice some messages and ignore others.

- Stimuli that are intense often attract our attention.
- Repetitious stimuli also attract our attention. Just as a quiet but steadily dripping faucet can come to dominate our awareness, people to whom we are frequently exposed become noticeable.
- Attention is also frequently related to contrast or change in stimulation. Put differently, unchanging people or things become less noticeable.
- Motives determine not only what information we select from our environment but also how we perceive people. For example, someone looking for a romantic adventure will be especially aware of attractive potential partners.

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What some problems and issues in forming impressions of people and situations are

Over the years, research into attribution has shown that when we explain the behaviour of others, we tend to overestimate the role of personal factors and underestimate the influence of situations. This bias is so universal that it has been called the *fundamental attribution error.* In one demonstration of the fundamental attribution error, experimenters randomly assigned subjects to participate in a quiz show in the role of either questioner or contestant. Then in front of the contestant and an observer, the experimenters told the questioner to devise a set of difficult questions to ask the contestant. Not surprisingly, many of the questions—created from the questioner's own store of esoteric knowledge— stumped the contestant. Yet when asked to rate the general knowledge of both participants, observers consistently saw the questioners as more knowledgeable than the contestants. The observers failed to take the situational roles into account and attributed the behaviour they witnessed to each person's level of knowledge.

In forming impressions of others, people are subject to other biases as well. For example, a great deal of research shows that people are often slow to revise their first impressions of others even when those views are not supported by the evidence. Part of the problem is that once we form an impression of someone, we tend to interpret that person's later behaviour in ways that seem to fit our impression. Another problem is that our first impression of someone may shape the way we treat that person— which, in turn, may influence his or her actual behaviour. This process is known as a *self-fulfilling prophecy*

.

What the relationship between attribution, stereotypes, prejudice and discrimination is

The various components that make up an attitude may not always be in agreement. Prejudice is no exception. A crucial factor in prejudice appears to be the affective component. Thus, an individual may intellectually accept that native people are not inherently less capable or lazier than whites, but may nevertheless avoid them. However, attitude components are more likely to be highly correlated in extreme bigots. This type of person will not only have strong negative feelings, but will usually have an extensive set of negative stereotypes, some of which may be quite bizarre.

Adolescent Development: socio-emotional domain

This lesson addresses the multitude of social roles, gender roles, relationships and social behaviours that characterize adolescence as a unique stage of development.

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What the process of socio-emotional development in adolescence is

Adolescence is often painted as a time of great difficulty and confusion. While the trials of this period vary from one culture to another, there is little indication that it is actually that bad for most adolescents. One of the hardest issues to deal with is trying to find an identity. Until adolescence, there is no pressure to find out about yourself. Now it becomes necessary to try out different roles to see which ones fit. Other societies, especially in non-industrialized nations, handle the transition from childhood to adulthood more simply. This transition is made through some kind of initiation, or rite of passage, meaning that a change of status is recognized by a formal ritual. We have no such ceremonies, so adolescents need to form their own subculture in order to avoid complete isolation. Forming groups and achieving a sense of identity are two crucial psychological issues during adolescence.

How we explain our social behaviour

Problem behaviour theory (Jessor, 1993) represents a comprehensive approach to understanding adolescent behaviour. In it, adolescent behaviour is viewed as being consistent with what the adolescent is like as a person (a "personality system"), how the adolescent is reacting to his or her home life and peer group (a "perceived environment system"), and what the adolescent tends to be doing (a "behaviour system"). Personality variables relevant to problem behaviour include low expectancies for success in school, high values for independence, and an absence of constraints against such behaviour, such as religious beliefs or concern about deviance. Predisposing characteristics of the social environment include opportunities to engage in problem behaviour and the influence of peer groups. Problem drinking also tends to be part of a more general pattern of behaviour, including the use of marijuana and other drug use, deviant/delinquent acts such as lying and stealing, and precocious sexual involvement, along with less participation in school and church activities

Self-understanding

This lesson speaks to the central theme of, Who Am I? by guiding the students to a better understanding of themselves as active participants in the social world.

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Lesson Objectives

- What self-concept is
- How we develop a self-concept
- How we change a self-concept
- The cultural influences on self-concept
- How gender influence self-concept

What self-concept is

The self-concept has four components: self-image, self-esteem (or self-regard), idealself and selfefficacy.

- Self-image is the sort of person we think we are. One way of assessing selfimage is to ask people to answer the question 'Who Am I?' 20 times. This typically produces two main categories of answers relating to social roles and personality traits. Self-image also includes body image.
- While self-image is essentially descriptive, self-esteem is essentially evaluative. Self-esteem is the extent to which we like or approve of ourselves, and how worthwhile we think we are.
- The ideal-self is the kind of person we would like to be. In general, the greater the gap between self-image and ideal-self, the lower the self-esteem.
- According to Bandura (1977), self-efficacy, or one's expectations of success in a given situation, is an important determinant of whether one will attempt to make changes in one's environment. Each day, we make many decisions based on our perceptions of the extent to which our actions will produce reinforcement. Our

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actions are based on our evaluations of our competency. Moreover, self-efficacy not only determines whether we will engage in a particular behaviour, it also determines the extent to which we will maintain that behaviour in the face of adversity.

How we develop a self-concept

According to Carl Rogers (1980), self-concept or identity is shaped partly by selfactualizing tendencies and partly by others' evaluations. In this way, people come to like what they are "supposed" to like and to behave as they are "supposed" to behave. Although this socialization process is adaptive, allowing people to get along in society, it often requires that they stifle the self-actualizing tendency and distort experience.

How we change a self-concept

Four ways self-esteem can be improved are through:

- Identifying the causes of low self-esteem and the domains of competence important to the self. People have the highest self-esteem when they perform competently in domains important to the self.
- Emotional support and social approval, including both adult and peer approval.
- Achievement, which has much in common with Bandura's cognitive social learning concept of self-efficacy, which refers to individuals' beliefs that he or she can master a situation and produce positive outcomes.
- Coping with a problem rather than avoiding it. If coping rather than avoidance prevails, people often face problems realistically, honestly and nondefensively. This produces favourable selfevaluative thoughts, which lead to the selfgenerated approval that raises self-esteem.

How gender influences self-concept

In the 1970s, as both males and females became dissatisfied with the burdens imposed by their stereotypical roles, alternatives to masculinity and femininity were proposed. Instead of describing masculinity and femininity as a continuum in which more of one

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means less of the other, it was proposed that individuals could have both masculine and feminine traits. This thinking led to the development of androgyny, which refers to the presence of desirable masculine and feminine characteristics in the same person. The androgynous male might be assertive (masculine) and nurturant (feminine). The androgynous female might be powerful (masculine) and sensitive to the feelings of others (feminine).

What the cultural influences on self-concept are

Individualistic cultures	**Collectivist cultures**
• Self is separate, unique, individual; should be independent, self-sufficient • Individuals should take care of themselves and immediate family • Many flexible group memberships; friends based on shared interests and activities • Reward for individual achievement and initiative; individual decisions encouraged; individual credit and blame assigned • High value on autonomy, change, youth, individual security, equality	• People belong to extended families or ingroups; "we" or group orientation • Person should take care of extended family before self • Emphasis on belonging to a very few permanent in-groups which have a strong influence over the person • Reward for contribution to group goals and well-being; co-operation with in-group members; group decisions valued; credit and blame shared • High value on duty, order, tradition, age, group security, status and hierarchy

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Self-concept

Central to our self-understanding is our self-concept. This lesson introduces the four aspects that together comprise our self-concept: self-image, self-esteem, self-efficacy and the ideal-self.

Lesson Objectives

- What a self-concept is
- How the self-concept develops
- What influences the development of a self-concept
- How important your self-concept is
- What we do when our self-concept is threatened

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What a self-concept is

The self-concept is defined as the sum of feelings, beliefs and impressions that individuals have of themselves - the self perceiving the self.

According to Carl Rogers (1980), the self is the part of experience that a person identifies with "I" or "me". Those who accurately experience the self - with all the preferences, abilities, fantasies, shortcomings and desires - are on the road to self-actualization.

How the self-concept develops

Identity formation is the central task of adolescents according to Erikson's theory of psychosocial development. According to Erikson (1968) issues in late adolescence - graduating from high school, going to college and forging relationships - challenge the adolescent's self-concept, precipitating an identity crisis. In this crisis, the adolescent must develop an integrated self-image as a unique person by pulling together selfknowledge acquired during childhood. If infancy and childhood brought trust, autonomy and initiative, according to Erikson, the adolescent will resolve the identity crisis positively, feeling self-confident and competent. If, however, infancy and childhood resulted in feelings of mistrust, shame, guilt and inferiority, the adolescent will be confused about his or her identity and goals.

Cooley (1902) argued that we construct our self-concepts from how we appear to others, which is then reflected back to us - what he called the looking-glass self. There are several processes by which "reflection" enables us to construct our ideas and impressions of ourselves:

- Social Identity refers to those aspects of a person's self-image that depend upon the social categories and groups to which he or she belongs. Turner (1982) describes social identification as a three-part process. First, there is social categorization: individuals perceive themselves

  and others in terms of membership in distinct categories or groups. Second, the typical norms, attitudes and behaviours that distinguish the groups determine behaviour to a large degree.

  Third, individuals conform to the stereotypes associated with the group.
- Perhaps most central to your social identity is your gender - male or female.

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- What does “masculinity” and “femininity” mean? What is called masculinity has been described as instrumentality or agency, a concern with achieving goals and being active in the world, while femininity is described as expressiveness or communion, being other-centered and concerned with interpersonal relationships.
- Self-perception theory suggests that people become aware of their own attitudes, feelings, values, dispositions and other internal characteristics in the same way that they form impressions of the characteristics of others - through observation of their own behaviour. The notion of self-knowledge through self-perception implies that, rather than thinking about what we really believe in, and then acting on those beliefs, we first act and then discover our beliefs and values from our own actions.
- Social comparison is based on three premises:
- Humans have a drive to evaluate their own opinions, feelings and abilities, and want to feel confident that they are accurate.
- In the absence of objective or nonsocial bases of assessment, individuals will evaluate themselves in comparison with others.
- People tend to compare themselves with someone similar to themselves in opinion, background or ability. In general, people compare themselves to someone equal when the goal is self-assessment, to someone better when the goal is self-improvement, and to someone inferior when the goal is selfenhancement.

What influences the development of a self-concept

The society in which we live, or the ethnic group to which we belong, shapes many of our beliefs, values and behaviours - and these play important roles in shaping the character of the personality. Dion and Dion (1993) argue that members of collectivist and individualistic cultures think differently about love and marriage. Their thesis is that romantic conceptualizations of love flourish only in individualistic cultures. Why? Because romantic

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notions of love and marriage focus narrowly on the specific needs of the individual. Romantic love is "selfish" in the dictionary sense of "seeking or concentrating on one's own welfare or advantage." Each individual has an idealized notion of a relationship with another that will bring him or her great gratification. Members of individualistic societies assume they have the right to enter into such selfsatisfying relationships and tend to display only marginal concern about the possible reactions of others.

In a collectivist society, on the other hand, the individual has a deep interactive dependency with his or her family and reference group, and a more extended selfconcept. In such societies, the individual must carefully consider the degree to which a prospective partner will fit into this relationship. In short, the self-concept that characterizes collectivist societies does not allow love and marriage to be considered exclusively in terms of the narrow interests of the individual.

The Ashanti people of West Africa name children according to the day of the week on which they are born because they believe they have different personalities. Police records indicated that a very high proportion of juvenile delinquents were born on a Wednesday (the day of the "naturally aggressive" personality), whilst a very low proportion were born on a Monday (the day of the "quiet and calm" personality). If Ashanti boys were treated in a way consistent with their names, they may consequently become what their names indicate they are really like (a self-fulfilling prophecy). In Maori culture, a person is invested with a certain kind of power (manna), given by the gods in accordance with family status and birth circumstances. This enables a person to be effective, whether in battle or everyday dealings with others. However, this power is not a stable resource but can be increased or decreased by the person's day-to-day conduct. A person who forgot a ritual observance or committed some misdemeanor would have his or her power decreased. A person's social standing, successes and failures, and so on are seen as dependent on external sources rather than internal states (such as personality or motivation). Indeed, manna is only one of the external forces which inhabit a person. Instead of representing themselves as the centre and origin of their actions (which is crucial to the Western concept of the self), the individual Maori do not own experiences such as fear, anger, love and grief. Rather, these experiences are visitations governed by the unseen world of powers and forces

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How important your self-concept is

Persons with high self-esteem
Are likely to think well of others. • Expect to be accepted by others. • Evaluate their own performance more favourably than people with low selfesteem. • Perform well when being watched: not afraid of the reaction of others. • Work harder for people who demand high standards of performance. • Are inclined to feel comfortable with
others they view as superior in some way. Are able to defend themselves against negative comments of others.

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Persons with low self-esteem

Are likely to disapprove of others.

- Expect to be rejected by others.
- Evaluate their own performance less favourably than people with high selfesteem.
- Perform poorly when being watched: are sensitive to possible negative reaction.
- Work harder for undemanding, less critical people.
- Feel threatened by people they view as superior in some way.
- Have difficulty defending themselves against other people's negative comments; more easily influenced.

What we do when our self-concept is threatened

When threatened, we all do our best to keep some kind of balance. We protect our inner selves from too much attack by using defense mechanisms:

- When we use repression we do not allow ourselves to remain aware of painful material; we push it out of consciousness. Repression is usually unhealthy.
- When we engage in denial, we refuse to admit that anything bad has happened.

  In some ways, denial is similar to repression. With repression, though, we are at least partly aware of the problem, and then we push it out of consciousness.

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With denial, we do not let the problem into consciousness in the first place.

- Sometimes we have trouble directly expressing what we feel because of the threat (real or imagined) that something terrible will happen as a result. So we vent our feelings elsewhere or on someone else, engaging in displacement.
- In reaction formation, what we express is the opposite of what we really feel. Reaction formation is sometimes a little hard to see in operation, since we can not read people's minds.
- When the emotions we feel are too overwhelming, we may try to eliminate them altogether. Talking coolly and rationally about a tragedy as if it were simply an event that we had observed is called intellectualization. We have taken all of the feelings out of our description.
- Identification with the aggressor occurs when we are mistreated for a long time by someone much more powerful than we are, and begin to take on that person's characteristics. We begin to identify with him or her, or try to gain favour.
- With regression we defend ourselves by “moving backward” and behaving like children. This defense is an extreme reaction to the frustration of having to be an adult and take responsibility.

We regress to a time when we were helpless children and someone had to take care of us.

- With rationalization we explain what we do in such a way that we avoid any responsibility for a bad outcome.
- Projection refers to the process of mentally giving to someone else our own thoughts or feelings. A continued pattern of such behaviour is self-destructive, since it does not help us face up to how we might be causing the problem.

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- Sublimation occurs when we channel our emotional energy into a constructive or creative activity. It is the only defense mechanism that is truly healthy and adaptive.

Culture and the Self

The predominant culture, or indeed sub-culture, has a profound influence on the development and maintenance of the self-concept. This lesson addresses the nature of being "Western" or "Eastern" in cultural background.

Lesson Objectives

- What culture means
- The influence does culture have on self-concept
- How cultural background influence your general health and outlook on life

What culture means

Culture has been defined as the accumulation of values, rules of behaviour, forms of expression, religious beliefs, occupational choices, and the like for a group of people who share a common language and environment. As such, culture is an organizing and stabilizing influence. It encourages or discourages particular behaviours and mental processes. It also allows people to understand others and anticipate their behaviour. It is a kind of group adaptation passed by tradition and example rather than by genes, from one generation to the next. Culture determines, for example, whether children's education is concerned with hunting or reading, how close people stand when they talk to each other, and whether or not they form lines in public places.

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The influence does culture have on self-concept

Individualistic cultures
• Self is separate, unique individual; should be independent, self-sufficient. • Individuals should take care of themselves and immediate family. • Many flexible group memberships; friends based on shared interests and activities. • Reward for individual achievement and initiative; individual decisions encouraged; individual credit and blame assigned. • High value on autonomy, change, youth,
individual security, equality.
Collectivist cultures
• People belong to extended families or ingroups; "we" or group orientation. • Person should take care of extended family before Self. • Emphasis on belonging to a very few permanent in-groups which have a strong influence over the person.

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- Reward for contribution to group goals and well-being; co-operation with in-group members; group decisions valued; credit and blame shared.

- High value on duty, order, tradition, age, group security, status and hierarchy

How your cultural background influences your general health and outlook on life

Cultures differ in their degree of fatalism and in their beliefs about whether it is possible to take control over their health. For example, in Germany, which has a highly structured social welfare system, people feel they have more psychological control over their health and work than Americans do. Might these cultural attitudes be related to mortality rates? The answer, remarkably, seems to be yes. In traditional Chinese astrology, certain birth years are considered unlucky, and people born in those years often fatalistically expect bad fortune. This expectation can become a self-fulfilling prophecy. In a study of many thousands of people matched by age and cause of death, Chinese-Americans who had been born in a year traditionally considered to be ill-fated died significantly earlier - one to five years earlier! - than whites who had been born in the same year and who had the same disease. The more strongly traditional the Chinese were, the more years of life they lost. These results held for nearly all causes of death studied, even when the researchers controlled for how well the patients took care of themselves and which treatments they were given.

Gender and the Self

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Lesson Objectives	• What culture means • What influence culture has on self-concept • How cultural background influences your general health and outlook on life

What gender means

Gender identity is our classification of ourselves (and others) as male or female, boy or girl, and so on. Gender role refers to the behaviours, attitudes, beliefs and so on which a particular society either expects from, or considers appropriate to, males and females on the basis of their biological sex. To be masculine or feminine, therefore, requires males and females to conform to their respective gender roles.

How a gender identity develops

- *Biological*: Supporters of a biological approach to gender development argue that males and females are biologically programmed for certain kinds of activities compatible with male and female roles.

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- *Humanistic theory* (Rogers): Rogers assumes that each person responds as an organized whole to reality as he or she perceives it. Rogers emphasized selfactualization, which he described as an innate tendency towards growth that motivates all human behaviour. To Rogers, personality is the expression of each individual's self-actualizing tendency as it unfolds in that individual's unique, perceived reality. Central to Rogers' theory is the self, the part of experience that a person identifies as "I" or "Me".
- *Socio-biological theory*: Socio-biologists (evolutionary theorists) argue that gender has gradually evolved over the course of human development as part of our broader adaptation to the environment. The relatively greater physical strength and lung capacity of males make them better suited to hunting and defending territory and family. The child-bearing and milkproducing capacities of females, however, make them ideally suited to child care and other nurturant roles.
- *Psychoanalytic theory*: Freud's theory is related to his explanation of moral development. Up until the resolution of the Oedipus complex, gender identity is assumed to be flexible. Resolution of the Oedipus complex occurs through identification with the same-sex parent, and results in the acquisition of both a superego and gender identity. As well as a weaker conscience, Freud also saw the development of gender identity as being weaker in girls than boys.
- *Social learning theory*: According to social learning theory, one reason girls and boys learn to behave differently is that they are *treated differently* by their parents. Social learning theory emphasizes the roles of observational learning and reinforcement. By observing others behaving in particular ways and then

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- imitating that behaviour, children receive reinforcement from significant others for behaviours considered to be sex-appropriate.

  Cognitive-developmental: The cognitive-developmental approach emphasizes the child's participation in developing both an understanding of gender and gender-appropriate behaviour. Children's discovery of the fact that they are male or female causes them to identify with members of the same sex (not the other way around as psychoanalytic and social learning theories suggest). According to cognitive-developmental psychologists, young children acquire an understanding of the concepts male and female in three stages:

- Gender labeling or basic gender identity: This occurs somewhere around age three and refers to the child's recognition that it is male or female.
- Gender stability: By age four or five, most children recognize that people retain their gender for a lifetime. However, there are limitations, in that children rely on superficial signs such as the hair length to determine the gender.
- Gender constancy: At around age six or seven, children realize that gender is immutable. Gender constancy represents a kind of conservation and, significantly, appears shortly after the child has mastered conservation of quantity.

What the differences between the genders are

Santrock (1999) states that genuine behavioural differences do exist between the sexes and people's stereotypes are not entirely inaccurate. But the differences are fewer in number, smaller in size and far more complex than stereotypes suggest.

- *Physical/Biological*: From conception on, females are less likely to die than males. Females also are less likely than males to develop physical or mental

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- disorders. Estrogen strengthens the immune system, making females more resistant to infection. Males have twice the risk of coronary disease as females. On average, males grow to be about 10 percent taller than females.
  Cognitive abilities: In the cognitive domain, it appears that there are three genuine gender differences. First, on the average, females tend to exhibit slightly better verbal skills than males. Second, starting in high school, males show a slight advantage on tests of mathematical ability. Third, starting in the grade school years, males tend to score higher than females on various measures of visual-spatial ability.
- *Social behaviour and personality*: First, studies indicate that males tend to be more aggressive than females, both verbally and physically. This disparity shows up in early childhood. Second, there are gender differences in nonverbal communication. The evidence indicates that females are more sensitive than males to subtle nonverbal cues. Females also smile and gaze at others more than males. Third, females appear to be slightly more susceptible to persuasion and conforming to group pressure than males are. Fourth, males are more sexually active than

  females are, and they have more permissive attitudes about causal, premarital and extramarital sex. Finally, males score higher on assertiveness scales, whereas females score higher on measures of anxiety, trust, empathy and nurturance. Sixth, females are more relationshiporiented than males, and that this relationship-orientation should be prized as a more important skill in our culture than it is currently held to be.
- *Communication*: Sociologist Deborah Tannen (1990) distinguishes between rapport talk and report talk. Rapport talk is the language of conversation and a way of establishing connections and negotiating relationships. Report talk is

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- talk that gives information. Males hold centre stage through report talk, while females prefer private, rapport talk and conversation that is relationshiporiented.

  Aggression: One of the most consistent gender differences is that boys are more aggressive than girls. Another is that boys are more active than girls. The aggression difference is especially pronounced when children are provoked. These differences occur across cultures and appear very early in children's development. Biological factors include heredity and hormones. Environmental factors include cultural expectations, adult and peer models, and social agents who reward aggression in males and punish aggression in females.

- *Emotional control*: An important skill is to be able to regulate and control your emotions and behaviour. Males usually show less self-regulation than females, and this low self-control can translate into behavioural problems. In one study, children's low self-regulation was linked with greater aggression, teasing others, overreaction to frustration, low cooperation and inability to delay gratification.

Males value power, competency, efficiency and achievement. They are always doing things to prove themselves and develop their power and skills. Their sense of self is defined through their ability to achieve results. Males are solution-oriented. They experience fulfillment primarily through success and accomplishment. Males are more concerned with outdoor activities, like hunting, fishing and racing cars. They are interested in the news, weather and sports and couldn't care less about romance novels and self-help books. Males are more interested in “objects” and “things” rather than people and feelings. While women fantasize about romance, men fantasize about powerful cars, faster computers, gadgets, gizmos, and new more powerful technology.

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Men are preoccupied with the “things” that can help them express power by creating results and achieving their goals. Achieving their goals is very important to a male because it is a way for him to prove his competence and thus feel good about himself. For males to feel good about themselves, they must achieve these goals by themselves. Males pride themselves in doing things all by themselves. Autonomy is a symbol of efficiency, power and competence.

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Females value love, communication, beauty and relationships. They spend a lot of time supporting, helping and nurturing one another. Their sense of self is defined through their feelings and the quality of their relationships. They experience fulfillment through sharing and relating. Rather than building highways and tall buildings, females are more concerned with living together in harmony, community and loving cooperation. Relationships are more important than work and technology. Personal expression, especially of feelings, is very important. Communication is of primary importance. To share their personal feelings is much more important than achieving goals and success. Talking and relating to one another is a source of tremendous fulfillment.

Instead of being goal oriented, women are relationship oriented; they are more concerned with expressing their goodness, love and caring. Females are very involved in personal growth, spirituality, and everything that can nurture life, healing and growth. Females are very intuitive. They have developed this ability through centuries of anticipating the needs of others. They pride themselves on being considerate of the needs and feelings of others (Gray, 1992, pp. 16-20).

What the socio-cultural influences on gender development are

Although parents do encourage sex-appropriate play, there is evidence that biological factors may play an initial role in children's preferences. Although fathers are less likely to give dolls to one-year-old boys than to one-year-old girls, the boys who do receive the dolls are less likely to play with them (Snow et al., 1983). Perhaps adult expectations and encouragement build upon children's preferences, producing an amplifying effect. Then, because boys' toys provide more opportunity for developing motor skills, visuospatial skills, and inventiveness, and girls' toys provide more opportunity for nurturance and social exchange, some important differences in sex roles may become established.

Once children begin to play with other children outside the home, peers have a significant influence on the development of their gender roles. In fact, Stern and Karraker (1989) found that the behaviour of two- to six-year-old children was even more influenced by the knowledge of a baby's gender than was the behaviour of adults. By the time children are three years old, they reinforce gender-typed play by praising, imitating or joining in the behaviour. In contrast, they criticize gender-inappropriate behaviour (Langlois and Downs, 1980). Parents indirectly encourage gender-stereotyped play by seeking out children of the same sex as playmates for their own children.

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Self-schemas and Identity

This lesson looks at self-identity in preparation for the lesson on identity management, or the ways in which we present ourselves both privately and publicly.

Lesson
- How we represent knowledge about ourselves

Objectives
- How self-identity develops
- What influences the family has on identity
- How gender influence identity development

How we represent knowledge about ourselves

We not only represent and store information about other people, but also about ourselves, although in a more complex and varied way. Most people have a complex self-concept with many self-schemata.

These include an array of "possible selves", or future-oriented schemata of what we would like to be (our ideal-self). Visions of future possible selves may influence some of the decisions we make, such as career choices

We learn about ourselves through introspection and observations of our behaviour, and then organize this information into self-schemas. We also interpret our past using schemas and theories about how our attitudes and behaviours are likely to change. As important as these sources of self-knowledge are, though, there is still something missing. These views portray people as solitary seekers of selfknowledge, with no consultation with or comparison to other people. The two ways in which we gain selfknowledge that are entirely social, in that we rely on other people to learn about who we are, are the "looking glass self" and social comparison theory.

How self-identity develops

Contemporary views of identity development suggest several important considerations.

- First, identity development is a lengthy process. Identity formation begins with the appearance of attachment, the development of a sense of self, and the emergence of independence in infancy, and reaches its final phase with a life review and integration in old age.

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- Second, identity development is extraordinarily complex. At the bare minimum, it involves commitment to a vocational direction, an ideological stance, and a sexual orientation. Canadian psychologist James Marcia (1980) analyzed Erikson's theory of identity development and concluded that four identity statuses, or modes of resolution, appear in the theory: identity diffusion, identity foreclosure, and identity moratorium and identity achievement.
- Identity diffusion is Marcia's term for the status of adolescents who have not yet experienced a crisis (that is, they have not yet explored meaningful alternatives) or made any commitments.
- Identity foreclosure is Marcia's term for adolescents who have made a commitment but have not experienced a crisis.
- Identity moratorium is Marcia's term for the status of adolescents who are in the midst of a crisis, but their commitments are either absent or only vaguely defined.
- Identity achievement is Marcia's term for the status of adolescents who have undergone a crisis and have made a commitment.

The process of identity development

As they try to sort through their possible (and multiple) selves, adolescents frequently take on a false self, acting in ways that they know are contrary to the core of their being - even if they are not sure what that core being is. According to one group of researchers (Harter, Marold, Whitesell and Cobbs, 1996), adolescents display three distinct types of false selves:

- *The acceptable false self.* This false self arises from the adolescent's perception that the real self is rejected by parents and peers - a perception often coloured by the adolescent's own self-hate. Adolescents who adopt a false self in order to be

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accepted tend to feel worthless, depressed, and hopeless; they engage in self-betrayal to hide their true nature. They also report low levels of real selfunderstanding.

- *The pleasing false self.* This second type of false self arises from a wish to impress or please others. It is quite common among adolescents. Those who adopt it appear to be less debilitated psychologically, and to have a greater selfunderstanding, than those whose false selves arise from a sense of rejection.
- *The experimental false self.* This type of false self is one that adolescents try out "just to see how it feels". Compared with adolescents who engage in the first two types of false behaviour, these adolescents report the highest levels of self-esteem and self-knowledge, partly because although they acknowledge that their experimentation is not their usual, expected behaviour, they do not feel it is false.

What influences the family have on identity

Parents are important figures in the adolescent's development of identity. In studies that relate identity development to parenting styles, democratic parents, who encourage adolescents to participate in family decision making, foster identity achievement. Authoritarian parents, who control the adolescent's behaviour without giving the adolescent an opportunity to express opinions, encourage identity foreclosure. Permissive parents, who provide little guidance to adolescents and allow them to make their own decisions, promote identity diffusion.

In addition to studies on parenting styles, researchers have also examined the role of individuality and connectedness in the development of identity. The presence of a family atmosphere that promotes both individuality and connectedness is important in the adolescent's identity development. Individuality consists of two dimensions: selfassertion, the ability to have and communicate a point of view; and separateness, the use of communication patterns to express how one is different from others.

Connectedness also consists of two dimensions: mutuality, sensitivity to and respect for others' views; and permeability, openness to others' views.

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What the socio-cultural influences on identity development are

The surrounding culture can aid identity formation in two major ways: by providing values that have stood the test of time and continue to serve their function, and by providing social structures and customs that ease the transition from childhood to adulthood. Whether a given culture actually provides these values and social structures depends primarily on how much the members of the culture agree regarding basic principles and on how stable life circumstances are from one generation to the next.

In a culture where virtually everyone holds the same moral, political, religious and sexual values, and where social change is slow, identity is easy to achieve. Most young people is such traditional cultures simply accept the roles and values they grew up with. In modern industrial and post-industrial societies, by contrast, cultural consensus is rare and continuity is rarer still. Everything is open to question by almost everyone. When anything is possible, nothing is easy.

How gender influences identity development

Erikson described two primary developmental crises in early and middle adulthood. The first is the establishment of intimacy, which is a criterion of having attained the psychosocial state of adulthood.

By intimacy, Erikson means the ability to form close, meaningful relationships with others without 'the fear of losing oneself in the process' (Elkind, 1970). Erikson believed that a prerequisite for intimacy was the attainment of identity, or the reconciliation of all our various roles into one enduring and stable personality. Identity is necessary because we cannot know what it means to love someone and seek to share our life with them until we know who we are and what we want to do with our lives. Thus, genuine intimacy requires us to give up some of our sense of separateness, and we must each have a firm identity to do this. Intimacy does not involve sexuality. Since intimacy refers to the essential ability to relate our deepest hopes and fears to another person, and in turn accept another's need for intimacy, it describes the relationship between friends just as much as that between sexual partners .

Some researchers believe that the order of stages proposed by Erikson are different for females and males. One view is that for males identity formation precedes the stage of intimacy, while for females intimacy precedes identity. These ideas are consistent with the belief that relationships and emotional attachments are more important concerns of females, while autonomy and achievement are more important concerns of males

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Identity Management

How does our behaviour change according to the situation? What techniques or process do we employ in order to maintain, or possibly change, our identity? We do, indeed, have two sides to our identities, public and private. This lesson addresses the management of our uniqueness.

Lesson

- How our behaviour change in social situations

Objectives

- How, and why, we disclose private and personal information about ourselves to others

How our behaviour change in social situations

Impression management can take one of three forms: manner, appearance and setting. Manner consists of a person's words and nonverbal actions. The second dimension of identity management is appearance - the personal items people use to shape an image. Along with clothing, other aspects of appearance such as jewelry, tanned or light skin, hairstyle and colour, all play a major role in identity management. A final way to manage identities is through the choice of setting - physical items we use to influence how others view us. In modern Western society the automobile is a major part of identity management.

People high in self-monitoring are social chameleons - they can skillfully alter their behaviour to match the current situation. They adjust what they say and what they do to the current situation in order to make a positive impression on others. In contrast, low

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self-monitors, tend to show a higher degree of consistency. They act much the same across a wide range of situations on the basis of their particular beliefs and attitudes. Self-monitoring involves more than differences in consistency, however. Persons high in selfmonitoring are generally better than low self-monitors at both reading others' emotional reactions and managing their own nonverbal cues. Thus they are generally more successful at making a good first impression. As a result, high self-monitors tend to be more successful in their careers than low selfmonitors. High self-monitors approach new situations by asking themselves: "How can I best please the people I have to deal with?" In contrast, low self-monitors ask themselves: "How can I best be me in this situation?" .

Jones and Pittman (1982) have described several strategic self-presentational techniques that people use in everyday life:

- First is *ingratiation*, where you flatter, praise and generally make yourself likeable to another, often higher-status person.
- Second is *self-promotion* - actively "blowing your own horn" by describing your talents, exhibiting your knowledge and generally setting out to impress people.
- The flip side of self-promotion is *basking in reflected glory*. You cannot be good at everything, and so you cannot self-promote successfully in all areas - but you can become close to talented or successful people and bask in their glory and fame. In other words, you can appear impressive to others because you know or associate with impressive people.
- A final self-presentational strategy, and the one that has attracted the most research attention, is *self-handicapping*. Using this strategy, you set up reasons, before the fact, for your failure. That is, before you even engage in a task, you can make sure that you have a ready-made excuse to explain your (potentially) poor performance. There are two major ways people selfhandicap:
- In its most extreme form, people create obstacles that reduce the likelihood they will succeed on a task, so that if they do fail, they can blame it on those obstacles rather than on their lack of ability. The obstacles people have been found to use include drugs, alcohol, reduced effort on the task and failure to practise.

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- The second kind of self-handicapping is less extreme. People do not create obstacles to success, but do devise ready-made excuses in case they fail.

How, and why, we disclose private and personal information about ourselves to others There are several characteristics of self-disclosure:

- Usually happens in dyads.
- Occurs incrementally.
- Occurs in relatively few relationships.
- Is relatively scarce.
- Usually occurs in the context of positive relationships.
- There are reasons for self-disclosure.
- Improves and expands interpersonal relationships.
- Acts as catharsis, getting it "off your chest".
- Self-clarification of your beliefs, opinions, attitudes and feelings by "talking it out".
- Self-validation by seeking confirmation of important parts of your self-concept.
- Reciprocity or disclosing information about yourself to encourage another person to do so also.

Lesson Objectives

- What defines the mass media?
- How viewing violence on television or in movies - and now in video games - affect the wa we behave
- The influence does the media have on developing aggression and violence, especially in adolescents
- The influence does the Internet have social relations and behaviour

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- The influence does the media have on society and our culture
- The relationship between cultural values, the media and censorship

- Identity management or revealing personal information to make ourselves more attractive.
- Relationship maintenance and enhancement.
- Impression formation.
- Social control.
- Manipulation calculated to achieve a desired result.

Mass Media

This lesson addresses the pervasive and profound influence that television, movies, newspapers, radio and magazines have on all aspects of our lives, both personal and social.

What defines the mass media

The mass media includes television, movies, radio, music, newspapers, magazines, journals, video games, and the Internet.

How viewing violence on television or in movies - and now in video games - affect the way we behave

In many Canadian cities and in more remote communities as well, television is a common bond. In Igloolik, Nunavut, for example, of 139 households, 134 have television sets. Studies show that one in five Canadian teens watches more than five hours a day, two out of five watch three or four hours, and two out of five watch one or two hours. By the time they enter junior high, these teens have already seen 100 000 violent acts and 8

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000 murders on television or in the movies. Many of the most popular video games involve players in gruesome acts of violence, such as decapitation ("Mortal Kombat") and hanging from a meat hook by a hole drilled in the neck by a vampire ("Night Moves").

- Aggression may be learned through observation. Moreover, when adults and children are exposed to new ways of aggressing against others - techniques they have not previously seen - they may add these new behaviours to their repertoire. Later, when angry, irritated or frustrated, they may put these behaviours to actual use in assaults against others.
- Media violence conveys the message that violence is an acceptable means of handling interpersonal difficulties. It may elicit additional aggressive ideas and thoughts - convincing viewers that violence is even more common in real life than it is. It may also lessen emotional reactions to aggression and the harm it produces, so that such outcomes seem less upsetting or objectionable. This process is known as desensitization.

The fact that television portrays so much aggressive behaviour concerns parents and educators, as well as social psychologists. Half of all prime time TV characters are involved in violent activity of some kind; about one-tenth kill or are killed; the perpetrators of these crimes go unpunished in nearly threequarters of violent scenes. Sixty-one percent of television programs contain violence - and that violence is often glamorized. Moreover, about 20 percent of males appearing on TV shows are engaged in law enforcement, whereas less than one percent are in law enforcement in the real world. Although the overall amount of violence on television is staggering, some programs account for a disproportionate number of violent acts overall.

Research generally supports the contention that viewers who regularly watch violent programs on television are more likely to be aggressive than are viewers who see less television violence. Further, one study found that children exposed to large doses of TV violence are less likely to help a real-life victim of violence; another found that viewers of violence were less sympathetic to real-life victims than were non-viewers Children who play violent video games also seem to act more aggressively at later ages. Some of the key effects of viewing violence on television are:

- It weakens viewers' inhibitions.

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- It may suggest new ideas and techniques to the uninitiated.
- It may activate or stimulate existing aggressive ideas and behaviours.
- It reduces a person's overall emotional sensitivity to violence - it desensitizes people.
- It introduces a fear of becoming a victim of violence.

Influence the media has on developing aggression and violence, especially in children

Surveys indicate that news coverage of violence against women and children was not used to educate the public but rather to fascinate and entertain. Television violence, in particular, has a significant effect on the frequency and type of aggressive behaviour expressed by adults and children. Aggressive children watch more media violence, identify more with violent characters, and believe more that the violence they observe reflects real life than non-aggressive children

Issues and challenges in adolescence

This lesson addresses the issues of adolescence using a research-oriented focus. Students can then analyze the research findings in terms of a theoretical perspective to connect theory with practice.

Lesson

- The role does nutrition play in healthy adolescent development

Objectives

- The two main types of eating disorders
- Roles do genetics and the environment play in eating disorders
- The issues and challenges involved with teen suicide
- The trends and issues with juvenile violence and crime

Role nutrition play in healthy adolescent development

A sizable minority of adolescents have one of four serious problems that interfere with normal, healthy eating. Any of these can make a teenager become a shorter and less wellproportioned adult:

- Childhood habits of overeating and under-exercising often worsen in adolescence, especially if the overweight young person experiences increased social rejection. For this reason, many chubby children become obese adolescents.

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- Drug use (including cigarettes and alcohol) often begins in adolescence, affecting eating patterns, altering appetites and digestive processes, and depriving young people of energy and growth.
- Food fads and strange diets are particularly attractive, and particularly harmful, during the rapid growth of adolescence.
- Severe undernourishment slows or even halts the all the changes of puberty, including growth and sexual maturation, and it is sometimes undertaken for precisely that reason.

The two main types of eating disorders

Although there are several types of eating disorders, two broad categories are anorexia nervosa and bulimia nervosa:

- Anorexia nervosa occurs primarily in females, and female anorectics outnumber males by a factor of 15:1.The disorder usually has its onset in adolescence, the period between 14 and 16 being the most common. For a diagnosis of anorexia to be considered, the individual must weigh less than 85 percent of normal or expected weight for height, age and sex. One important characteristic of anorectics is a distorted body image in which the individual does not recognize the body's thinness. Estimates of anorexia incidence vary. American data suggest that one in 250 females may experience the disorder. In Britain, the figure is somewhat higher, ranging from one in 100 to four in 100.
- Bulimia nervosa is characterized by periodic episodes of “compulsive” or “binge” eating, the rapid and seemingly uncontrolled consumption of food, especially food rich in carbohydrates. The binge is terminated either by abdominal pain or, in the case of the purging type, by the expulsion of food using diuretics, laxatives or self-induced vomiting. In bulimia nervosa, the

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- frequency of binge eating typically averages at least two or three times a week, and sometimes as often as 30 times a week. Most bulimics are women, with fewer than five percent of the cases presenting for treatment being men. Bulimics recognize that their eating behaviour is abnormal and feel frustrated by it, but they are unable to control the urge voluntarily. Like anorectics, bulimics are unduly concerned with their body weight/shape. Although able to maintain a normal body weight, they tend to fluctuate between weight gain and loss.

Roles genetics and the environment play in eating disorders

Anorexia may have a genetic basis. There is a tendency for the disorder to run in families, with first- and second-degree relatives of anorectic individuals being significantly more likely to develop the disorder compared with first- and second-degree relatives of a control group on non-anorectics The behavioural interpretation sees anorexia as a phobia concerning the possibility of gaining weight. sociocultural explanations focus on societal norms and the cultural idealization of the slender female as possible causes for fears of being fat.

Noradrenaline, serotonin, hormones and endorphins may all play mediating roles in bulimia nervosa. Anorectics and bulimics share many psychological traits, along with the goal of maintaining a suboptimal body weight. The same person may also alternate between the same disorder.

The issues and challenges involved with teen suicide

Adolescents under the age of 20 are much less likely to kill themselves than adults are. However, adolescents think about suicide often - one of the many signs that depression is prevalent during these years. A review of studies from many nations finds that suicidal ideation, that is, thinking about committing suicide, is so common among high school students that it might be considered normal.

Suicide is commonly thought of as a response to a specific and immediate psychological blow. However, it usually is the final result of diffuse and long-standing problems within the individual, as well as within the family and social environment. Some of these problems are:

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- Being temperamentally inclined toward fits of rage or bouts of depression
- Having depressed, suicidal or alcoholic parents
- Experiencing the early loss of an important care-giving parent, grandparent or older sibling through divorce, abandonment, imprisonment or death
- Growing up with few steady friends, either because of one's personal traits or because of external circumstances, such as moving frequently Experiencing educational pressures.

Predictors of juvenile delinquency include:

- negative identity
- a low degree of self-control
- early initiation into violence and antisocial behaviour
- low expectations for education and little commitment towards education
- low school achievement in the early grades
- heavy peer influence and low resistance to peer pressure
- low socioeconomic status
- a lack of parental monitoring, low parental support and ineffective discipline

  Living in an urban, high crime and high mobility neighborhood.

Looking through the eyes of the systems of supports

This lesson addresses the influence that the parents, family and peers have on adolescent development.

Lesson How does the socio-cultural environment influence adolescent development? **Objectives**

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What importance the family is in adolescent development

In adolescence, when the winds of change blow particularly strong, parents and peers become especially powerful influences, for good or ill. Indeed, virtually every aspect of adolescent behaviour is directly affected by the family.

What roles and influence do peers have on adolescent development?

Adolescents help each other negotiate the tasks and trials of growing up in many ways. Among the special functions performed by peer relationships and close friendships, Brown (1990) finds the following four most noteworthy:

- *Self-help*. Physical changes confront the young person with new feelings, experiences and challenges to self-esteem. Peers provide both information and the companionship of those who are going through the same changes. Peers are able to listen to concerns and provide specific advice as few adults can.
- *Social support*. Friends provide social protection against the turmoil of adolescence, such as the transition to larger, more impersonal high schools.
- *Identity formation*. The peer group aids the search for self-understanding and identity by functioning as a mirror that reflects dispositions, interests and capabilities.

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- *Values clarification*. Friends are a sounding board for exploring and defining values and aspirations. By experimenting with viewpoints, philosophies and attitudes towards themselves and the world, with others who are willing to listen, argue and agree, adolescents begin to discover which values are truest to them.

How to resist peer pressure

Realize that you are being manipulated. Others are deliberately trying to get you to change your mind. If you are the only holdout, you are more likely to give in. Try to get just one other person on your side, and you will feel much stronger about your position.

- People want their behaviour and attitudes to be in harmony. Consequently, even if they know of some negative effects of what they are doing, they probably would not own up to them, because to do so would cause cognitive dissonance. Thus, you should be aware that you are probably not getting the whole story from the group.
- For all age groups, peers are more persuasive than other people are. The more prestige particular peers have, the greater their influence. Use this tendency to your advantage by finding peers who have had a bad experience with whatever you are being urged to do and listen to what they have to say.
- Minority opinions can have an effect on the majority. One way to increase this effect is to give consistent and repeated statements of your position. Often, this technique has a private effect on others - one that is not publicly expressed.
- If all the members of a group are on one side, doesn't that mean that they are more likely to be right than a single, lone individual? Actually, a group's

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decision making is usually not as accurate as an individual's, particularly if the individual is well informed and highly rational.

- When attempting to persuade someone, group members often try to make that person feel embarrassed for not going along. Embarrassing people is an age-old tool for making them conform. If you know that ahead of time, perhaps it will have less impact on you.
- One of the best ways to resist conformity is to be exposed to all the arguments in favour of some risky behaviour and then learn to refute these arguments, one by one.

The educational issues and challenges to be addressed in adolescence

Instead of an appropriate person-environment fit, a volatile mismatch forms between many adolescents and their schools. Compared to elementary schools, most secondary schools have rigid behavioural demands, intensified competition, and more punitive grading practices, as well as less individualized attention and procedures. The impersonal and bureaucratic nature of many high schools is particularly destructive for proper person-environment fit. Many secondary schools attempt to educate more than a thousand students at a time, each of whom is scheduled to travel from teacher to teacher, room to room, LESSON to LESSON on a regular schedule. Some teachers do not even know the names of the hundred or more students they teach, much less their personality traits, intellectual interests and aspirations.

The influence culture has on interpersonal relationships

One of the main dimensions on which cultures differ is individualism-collectivism. Individualism places greater emphasis on personal achievement and self-reliance.

Collectivism, by contrast, places priority on the welfare and unity of the group. Goodwin (1995) argues that "love, at least in its passionate stomach-churning Hollywood manifestation, is largely a Western and individualistic phenomenon and that in Western cultures, marriage is seen as the culmination of a loving relationship". In cultures where arranged marriages occur, the relationship between love and marriage is the other way around, and marriage is seen as the basis on which to explore a loving relationship. The

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cultural background in which people have learned about love is important in shaping their concept of it **Spiritual Growth**

In his summary of the literature, Kenneth Hyde (1991) describes many religious aspects of adolescence, such as their behavior, attitudes, influences, and understandings of religion. While most Christian schools and churches have emphasized learning about the Bible, God, and beliefs, the last few years have produced a new emphasis upon the experiential aspects of faith, often termed "spirituality." This is in part because one can know a great deal about God, and even make an initial commitment to the Christian faith, yet not have an active, ongoing relationship with God that includes religious experiences. "Spirituality" emphasizes awe, wonder, and other experiences that are beyond mundane, everyday life, and that connect the individual to something transcendent and/or of ultimate importance. While spirituality includes experiences with God, it can also include awe and wonder from experiences such as viewing a beautiful sunset that overwhelms the person emotionally and even perceptually. Spiritual experience can be either positive and constructive, or negative and counterproductive (Hay & Nye, 1998).

A Few Negative Spiritualities

[this designation is similar to what Berryman (1991) terms "pseudo-play"]

1. Violence
2. Suicide
3. Bullying
4. Illegal Drug Use
5. Risk-Taking
6. Controlling with Detachment and Disguise (by "spirituality sponges" who are parasites, says Berryman)
7. Lack of Moral Impulse/Corruption
8. Lack of Concern for Others
9. Meaninglessness
10. Prejudice

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11. Materialism

12. Evil Personality (M. Scott Peck)

For some people, the progression is not as uniform as Darling's model may suggest. Some mistakenly confuse moods with spirituality, and thus their perception of their walk of faith may be cyclical—hills and valleys which reflect positive and negative emotions that can change from day to day. These are not to be confused with times of struggle and victory which are normal aspects of the Christian life, sometimes described as "mountain top experiences" and "valleys" or "dry times in the wilderness." Others live out a crisisbased faith, marked by occasional moments of spiritual experience during revival services or retreats; sudden shifts from a normally mundane spiritual existence. Perhaps a combination of Darling's loops, and occasional valleys and "mountain top experiences" are the most common components of healthy Christian living, loops and cycles that are part of a long-term incline marking spiritual growth and development. Spiritual adolescence may or may not be part of that progression.

In sum, healthy Christian living is marked by growth in understanding and a deepening relationship with God. Spiritual development involves confessing and turning from sin, experiencing difficult times when God seems remote, contrasted with moments of spiritual vitality—times of worship and felt love—when God is experienced as being "closer than a brother." Spiritual formation activities can assist this process, including prayer, retreats, meditation, fasting, and the like.

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Action research in adolescent development

This summary lesson will integrate all of the concepts covered in this unit, by actively engaging students in the design and conduct of research in developmental psychology.

Lesson issues and LESSONs for research in adolescence

Objectives

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Suggested Research LESSONs and Issues

Type of Research	Suggested Research LESSONs

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Survey - Conducting research using a survey involves going out and asking questions about the phenomenon of interest.	• What should the age to drive a vehicle be? What should be the legal age to consume • alcohol? • What qualities do you admire most in a friend? • What attracts you to the opposite sex? • Moral dilemmas: • Is it right to steal life-saving medicines that could save the life of a family member? • Is it right to tell the police if you know that a friend has committed a crime? • If you found a large sum of money in an unmarked envelope, should you keep it? • Adolescent lifestyles: Smoking and drinking • Do you believe in God? Heaven? Hell? Life after death? • Stereotypes and biases: Complete this sentence.

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	• Professional athletes are ….
	• Men are …. • Women are …. • Parents are ….

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Naturalistic Observation In naturalistic research, the observer does not intervene at all. For all intents and purposes, the researcher is invisible and works hard not to interrupt the natural dynamics of the situation being investigated.	• How does the media influence adolescent perceptions of body image? • What are the eating habits of adolescents? • What different types of group behaviours are demonstrated by adolescents? • Are there differences in gender roles demonstrated by adolescents? • In what ways are bias, stereotypes, prejudice, and discrimination against adolescents demonstrated? • How does culture influence adolescent development? • How does the media represent adolescents?

LESSON A LESSON research study involves the acquisition, synthesis, organization, and presentation of information. Typically, the LESSON research study will involve both	• What is anorexia nervosa? • What is bulimia nervosa? • What are the issues surrounding teen suicide? • What are the issues and challenges faced by teen mothers?

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paper based as well as web-based resources. Additional information may be gained through the other research methods and strategies.	How do the cultural industries (beauty, fashion, music) influence adolescent development?
Case Study A case study is an intensive study of one individual Typically, the case study is based on interviews with the subject regarding his or her background, present thinking or actions; it may also utilize interviews of others who know the individual.	• Who was Rosa Parks? • Who was the Reverend Martin Luther King Jr.? • Who was Mahatma Gandhi? • Who was Mother Theresa? • Who is Nelson Mandela?
Interview The interview method of research, typically, involves a face-to-face meeting in which a researcher (interviewer) asks an individual a series of questions.	• What was it like to be a teenager when your parents were adolescents? • How have the parenting styles and strategies of your parents changed as you became an adolescent? • How have families changed since your parents were teenagers?

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Perspectives on adolescent development

This summary lesson considers how the lifespan approach to human development can be used as a lens through which we can view adolescent development.

Lesson Objectives

- How adolescent development described from the perspective of the lifespan approach
- How the key issues and questions in developmental psychology relate to adolescent development
- How each of the six theoretical perspectives on human development explain adolescent development

How adolescent development described from the perspective of the lifespan approach.

The lifespan perspective on human development has seven basic characteristics. Development is:

- Life-long
- No age period dominates development.
- Multi-dimensional
- Development consists of biological, cognitive, socio-emotional and spiritual dimensions.
- Multi-directional
- Some aspects of development increase, while others decrease.
- Plastic
- Depending on the individual's life conditions, development may take many paths.
- Historically-embedded
- Development is influenced by historical conditions.
- Multidisciplinary

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- Psychologists, sociologists, anthropologists, neuroscientists and medical researchers all study human development and share a concern for unlocking the mysteries of development throughout the lifespan.
- Contextual
- The individual continually responds to and acts on contexts, which include a person's biological makeup, physical environment, and social, historical, and cultural contexts.

Key issues and questions in developmental psychology related to adolescent development

From the time of its establishment, several key issues and questions have dominated the field of developmental psychology. Among the issues are the nature of developmental change, the importance of critical periods, lifespan approaches versus the more focused approaches, and the nature-nurture issue.

- **Continuous change versus discontinuous change:** In continuous change, developmental change is gradual, with achievements at one level building on those of previous levels. In contrast, discontinuous change occurs in distinct stages or steps. Each stage brings about behaviour that is assumed to be qualitatively different from behaviour at earlier stages.
- A **critical period** is a specific time during development when a particular event has its greatest consequences. Critical periods occur when the presence of certain kinds of environmental stimuli are necessary for development to proceed normally.
- **Lifespan approaches versus a focus on a particular period:**

  Developmentalists now believe the entire lifespan is important, for several reasons. One is the discovery that developmental growth and change continue during every part of life. Furthermore, to understand fully the social influences

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on people of a given age, we need to understand the people who are in large measure providing those influences. For instance, to understand development in infants, we need to unravel the effects of their parents' ages on the social environment.

- **Nature versus Nurture:** One of the enduring questions of development involves how much of people's behaviour is due to their genetically-determined nature and how much is due to nurture, the physical and social environment in which a child is raised. In this context, **nature** refers to traits, abilities and capacities that are inherited from one's parents. Nature encompasses any factor that is produced by the predetermined unfolding of genetic information - a process known as maturation. These genetic, inherited influences are at work as we move from the onecelled organism that is created at the moment of conception to the billions of cells that make up a fully-formed human being. In contrast **nurture** refers to the environmental influences that shape behaviour. Some of these influences may be biological, such as the impact of a pregnant mother's substance abuse on the fetus, or the amount and kind of food available to children. Other environmental influences are more social, such as the ways parents discipline their children and the effects of peer pressure on adolescents.

How each of the six theoretical perspectives on human development explains adolescent development

Lifespan development has produced a number of broad conceptual perspectives representing different approaches to development. Each broad perspective encompasses a number of theories, explanations and predictions concerning phenomena of interest. A theory provides a framework for understanding the relationships among an organized set of principles or facts. The six major theoretical perspectives include:

- **Psychodynamic Perspective: Focusing on the inner person. Rooted** in

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Freud's theory, the psychodynamic approach maintains that all behaviour and mental processes reflect the constant and mostly unconscious psychological struggles that rage silently within each person. Usually, these struggles involve conflict between the impulse to satisfy instincts or wishes and the need to play by the rules in society. Anxiety, depression and other disorders are outward signs of this inner turmoil.

- **The Behavioural Perspective: Considering the outer person.** As founded by John Watson, the behavioural approach views behaviour and mental processes as primarily the result of learning. Psychologists who take this approach see rewards and punishment acting on the raw materials provided by genes, evolution and biology to shape each individual. So, whether considering a person's aggression or drug abuse, behaviourists would look at that person's learning history. Since people learn problem behaviours, they can also learn to change or even prevent them by unlearning old habits and developing new ones.
- **The Cognitive Perspective: Examining the roots of understanding. The** cognitive perspective focuses on how people take in, mentally represent and store information. Cognitive psychologists then relate perception and information processing to patterns of behaviour. They study such areas as decision-making, problem-solving, interpersonal attraction and intelligence. Aggression, for instance, might be viewed as a result of poor problem solving.
- **The Humanistic Perspective: Concentrating on the unique qualities of human beings.** According to the humanistic approach, our capacity to choose how to think and act determines our behaviour. Each person's unique perceptions – not instincts, cognitive processes, or rewards and punishments – dictate the choices made. Humanistic psychologists believe that people are

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essentially good, that they are in control of themselves, and that they seek to grow toward their highest potential.

- **The Evolutionary Perspective: Focusing on biology as the determinant of development.** Darwin's ideas on evolution and adaptation of species laid the foundation for the evolutionary approach. The evolutionary approach to psychology holds that the behaviour of animals and humans today is the result of evolution through natural selection. Psychologists who follow this approach are concerned with the adaptive value of behaviour, the anatomy and biology that make it possible and the environmental conditions that encourage or discourage it.
- **The socio-cultural Perspective: Emphasizing the systems of support. Calling** attention to the external influences on human behaviour such as the physical surroundings and social interactions that provide incentives, opportunities and pathways for growth, Urie

Bronfenbrenner (1979) began to emphasize the ecological approach to the study of human development. In Bronfenbrenner's application of this concept, human ecosystems include both the physical environment (the climate, the space per person, that arrangement of the dwelling) and the social environment (the people, the culture, the economy)

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LESSON FOUR CHILD DEVELOPMENT THEORIES

Sigmund Freud

The theories proposed by Sigmund Freud stressed the importance of childhood events and experiences,

but almost exclusively focused on mental disorders rather that normal functioning.

According to Freud, child development is described as a series of 'psychosexual stages.' In

"Three Essays on Sexuality" (1915), Freud outlined these stages as oral, anal, phallic,

latency and genital. Each
stage involves the satisfaction of a libidinal desire and can later play a role in adult personality. If a

child does not successfully complete a stage, Freud suggested that he or she would develop a fixation

that would later influence adult personality and behavior. Learn more in this article on Freud's stages of psychosexual development.

Erik Erikson

Theorist Erik Erikson also proposed a stage theory of development, but his theory encompassed

development throughout the entire human lifespan. Erikson believed that each stage of development

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was focused on overcoming a conflict. For example, the primary conflict during the adolescent period

involves establishing a sense of personal identity. Success or failure in dealing with the conflicts at
each stage can impact overall functioning. During the adolescent stage, for example, failure to develop

an identity results in role confusion. Learn more about this theory in this article on Erikson's stages of psychosocial development.

Cognitive Child Development Theories

Theorist Jean Piaget suggested that children think differently than adults and proposed a stage theory of
cognitive development. He was the first to note that children play an active role in gaining knowledge

of the world. According to his theory, children can be thought of as "little scientists" who actively

construct their knowledge and understanding of the world. Learn more in this article on Piaget's stages of cognitive development.

Behavioral Child Development Theories

Behavioral theories of child development focus on how environmental interaction influences behavior

and are based upon the theories of theorists such as John B. Watson, Ivan Pavlov and B. F.

Skinner. These theories deal only with observable behaviors. Development is

considered a reaction to rewards,

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punishments, stimuli and reinforcement. This theory differs considerably from other child development

theories because it gives no consideration to internal thoughts or feelings. Instead, it focuses purely on

how experience shapes who we are. Learn more about these behavioral theories in these articles on classical conditioning and operant conditioning.

Social Child Development Theories

John Bowlby

There is a great deal of research on the social development of children. John Bowbly proposed one of

the earliest theories of social development. Bowlby believed that early relationships with caregivers

play a major role in child development and continue to influence social relationships throughout life.

Learn more in this overview of attachment theory.

Albert Bandura

Psychologist Albert Bandura proposed what is known as social learning theory. According to this

theory of child development, children learn new behaviors from observing other people. Unlike

behavioral theories, Bandura believed that external reinforcement was not the only way that people

learned new things. Instead, intrinsic reinforcements such as a sense of pride, satisfaction

and accomplishment could also lead to learning. By observing the actions of others,

including parents and peers, children develop new skills and acquire new information.

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Lev Vygotsky

Another psychologist named Lev Vygotsky proposed a seminal learning theory that has gone on to

become very influential, especially in the field of education. Like Piaget, Vygotsky believed that

children learn actively and through hands-on experiences. His sociocultural theory also suggested that

parents, caregivers, peers and the culture at large were responsible for the development of higher order

functions.

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LESSON FOUR: ADULTHOOD

Development in Early & Middle Adulthood

Adulthood has no signpost to announce its onset (as adolescence is announced by puberty). In technologically advanced nations, the life span is more than 70 years. Developmental psychologists usually consider early adulthood to cover approximately age 20 to age 40 and middle adulthood approximately 40 to 65.

Early adulthood. In **early adulthood**, an individual is concerned with developing the ability to share intimacy, seeking to form relationships and find intimate love. Long-term relationships are formed, and often marriage and children result. The young adult is also faced with career decisions.

- Choices concerning **marriage** and **family** are often made during this period. Research shows that divorce is more likely among people who marry during adolescence, those whose parents were divorced, and those who are dissimilar in age, intelligence, personality, or attractiveness. Separation is also more frequent among those who do not have children. Most people who have divorced remarry; consequently, children may experience more than one set of parents.

- Such alternatives to marriage as "living together" (**cohabitation**) have become more common. In 1997, the Census Bureau estimated that 4.13 million unwed couples lived in the United States.

- **Work/career choice** affects not only socioeconomic status but also friends, political values, residence location, child care, job stress, and many other aspects of life. And while income is important in both career selection and career longevity, so are achievement, recognition, satisfaction, security, and challenge. In the modern cultures of many nations, the careers of both spouses or partners frequently must be considered in making job choices.

Middle adulthood. In middle adulthood, an important challenge is to develop a genuine concern for the welfare of future generations and to contribute to the world through family and work. Erik Erikson refers to the problem posed at this stage as

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generativity vs. self-absorption. Robert Havighurst lists seven **major tasks** in the middle years.

- accepting and adjusting to physiological changes, such as menopause
- reaching and maintaining satisfaction in one's occupation
- adjusting to and possibly caring for aging parents
- helping teenage children to become responsible adults
- achieving adult social and civic responsibility
- relating to one's spouse as a person
- developing leisure-time activities

While a **midlife crisis** is not regarded as a universal phenomenon, during one's 40s and 50s comes the recognition that more than half of one's life is gone. That recognition may prompt some to feel that the clock is ticking and that they must make sudden, drastic changes in order to achieve their goals, while others focus on finding satisfaction with the present course of their lives.

Development in Late Adulthood

Late adulthood (old age) is generally considered to begin at about age 65. Erik Erikson suggests that at this time it is important to find meaning and satisfaction in life rather than to become bitter and disillusioned, that is, to resolve the conflict of **integrity vs. despair**. It has been estimated that by the year 2030, Americans over 65 will make up 20% of the population. Despite the problems associated with longevity, studies of people in their 70s have shown that growing old is not necessarily synonymous with substantial mental or physical deterioration. Many older people are happy and engaged in a variety of activities. **Gerontology**, an interdisciplinary field that studies the process of aging and the aging population, involves psychology, biology, sociology, and other fields.

Theories of successful ageing. Theories of successful aging include the following:

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- The **disengagement theory** states that as people age, their withdrawal from society is normal and desirable as it relieves them of responsibilities and roles that have become difficult. This process also opens up opportunities for younger people; society benefits as more-energetic young people fill the vacated positions.

- The **activity theory** contends that activity is necessary to maintain a "life of quality," that is, that one must "use it or lose it" no matter what one's age and that people who remain active in all respects—physically, mentally, and socially—adjust better to the aging process. Proponents of this theory believe that activities of earlier years should be maintained as long as possible.

Ageism. Ageism may be defined as the prejudice or discrimination that occurs on the basis of age. Although it can be used against people of all ages, older people are most frequently its target and it may often result in forced retirement. Stereotyping of the elderly is also an aspect of ageism, as seen in such a statement as "He drives like a little old lady."

Physical changes. People typically reach the peak of their physical strength and endurance during their twenties and then gradually decline. In later adulthood, a variety of physiological changes may occur, including some degree of atrophy of the brain and a decrease in the rate of neural processes. The respiratory and circulatory systems are less efficient, and changes in the gastrointestinal tract may lead to increased constipation. Bone mass diminishes, especially among women, leading to bone density disorders such as osteoporosis. Muscles become weaker unless exercise programs are followed. The skin dries and becomes less flexible. Hair loss occurs in both sexes. There is also decreased sensitivity in all of the sensory modalities, including olfaction, taste, touch, hearing, and vision.

Cognitive changes. The study of cognitive changes in the older population is complex. Response speeds (neural and motor) have been reported to decline; some researchers believe that age-related decrease in working memory is the crucial factor underlying poorer performance by the elderly on cognitive tasks.

- **Intellectual changes** in late adulthood do not always result in reduction of ability.

While **fluid intelligence** (the ability to see and to use patterns and relationships

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to solve problems) does decline in later years, **crystallized intelligence** (the ability to use accumulated information to solve problems and make decisions) has been shown to rise slightly over the entire life span. K. Warner Schaie and Sherry Willis reported that a decline in cognitive performance could be reversed in 40% to 60% of elderly people who were given remedial training.

- **Dementias** are usually responsible for cognitive defects seen in older people. These disorders, however, occur only in about 15% of people over 65. The leading cause of dementia in the United States is **Alzheimer's disease**, a progressive, eventually fatal disease that begins with confusion and memory lapses and ends with the loss of ability to care for oneself.

Retirement. Retirement at age 65 is the conventional choice for many people, although some work until much later. People have been found to be happier in retirement if they are not forced to retire before they are ready and if they have enough income to maintain an adequate living standard. Chronic health problems such as arthritis, rheumatism, and hypertension increasingly interfere with the quality of life of most individuals as they age.

Widowhood. Women tend to marry men older than they are and, on average, live 5 to 7 years longer than men. One study found ten times as many widows as widowers. Widowhood is particularly stressful if the death of the spouse occurs early in life; close support of friends, particularly other widows, can be very helpful.

Death and dying. Death and dying has been studied extensively by Elisabeth KüblerRoss, who suggested that terminally ill patients display the following five basic reactions.

- **Denial**, an attempt to deny the reality and to isolate oneself from the event, is frequently the first reaction.

- **Anger** frequently follows, as the person envies the living and asks, “Why should I be the one to die?”

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- **Bargaining** may occur; the person pleads to God or others for more time.
- As the end nears, recognition that death is inevitable and that separation from family will occur leads to feelings of exhaustion, futility, and deep **depression**.
- **Acceptance** often follows if death is not sudden, and the person finds peace with the inevitable.

People who are dying are sometimes placed in a **hospice**, a hospital for the terminally ill that attempts to maintain a good quality of life for the patient and the family during the final days. In a predictable pattern after a loved one's death, initial shock is followed by grief, followed by apathy and depression, which may continue for weeks. Support groups and counselling can help in successfully working through this process.

Darling's Spiritual Growth Paradigm

Harold Darling (1969) has suggested a four-stage model of spiritual development based

Upon the stages of physical and emotional development. Just as individuals develop through

Birth, childhood, adolescence, and adulthood, likewise these terms may be used to describe Spiritual development.

Darling's first spiritual stage is "birth." The initial crisis in physical and social

Development is when the newborn first sees the light of the
outside World. Darling writes:
Life in Christ begins with a birth experience. This "new birth" is

Attended by pain and suffering; it comes about by confession of our

Sins, commitment of our total being to God, and faith that God

Honors that commitment (pp. 142-143).

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The Bible repeatedly describes conversion as birth. Christ's conversation with

Nicodemus is perhaps the most vivid comparison of physical and spiritual birth (John 3). This analogy for conversion is also mentioned in John 1:13; Galatians 4:29; 1 Peter 1:23; and I John

2—5. The "born again" experience is a thoroughly biblical concept.

Human birth is preceded by a period of preparation in which the embryo

and later the fetus develops through stages. Engel and Norton (1975)

have suggested a process of development prior to spiritual birth in

which God brings an individual to salvation. This development process

includes:

1. awareness of a supreme being through general revelation;

2. initial awareness of the Gospel;

3. grasping the implications of the Gospel;

4. a positive attitude toward the Gospel;

5. personal problem recognition; 6, a decision to act;

7. repentance and faith in Christ,

Engel and Norton state that successful evangelism will stimulate movement through these stages until spiritual birth occur.

Subsequent to birth, one enters the stage of childhood. Says Darling: The childhood stage of Christian living is characterized by growth, security, spontaneity, and conformity. We accept the teachings of authority (specifically parents and church) without questions. We are living in the "awe" stage of our relationship to God, and our obedience follows a rather easy, natural course (pp. 143-144).

Biblical analogies to such a spiritual stage are abundant. Jesus stated that one must become like a child to enter the kingdom of God (Matthew 18:4). He also stated that the kingdom of God belongs to those who are like children (Mark 10:14). Christ compares His disciples to youngsters (Matthew 11:25). Likewise, Peter compared young Christians to children:

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“Like newborn babies, crave pure spiritual milk, so that by it you may grow up in your salvation” (1 Peter 2:2, NIV)

Eventually most Christians begin to outgrow spiritual childhood. Doubts and questioning begin to characterize a new stage of spiritual development—adolescence. Darling states:

We become vaguely discontented with the spiritual status quo… our Christian living is too ordinary; our God is too small…such an awareness produces a crisis: something must be done (p. 145).

Spiritual adolescence has been further explored as a necessary transition by Berry (1980). Citing a number of case histories from his counseling practice, he emphasizes the commonness of such an experience. Often persons going through this turbulent period of spiritual growth will seek out a counselor or pastor to assist them. Berry suggests that these individuals should be instructed to see the present situation as a “Jordan-crossing” experience and to direct the counselee’s attention to Canaan on the other side (spiritual adulthood). In the process of this change the “narcissism of childhood” becomes less prominent.

Such a period of spiritual turbulence has often preceded vibrant ministries, both among those espousing Wesleyan theology and those who hold to Reformed doctrines. Edman (no date) describes such experiences in the lives of J. Hudson Taylor, Walter Wilson, D.L. Moody, Richard Halverson and others prior to being greatly used by God.

Borrowing heavily from psychodynamic theory, Darling suggests seven possible means of resolving the conflicts of spiritual adolescence. Six of the seven are spiritually dysfunctional, with the seventh being the key to spiritual adulthood. Spiritual “neurosis” is one means used to reduce the conflicts of this third stage; the individual becomes a “spiritual hypochondriac” in which there is abnormal concern about the spiritual condition? A second alternative is spiritual “psychosis” in which minor issues and doctrines are elevated, and a fanaticism develops which lacks depth and power.

A third possibility is re-enslavement in which the person is overcome with defeat and life becomes much like that which existed prior to salvation. A fourth path, is regression; returning to the narrow conformity of childish immaturity in which others are allowed to do the thinking. Fixation can be a fifth way of attempting to resolve the crisis. This involves a perpetual adolescence with turmoil, and

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faultfinding a permanent way of life. Darling writes: There is however, another alternative, and that is surrender. We embrace the way of faith, commitment, and abandonment; the Holy

Spirit takes control. We appropriate the power that has motivated the great souls of all the ages, enabling them to overcome their self-centeredness and unholy fear. . . . This is the life, we confidently assent, that Christ promised us in John 10:10, "I came that they may have life, and have it more abundantly" (p. 146).

This surrender introduces a fourth stage, that of adult maturity. Again, a number of

Scriptures describe maturity in contrast to earlier spiritual immaturity, using the human growth analogy. Elementary truths are considered to be milk for infants, while mature Christians are to eat "solid foods" (i.e., advanced teaching) as an adult would (Hebrews 5:13-14). Rather than students, as children would be, individuals become teachers as spiritual adults (Hebrews 5:12). Paul says, "Then we will no longer be infants . . . blown here and there by every wind of teaching. [but) instead, speaking the truth in love, we will in all things grow up into him..," (Ephesians 4:14-15, NIV).

Spiritual adulthood possesses at least some of the characteristics of selfactualization. Oakland (1974) suggests that the Wesleyan view of

sanctification may be equated with self actualization, including Maslow's peak experiences. Elsewhere in his book, Darling describes the abundant Christian life in terms not uncommon in actualization psychology: confident surrender, transparent spontaneity, adjusting tension, purposeful integration, and wholehearted affirmation.

During both childhood and adulthood, Darling suggests that there are "loops" in spiritual growth. These are periods of struggle, which include confrontation with

"sins, glaring imperfections, moods, destructive tensions, and ever-present proneness to wander from the path of faith and obedience" (pp. 146-147). Rather than succumbing to these, however, the adult practices confession, affirmation, and appropriation through prayer and acting upon the insight received. Such experiences are integral to the growth process, regardless of the spiritual stage.

These spiritual loops are not unlike Bright's (1966) concept of spiritual breathing.

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Bright suggests that when a problem is found in one's life which is displeasing to God, one should first confess and repent (exhaling) and then surrender and appropriate (inhaling). Darling's spiritual "loops" also remind one of Wesley's account of growth prior to, and subsequent to, a second crisis experience.

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